Canadian Community Health Survey (CCHS) 2016

Interviewer Manual (CAPI/CATI)

Collection Planning and Research Division

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1. Survey overview

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Overview

Introduction

The Canadian Community Health Survey (CCHS) is a large cross-sectional survey conducted by Statistics Canada to gather information on the health of Canadians. The survey began collecting data in 2001, with continuous collection beginning in 2007. CCHS will remain in continuous collection for 2016.

This manual was prepared to provide guidance and assistance to the CAPI & CATI interviewer throughout training and during data collection of this vital health information. It is important to be familiar with this manual and to ensure that it is available as a resource at all times during your work on the CCHS.

This chapter presents general information on the survey and guidelines about informing the respondent and provides:

- recent survey highlights
- how the data will be used
- letters and brochures, and
- questions and answers.

What's new in the survey?

Introduction

The CCHS 2015 was part of an overall survey redesign. As a result, the 2016 collection period does not consist of major changes to the survey or the application.
Currently there is one new module for the CCHS 2016. Medical Doctor Attachment (MDA). This module is an optional module available only to British Columbia.

What's new

1. CATI - Moving back from Telephone number 2 to telephone number 1.
2. CATI/CAPI - Brochures – Now province/territory specific.
3. Linking Tax Data.

Moving back to telephone number 1

An update to the application for 2016 has been made to the 18+ sample that have multiple phone numbers.

An update has been made to the 18+ sample that have multiple phone numbers. This was updated as a result of the "stop-transfer" functionality that was added to the CCHS 2015 (for the CATI to CAPI transfers).

Once the stop-transfer function has been activated (turned on), cases that have moved to telephone number 2 (TELNUM2) due to no contact from telephone number 1 (TELNUM1), the following will occur:

- If TELNUM2 is coded an outcome code of 2, 3, or 5, the application will move back to TELNUM1.
- TELNUM1 is continued to be called until the case is resolved or the collection period ends (If TELNUM1 is coded an outcome code of 2, 3 or 5, it will be sent home as a final code).

This has been programmed into the application; you will be required to simply follow the phone number provided on the screen.

Brochures

For 2016, each province/territory will now have its own CCHS brochure. Each brochure will contain province/territory information with specific references and uses of the CCHS data (where applicable) in that area.

In addition, the design of the overall look of the brochure will differ from province/territory, which each being a different color and using city images for background images that are specific to the province/territory.

Linking Tax Data

The 2016 questionnaire/application has been modified so that respondents with records that contain linkable tax data will no longer be asked the income question (unless they refuse the linkage statement in ADM). These cases will be pre-identified by Head Office. Individuals whose records cannot be linked to tax data (again preidentified by HO), <u>will</u> be asked the INC question.

This change has been made in the application and will occur automatically.

Survey information

Introduction

The Canadian Community Health Survey (CCHS) is a cross-sectional annual survey that collects information related to health status, health care utilization and health determinants for the Canadian population. It relies upon a large sample of respondents (65,000) and is designed to provide reliable estimates at the health region level.

Information from CCHS helps researchers and analysts track population health and the impact of health programs by health region. The survey results will also allow more detailed study of groups such as seniors, off-reserve Aboriginals, and single mothers to name a few.

The Canadian Community Health Survey marks a turning point in collecting information on the health of Canadians. It provides valuable and timely data on Canadians' health status, risk factors, and health care use. As a result of information collected in previous years, health-related characteristics of the population can be compared not only at provincial and territorial levels, but at the local health region level as well.

A health region is a geographic area of health planning, spending and control that falls under each Provincial and Territorial Ministry of Health. 117 health regions have been identified across the country.

Statistics Canada sometimes splits or combines some health regions within a province for sampling purposes.

Objectives

The three main objectives of the Canadian Community Health Survey are:

1. **To provide regular and timely estimates of health information.**

   More data is needed on a regular and timely basis in order to follow trends, behaviours and the impact of policy and program changes.

2. **To provide estimates at both the provincial, territorial and health region levels.**

   The information is to be gathered at a provincial and sub-provincial level by health regions. By collecting information at this level, data can be tailored to meet the specific needs and uses of communities, and can be used at the level at which health planning occurs.

3. **To provide flexibility to vary content to meet unique regional, provincial and territorial needs.**

   By providing some choice as to the content of the survey, we allow health regions to select topics that are important to them while at the same time providing common content for comparison purposes at all levels.

Users and uses

Health is a foremost topic in the public eye. Whether it is the number of health care workers, statistics on sickness and disease, the effects of lifestyle on health – people are concerned and want to know the issues affecting them.

The data is used for:

- Planning,
- Budgeting and spending, and
- Development and tracking of programs and policies.

There are many users of the data, including:

- Health Canada,
- Provincial and territorial ministries and departments of health,
- Regional health planners across the country,
- University researchers,
- News media,
- Insurance and pharmaceutical companies, and Consultants.

Survey focus

The CCHS population health approach provides indicators related to mental and social well-being, quality of life, life satisfaction, income, employment, and working conditions, education and other factors known to influence health.

The content focuses on three main areas of interest:

1. **Health Determinants** are factors that help determine what makes a person healthy or unhealthy, such as smoking, physical activities and food security. Much of the health research indicates that we need to look at the big picture of health to examine factors both inside and outside the health care system that affect our health. Health is determined by complex interactions between social and economic factors, the physical environment and individual behaviour. These factors are referred to as ‘determinants of health’.
2. **Health Status** indicators are indicators of the state of health of an individual, group or population at a specific point in time measured against identifiable standards, such as chronic conditions and injuries.
3. **Health System Utilization** modules, which measure the use of services such as colorectal cancer screening, flu shots, and home care.

Project codes

Interviewer time should be charged to the following project codes:

Description	Code
CAPI	4145
CATI	4146
CATI North	4430

Survey calendar

The table below presents the various collection dates for the CCHS.

Collection Period ID	Start Date	End Date
201601	January 4, 2016	March 31, 2016
201604	April 4, 2016	June 30, 2016
201607	July 4, 2016	September 30, 2016
201610	October 3, 2016	December 24, 2016

Note: Dates may change due to operational constraints.

Response rate

The targeted response rate for CCHS 2016 is **65.5%** overall including CAPI and CATI.

Interview length / time

The CCHS interview will be approximately **60** minutes.

The C2 component, consisting of health and socio-demographics modules, will take approximately **45-50** minutes to complete, on average.

Allow an additional **10-15** minutes for Entry, Exit and Household rostering.

Voluntary status

This survey is voluntary and is collected under the authority of the Statistics Act.

Sample

Sample distribution by Regional Office:

Regional office	CATI	CAPI	Expected Transfers CATI to CAPI
Halifax	9,251	3,946	1,363
Sherbrooke/Montréal	17,687	3,839	2,193
Sturgeon Falls/Toronto	27,132	5,911	3,864
West	29,656	11,957	5,045
Total	**83,736**	**25,652**	**12,465**

*Transfer numbers are estimates.

Target population

The CCHS covers the population **12 years of age and over** living in the ten provinces and the three territories.

Excluded from the survey's coverage are: persons living on reserves and other Aboriginal settlements in the provinces; full-time members of the Canadian Forces; the institutionalized population and persons living in the Quebec health regions of Région du Nunavik and Région des Terres-Cries-de-la-Baie-James. Altogether, these exclusions represent less than 3% of the target population.

However, the sample will come from two frames (explained below). Each frame covers a different age group and has a different sampling unit.

Sample frame

The sample for CCHS 2016 will be drawn from the Area frame (HSF) and from the CCTB (Canadian Child Tax Benefits) frame. Below is a summary of some important information about the Area & CCTB frames.

	Area Frame		CCTB Frame
Target population	18 years and older		12-17 years
Sampling unit	Dwelling		Person
Collection unit	Person		Person
Mode of Collection	CATI CAPI	CAPI & CATI-CAPI transfers	CATI only
Respondent Selection	Random selection		Pre-selected

Sampling & collection unit

AREA FRAME

- A **sample of dwellings** is selected (designed to target the population aged 18+).
- One eligible household member (18+) will be randomly selected by the computer application to complete the questionnaire.
- Cases will start as CAPI if there are no phone numbers available.
- Cases with at least one phone number available will start as CATI.
- CATI cases with out-of-service phone numbers and phone numbers that do not reach the selected addresses will be transferred to CAPI.

CCTB FRAME (CATI only)

- A sample of persons is selected from the CCTB frame (designed to target the population aged 12-17).
- One child (12-17 years) is pre-selected to complete the questionnaire (CATI environment only).
- No-contact cases will be traced.

Sample Overlap

The sampling methodology for CCHS is designed to avoid overlap between collection modes (CAPI and CATI) and to avoid duplication within a two-year period. As such, it is unlikely that a household would have been sampled for CCHS recently or sampled twice within a single collection cycle.

However, while expected to rarely happen, it is still possible for an household to have been sampled for CCHS recently or sampled twice within a single collection cycle.

Mode of collection

The mode of collection for CCHS 2016 will be CATI, CAPI and transfers from CATI to CAPI.

Mode	Sample
CATI	All sample selected from the CCTB frame and all sample selected from the Area frame with at least one telephone number available will start as CATI.
CAPI	All sample selected from the Area frame without a telephone number will start as CAPI.
CATI to CAPI	Only cases from the Area frame can be transferred to CAPI.

Respondent selection

All respondents 18 years and over are randomly selected after the roster is completed. All respondents 12-17 years old are pre-selected.

Validation

To ensure the accuracy and quality of the survey, Regional Office managers and supervisors will contact randomly selected households during each collection period to verify that a legitimate interview took place. They will also provide progress reports on a regular basis to interviewers.

Monitoring and feedback

Monitoring is a required management tool used to ensure data quality and timeliness.

The main purpose of monitoring is to provide interviewers with a constructive, supportive and balanced feedback based on what the monitor hears interviewers say to the respondent. In this way they will help interviewers maintain and/or improve a high degree of professionalism.

The monitoring and feedback process is objective and comprehensive.

There are specific behaviours or skill sets and characteristics that will be examined by the monitor, who will discuss their observations with you during feedback sessions.

During CATI monitoring, Regional Office staff will monitor telephone calls in the CATI environment using the current monitoring system, in accordance with monitoring guidelines, and provide feedback to interviewers.

During CAPI monitoring parts of the interview are recorded. CAPI monitors will listen to the recordings to ensure that the four pillars of successful interviewing are being followed and that the survey specific requirements are being respected so that data quality was ensured during the collection activity. Please refer to the SSO manual to review the four pillars of successful interviewing in more detail if necessary (Prepare, Engage, Conduct and Close the interview). Supervisors will provide feedback to interviewers.

All interviewers will be provided with constructive, supportive and balanced feedback based on what the monitor hears the interviewer say to the respondent.

Respondent relations

Introduction

Successful respondent relations are the key to obtain accurate data. The CCHS has at its disposal a number of resources Interviewers can utilize to inform respondents of the surveys importance and uses, as well as to address questions and potential participation refusals.

Survey materials

Ensure that you receive the following supplies and always have them available while interviewing;

- Interviewers manual,
- Note to Parents/Guardians,
- Quick Reference Card,
- Cluster list and diagram (FO2),
- Brochure (province specific),
- Introductory letter,
- Notice of visit pads (CAPI).

Introductory letter

A copy of the introductory letter used for the survey is presented in Appendix F.

No contact letter

A copy of the no contact letter used for the survey is presented in Appendix F.

Refusal letter

A copy of the refusal letter used for the survey is presented in Appendix F.

Brochure

You will be provided with extra copies of the brochures by your regional office.

Questions and answers

The following table provides examples of basic questions that may be asked by the respondent and the answers the interviewer should provide.

A full list of questions and answers can be found in Appendix F.

Question	Answer
What is the Canadian Community Health Survey?	The Canadian Community Health Survey is a **national health survey** that asks a representative sample of people living in Canada about their health and wellbeing, factors that affect their health and their use of health care services. This survey will provide health information to local community health planners to evaluate existing health promotion programs, and to plan future ones.
What are the objectives of this survey?	To support local health units by providing them with timely information they need to evaluate existing policies and programs and to design new ones suited to their communities. To provide more current, detailed and uniform health information in every province.
What kinds of questions will you ask?	The survey asks a variety of questions about: • the current state of your health • behaviors thought to be related to your health • your use of health care services • socio-demographic information concerning your household
Who uses this information? How will it be used?	• **Provincial health ministries and federal government departments** will use this information when making funding decisions, developing priorities and identifying areas of concern for legislation, new policies and programs. • **Researchers** will have more health information to work with than ever before, in order to monitor, analyze and measure those factors affecting health and to report on them. • **Local health authorities, community groups and social agencies** need this type of information to determine how to best allocate resources in order to improve the health of the people living in their community. A key use for the information from this survey will be the development of public education campaigns primarily aimed at health promotion and disease prevention. • **The media** will use this information to raise awareness about health, and to report on trends in the health status of Canadians.

Refusal based on recent survey participation

A respondent may decline to be interviewed on the basis that they have recently completed a survey for Statistics Canada. In such situations it is important for interviewers to probe in order to determine whether it was in fact a survey conducted by Statistics Canada, the timeframe when the survey was completed, whether it was a personal or telephone interview and whether the survey was the CCHS.

Recent participation in other Statistics Canada surveys

Participation in other Statistics Canada surveys does not disqualify a person or household from being interviewed for the CCHS. This is true even when the survey recently completed was also a health-related survey. It is critically important that interviewers communicate this to reluctant respondents.

If...	Then...	And...
… the respondent refuses to participate on the basis that they recently participated in CCHS (or another survey) either in person or by telephone;	…gather as much information as possible from the respondent;	…ask your supervisor to send a verification request to SMA via JIRA.
… SMA and methodology can confirm that the respondent recently participated in CCHS;	…SMA will send instruction to code the case 63;	…your supervisor will final code 63 – Already interviewed for this survey.
… SMA and methodology confirm that the respondent has participated in another StatCan survey;	…SMA will send instructions to code the case 66;	...your supervisor will final code 66 – Sample Overlap.
… the case is not verified by SMA as a case of recent participation;	…interviewer will be asked to make every attempt at refusal conversion, with an emphasis on the random selection of households and/or the importance of the valuable health information being collected;	…if respondent refuses, the case must be final coded as a refusal (code 80) with the reason for refusal (at OC_ N04) as 9 – “Recently completed a survey (doesn’t want to again)”.

2. Collection application and procedures

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Overview

Introduction

This chapter presents:

- the different features of the collection application, and
- the procedures of the survey.

Collection application

Introduction

This section presents the different features of the collection application.

In scope / out of scope

Hutterite colonies are considered in-scope for CCHS.

However, there are some challenges when trying to conduct interviews over the phone:

- While some households do have their own direct lines, frequently the phone number that is available is a main number for the colony switchboard.
- Make sure to get contact with someone living at the address selected and collect data only for this address.
- There could be a 10 minute time limit per phone call. The call ends without warning.

Proxy interview

A proxy interview is one conducted with **another knowledgeable member of the household** or in **very rare circumstances, with a knowledgeable non-household member** on behalf of the selected respondent.

Every effort must be made to arrange an interview with the selected respondent.

Proxy interviews are to occur **only** if the **mental or physical health of the selected** member makes it **impossible** to complete the interview during the collection period.

If the proxy interview is conducted with a non-household member, interviewers will be required to add a **Remark (F4)** to explain how the non-household member relates to the respondent and the reason why the survey is conducted with a non-household member.

An example of a non-household member could be a child or a family member who is taking care of the selected respondent but do not live in the same household.

Partial interview

The partial cut-off point for the CCHS is at the first question in the **Primary Health Care** module (**PHC_Q005**).

Data sharing and linkage

Statistics Canada has entered into data sharing agreements, under section 12 of the Statistics Act, to share the survey information with:

- the provincial and territorial health ministries,
- Health Canada and,
- the Public Health Agency of Canada.

The information the respondents provide can only be shared with the consent of the respondent. The data sharing questions are asked at the end of the interview in the ADM module. ADM_R025 applies to all provinces and territories except Quebec, and ADM_R030 applies to Quebec only. The data sharing question is also asked in the Exit module (PS_) if the case is exited prior to completion and after the partial cut-off point at the first question in the Primary Health Care module (PHC_Q005).

If the respondent chooses to share his/her answers with the provincial/territorial health ministries, Health Canada and the Public Health Agency of Canada, these organizations are required to protect the information and to keep it confidential. As a result of signing the data sharing agreement, these organizations have the obligations as Statistics Canada to keep the information confidential.

The agreements with these organizations **prohibit them from disclosing any identifiable information** about respondents based on information collected by Statistics Canada. They can only release aggregate data from the information they receive from Statistics Canada.

Statistics Canada releases only non-identifiable statistical information through which the **identity of individuals cannot be revealed**.

The purpose of data sharing is to:

- allow those responsible for health care services (provincial and territorial ministries of health) to work directly with survey information;
- reduce respondent burden by collecting data only once;
- improve the quality of the survey results; and
- reduce the cost of data collection.

Provinces/territories use the shared information to ‘link’ or connect their share file with other files they already have, such as records of hospital visits or use of specific services.

We inform respondents of this linkage by reading the linkage statement

ADM_Q005: Statistics Canada, **the provinces and territories may combine your responses with information from other survey or administrative sources. This may include information on past and continuing use of health services such as visits to hospitals, clinics and doctor’s offices**.

Respondents can refuse to have their information linked. After being informed of the linkage, if the respondent does not object to the data linkage, Enter 1 and continue with the survey. If the respondent strongly objects to the data linkage, Enter 2 or 3, and no linkage will take place.

Data sharing and linkage (continued)

What is "data linkage"?

Data linkage involves combining their questionnaire answers to provincial records of the respondent's use of health care services, such as number of doctor visits or admissions to hospital. This does not include private medical records held by doctors.

Data linkage is intended to make the questionnaire answers more useful and more reliable.

Why do we do data linkage?

It is important to link health status and indicators to the actual cost of providing health care. Combined with other information from the survey, it will help answer questions such as:

- Do the people most in need receive the health services they require?
- Does visiting a doctor regularly or receiving regular physical exams help people remain healthy?
- Does the stress at work and at home affect the number of times people visit a doctor?
- Is there continuity of care? Is this related to improved health?

Who does data linkage?

Linking is done by the provincial ministries of health in a confidential, secure environment. Only information collected during the survey can be linked to these other files.

When is linking done / How long does it last?

There is no time limit on data linkage - it can be done at any time after the survey is collected. This does not mean an on-going database is being kept; linkage requests must answer a specific research question at a specific point in time.

For example, in 2006 a researcher may want to look at cancer rates among people who reported smoking 5 years earlier. A one-time linkage could be done of current (2006) cancer patients against the 2000/2001 CCHS data to examine any connections.

The next time a similar question is researched (i.e. 5 years later), another separate linkage request would have to be submitted for approval by the province.

How do I know the information will be kept confidential?

Provincial ministries of health are bound by oath under the same agreement as Statistics Canada to maintain confidentiality. No information that could identify an individual remains on the linked files.

In addition, provinces have their own strict requirements regarding linkages. The information requested must be used for research purposes only and the linkage agreed to by the province. In most cases, only one person has access to the linkage files.

What does this mean?

A **share** file containing the information about all respondents who agree to data sharing is given to the ministries of health.

The **share** file does not contain identifying information such as the name, address or health card number of the respondent.

If the respondent agrees to **data sharing**, Statistics Canada may supply this identifying information to provincial and territorial ministries of health.

Provinces/territories use this information to 'link' or connect their share file with other files they already have, such as records of hospital visits or use of specific services. This results in a **linked file** which can be used to examine some research questions that cannot be answered using the share file by itself.

Identifying information is given to provinces/territories only on request and only in connection with a specific research project. It is supplied on a physically separate file which must be destroyed as soon as the linked file has been created.

Remember:

- The **linked** file contains information for those respondents who agree to share and link their information. **No identifiable information** such as name, address, date of birth or health card information remains on the file once the linkage is complete, i.e. this information is destroyed once the linked file is created.
- This linked file does not contain any information that could identify the respondent personally and is only used within the provincial/territorial ministry of health.
- The **provincial and territorial ministries of health**, like Statistics Canada, are **prohibited from disclosing any information which may be used to** directly or indirectly **identify** a person.
- The provincial and territorial ministries of health and Statistics Canada are **required to protect the information and to keep it confidential.**

Example of studies using a linked file

- A ministry of health links survey information on physical activity with information on doctor billings to determine whether costs to the health care system are lower for people who are active.
- A tele-health program collects the health insurance number of all callers and links them to CCHS data. The information is used to study the socio-economic profile (age, sex, income, immigration status) and health profile (self-perceived, number of chronic conditions) of their callers.
- A provincial ministry of health links CCHS data on chronic conditions to information from a seniors' medication insurance program to find out which types of medications are most commonly prescribed for conditions like arthritis or angina.

Informing the respondent

If a respondent seems hesitant to give out this information, inform them that:

- The information will be kept confidential
- It will be used for statistical purposes only
- It will be given only to the provincial health ministries who are under obligation to respect the same confidentiality requirements as Statistics Canada.

Be prepared to answer any questions they may have.

Outcome codes

It is important to code each case accurately. A complete list of CCHS outcome codes and definitions is provided in Appendix C of this manual.

Interviewers should familiarize themselves with all outcome codes especially the following:

Outcome Codes 02, 03 and 05

- 02 - Incorrect phone number
- 03 - Phone number not in service
- 05 - Recorded message service

CATI interviewers must use codes 2, 3 and 5 as final codes to enable the application to use all available telephone numbers before the case is finalized. A message will pop up informing interviewers when other telephone numbers are available. Once all numbers are used up and no contact has been made, a case will be automatically coded 68 for transfer to CAPI.

Outcome Codes 04, 11, 12, 13, 14 and 15

Cases with the following codes may be considered for transfer to CAPI at the end of Phase 2.

- 04 - Fast busy signal / strange noise / dead silence / fax machine
- 11 - No one home / no answer
- 12 - Regular busy signal
- 13 - Answering machine or service – no message left
- 14 - Answering machine or service – message left
- 15 - Call screened / blocked / forwarded

Again we would like to stress for the accurate use of the above codes and to be able to make clear distinctions between outcome code 5 (recorded message service) and outcome codes 12, 13 and 14.

Outcome Code 27

- 27 - Contact with non-household member (CATI and CAPI)

Outcome Code 29

- 29 - Personal interviews requested / required (CATI)

Outcome code 29 is to be used only when a personal interview is requested by a respondent. Personal interviews should not be offered. Cases coded 29 will be verified and manually coded 68 by a supervisor.

Outcome Codes 30 , 36 and 38

- 30 - Tracing / research required
- 36 - Unable to trace/research
- 38 - Tracing/research source appointment

Outcome codes 30, 36, and 38 should be used mainly for the CATI 12-17 group. Tracing is not permitted for the CATI & CAPI 18 + group, but some research may be necessary.

Outcome Code 61

- 61 - Not eligible due to age is used for CCTB only

This is an automatic code used to ensure that the age limit is respected for each survey frame.

Outcome Codes 63 and 66

- 63 - Already interviewed for this survey
- 66 - Sample overlap

Outcome codes 63 and 66 should be approved by SMA.

Ask your supervisor to submit a JIRA for SMA approval.

Outcome Code 68

- 68 - Transferred to other collection mode

This code applies to CATI only; it is used to move cases from CATI to CAPI. Cases final coded 2, 3 and 5 will be automatically coded 68 for transfer to CAPI. Cases final coded 29 and cases identified by SMA for transfer to CAPI will be manually coded 68 by your supervisors.

Note: No CAPI to CATI transfers.

Collection procedures

Introduction

This section presents the procedures to follow in specific situations.

Roster

The Contact / Demographics section is the first part of the questionnaire, also referred to as “Entry”.

The purpose of this section is to collect information on the household, including a **roster** of household members, their ages, relationships and other demographic information.

This information may be obtained from any adult member of the household who is 18 years of age or older. From the collected information, the application automatically selects a specific respondent 18 + to complete the remainder of the questionnaire.

The names of respondents 12-17 years old are selected before contact is made.

Contact/ demographic component (Entry)

The Contact/Demographic component is the first section of the questionnaire to be completed. It consists of contact information, household composition, demographics (for each member listed), housing information and the language of the interview. It is also known as Entry.

For 2016 there are 2 Entry flows for CCHS CATI, but only one entry flow for CCHS CAPI. One Entry is used for adult respondents, aged 18 years or older and the other for child respondents aged 12 – 17 years old.

If a case is set to select an adult respondent, the Entry component can be answered by any knowledgeable adult household member. The selection of a respondent takes place automatically after this section has been completed. The selected respondent will then be asked to complete the rest of the questionnaire.

If a case has a pre-selected child respondent, the Entry component should be answered by the listed parental guardian or another knowledgeable adult. If a selected 17 year old child is living alone with no other adults in the household he or she can be asked to complete the Entry component. Rostering is still required even though a respondent has been pre-selected.

Afterwards the child will be asked to complete the rest of the questionnaire provided parental consent is given.

Dwellings

To ensure accurate coverage within the sample, it is important to understand the concepts of a dwelling, a household and the usual place of residence. It is also important to establish which dwellings are eligible and which are ineligible.

The tables on the following pages outline the eligible and ineligible dwellings for this survey.

Definition of a dwelling

A dwelling is a set of living quarters, in which a person or a group of people reside or could reside (i.e., private home, apartment, duplex, etc. – and in very rare instances institutions or hospitals).

Eligible dwellings

Eligible dwellings in this survey are mainly dwellings where a person or a group of people reside and that are their usual place of residence.

TYPE OF DWELLING	ELIGIBLE DWELLINGS
Private Home	A private home is any dwelling that is the usual place of residence of at least one of the persons living there. The usual place of residence is considered to be the dwelling in which a person usually lives. The person may be: • A family member • A roomer • A boarder • An employee Examples: • House (single, double, row) • Duplex or triplex • Apartment • Mobile home
Private Home AND Business	A private home and business is any dwelling that serves both as a business and the usual place of residence, such as a business operating out of a home. The phone may be answered using the business name.
Non-Institutional Collectives	A dwelling that is communal in nature or a private dwelling that is determined to be occupied by 10 or more unrelated people (if fewer than 10 people reside there, it is classified as a private home) and has to have <u>at least one person that consider it as their usual place of residence</u>. Examples: • Lodging or rooming house • Motel, hotel or tourist home • Staff residence, work camp, mission, group home or YM/YWCA

TYPE OF DWELLING	ELIGIBLE DWELLINGS
Institutional Collectives	A dwelling that is institutional may be, under certain conditions, considered in scope. Examples: • Facility for the disabled • Nursing Care home • Establishment for care of children and minors • Young offenders facility *If there are owners, employees or spouses of patients/residents living in these Institutions and consider it as their usual place of residence, the dwelling would be in scope. Other examples: • Senior citizen's residence • Religious establishment (e.g., monastery, convent) *If these institutions offer little to no minimal care to the residents the dwelling would be in scope.
Secondary/ Seasonal dwellings	A private home where the dwelling is the usual place of residence for any persons residing there at the time of the survey. Examples: • Cottage or vacation home • Trailer • Secondary dwellings (for example a secondary dwelling for work or attending school) *A person should be living at the dwelling for the majority of the year for it to be considered in scope.

Ineligible dwellings

Ineligible dwellings in this survey are mainly dwellings that are not the usual place of residence for respondents, or dwellings that are institutions.

TYPE OF DWELLING	INELIGIBLE DWELLINGS
Secondary/ Seasonal dwellings	A private home where the dwelling is not the usual place of residence for any persons residing there at the time of the survey. Examples: • Cottage or vacation home • Trailer • Secondary dwellings (for example a secondary dwelling for work or attending school)
Business Only	The dwelling serves as a business only and there is no person that considers the dwelling their usual place of residence.
Institutional Collectives	A dwelling that is institutional is ineligible and is considered out of scope. Examples: • Hospitals • Long-term care facilities • Homes for seniors and religious establishments with **moderate - full care** • Facilities for the disabled, for the care of children or minors, for young offenders, and nursing homes with only residents living there (no spouses, owners, or employees living there) • Treatment centers • Penal institutions

Introduction

In order to obtain proper coverage and avoid duplication, it is important to understand which members in a household must be listed in the household roster and which should not. Below is an explanation of which potential household members should be included and excluded and which included in the roster. In some cases, a specific member of a household may not be in-scope for the survey (such as children under the age of 12, full time Canadian Armed Forces (CAF) members, etc.) but it is very important to include them when the household is rostered. The application takes this into account when selecting a respondent.

Definition of a household

A household is any person or group of persons living in a dwelling. It may consist of :

- one person living alone,
- a family sharing the same dwelling,
- a group of people who are not related but share the same dwelling.

Usual place of residence

The usual place of residence is considered to be the dwelling in which a person usually lives. The person may be:

- a family member,
- a tenant,
- a roomer,
- a boarder,
- an employee.

Household membership rules

Household membership rules are guidelines used to establish who should be included as household members in the demographic questions. Some rules identify persons who should not be included (for example, visitors from other countries).

To be included on the list for a particular dwelling, a respondent must either:

- **regard the dwelling as his/her usual place of residence**

or

- **be staying in the dwelling at the time of the survey and have no usual place of residence elsewhere.**

List **all** of the persons who consider the selected dwelling to be their usual place of residence.

PEOPLE PRESENT IN A DWELLING WHO SHOULD BE INCLUDED AND LISTED AS HOUSEHOLD MEMBERS

Landed Immigrants with a Canadian residence

Members of the Canadian Armed Forces (CAF) who are:

- part-time (reserve) members of the CAF
- civilian employees of National Defense
- living in a household with non-CAF members

Students

- Single students are included in the parental dwelling if they have spent 30 or more days there in the past 12 months; otherwise, they are included in the dwelling they occupy during the survey period.

Inmates of Institutions

- Who have been institutionalized less than six months and consider the selected dwelling to be their usual place of residence.

Staff of an Institution and their Families

- Who state the institution as their home and have no usual place of residence elsewhere
- This includes owners (and their families), employees (and their families) or spouses of patients/residents living in an Institution.

Persons with Two Residences

- Persons with a dwelling both inside and outside Canada are considered usual residents of their Canadian dwelling.
- Persons with two dwellings inside Canada, if they spend the larger part of the year in the selected dwelling.

Married Persons

- Who are currently away from their spouse's selected dwelling because of work/school.

Children of Separated or Divorced Parents

- Children who are in the joint custody of both parents where the custody is shared equally (i.e., they live an equal amount of time with each parent), should be considered as usual residents of the dwelling if they are present in that household at the time of rostering.
- Children of Separated or Divorced Parents who reside more than half their time in the selected residence.

PEOPLE PRESENT IN A DWELLING WHO SHOULD BE INCLUDED AND LISTED AS HOUSEHOLD MEMBERS, continued
Non-permanent residents • Non-permanent residents (NPRs) are persons residing in Canada who are neither Canadian citizens nor landed immigrants. They are: o persons applying for refugee status; o students from other countries attending school in Canada on student visas; o persons from other countries in Canada on work permits; o persons who have a Minister's permit to reside in Canada; and o persons applying for landed immigrant status who are sponsored by a family member who has already received this status; and o Non-Canadian born dependants of the above four categories.

PEOPLE PRESENT IN A DWELLING WHO SHOULD NOT BE INCLUDED OR LISTED AS HOUSEHOLD MEMBERS

Foreign Residents

- Representatives of foreign governments and their families.
- Visitors from other countries.

Students

- Single students spending less than 30 days in the last 12 months in a selected parental dwelling.

Members of the Canadian Armed Forces

- Full-time members of the CAF (students of any Officer Training Plan operated by the CAF are considered as regular CAF members).
- Who are within 30 days after retiring from previous full-time military service.

Note: If there are both CAF and civilian household members living in the selected dwelling, demographic information on all household members must be collected.

Persons with two residences

- Persons with two dwellings inside Canada, if the larger part of the year is not spent at the selected dwelling.

Inmates of Institutions

- Who have been institutionalized for six months or longer, e.g., in a prison or hospital.

Married Persons

- Who are staying in a selected dwelling other than spouse's because of work/school.

Children of Separated or Divorced Parents

- Children who are in the joint custody of both parents where the custody is shared equally (i.e., they live an equal amount of time with each parent), should not be considered as usual residents of the dwelling if they are not present in that household at the time of rostering.
- Children of Separated or Divorced Parents who reside less than half their time in the selected residence.

Privacy and Parental Concerns

Asking sensitive questions of **12 to 17 year olds** may be cause for concern with some parents. The following steps are designed to address their concerns.

Steps to follow

For selected respondents **aged 12 to 15,** interviewers must:

1) **Obtain verbal permission from parent/guardian to interview with the selected youth**. A new module has been added to the Entry block. When the selected respondent is under 16 years of age, verbal consent from the parent or guardian must be obtained. If the parent or guardian is unavailable to provide consent, press <F10> to exit the application and assign an outcome code of 23, 24, or 25 as appropriate.
2) **Respect privacy**. If privacy cannot be obtained to interview the selected youth over the phone (i.e. another person listening in) the interview must be coded as a refusal with a permanent note stating that confidentiality could not be respected.
3) **If a parent/guardian is hesitant about giving consent to share and link**, stress the importance of the survey and how the data will be used to help children.
4) **If parents ask to know more about the type of questions asked in the survey**, the interviewer can direct them to the topics listed in the note to parents. If a parent/guardian asks to see the actual questions a copy of the questions will be made available from the Regional Office. You must provide them with a copy as soon as possible.
5) If the child is less than 14 we will be asking the parents' permission to **share** information collected and if the child is under 18, we will ask the parents' permission to **link** the information collected.
6) **Some questions must be answered by the parent/guardian**. The "PMK" (Person Most Knowledgeable) module is asked of respondents under 18 years of age. For respondents 12-17 chosen using the CCTB frame, the survey will ask to speak with the PMK in order to complete certain modules before ending the survey. In rare cases when the household does not contain anyone over the age of 17, the PMK module is skipped and it is assumed that the selected respondent can answer the household questions.

Critical Notes

A critical note is used to identify specific demographic errors which affect data quality. Critical notes begin with three asterisks (***) and are entered in the permanent notes.

The only acceptable uses of critical notes are changes to the selected respondent's:

- Name
- Gender
- Age
- Date of birth
- Relationship to other household members

To modify the health number information, address, postal code, or telephone number (including cell phone) does not constitute a critical note.

These corrections can be entered in the permanent notes without the preceding three asterisks.

It is important for interviewers to make use of the critical notes only when necessary; this helps to isolate the crucial demographic errors in the data, which are later updated in processing.

Transferring cases from CATI to CAPI

During each phase, cases will be transferred from CATI to CAPI.

Outcome code 68 will be used to transfer cases from CATI to CAPI. Code 68 applies **only** to CATI 18+ (from Area frame).

There will be no transfers from the CCTB frame, and no transfers from CAPI to CATI.

CATI 18+ cases final coded 2, 3, 5 will be ***automatically** final coded 68 for transfer to CAPI. Cases coded 29 will be verified and manually coded 68 by your supervisor. Cases coded 68 become voids in CATI.

Some cases final coded 04, 11, 12, or 15 will be approved for transfers by SMA.

Note: *If no additional phone numbers are available.

Moving back to telephone number 1

An update has been made to the 18+ sample that have multiple phone numbers. This was updated to take into consideration the "stop-transfer" functionality that was added in CCHS 2015.

Once the stop-transfer function has been activated (turned on), cases that have moved to telephone number 2 (TELNUM2) due to no contact from telephone number 1 (TELNUM1), the following will occur:

- If TELNUM2 is coded an outcome code of 2, 3, or 5, the application will move back to TELNUM1.
- TELNUM1 is continued to be called until the case is resolved or the collection period ends (If TELNUM1 is coded an outcome code of 2, 3 or 5, it will be sent home as a final code).

This has been programmed into the application; you will be required to simply follow the phone number provided on the screen.

Phases of collection

Phase One – 1st month of collection period:

- Collection begins for all CATI & CAPI urban and some rural;
- CATI identifies cases for tracing or transfer to CAPI;
- Cases transferred to CAPI overnight;
- CAPI receives and assign transferred cases;
- CAPI plans for collection in rural areas where collection has not started.

Note: By the end of Phase 1, each CATI case must be attempted **at least 5 times.**

Phase Two – 2nd month of collection period:

- CATI continues to identify cases for transfer to CAPI;
- CAPI in full collection (in all rural and urban areas);
- CATI to CAPI transfers continue;
- Tracing continues;
- CAPI continues to accept and assign transferred cases.

Note: By the end of Phase 2, it is recommended that each CATI case will be attempted **at least 20 times**. All CATI cases for transfer to CAPI will be identified by the end of Phase 2.

Phase three – last 4 weeks of collection period:

- Collection continues for both CATI & CAPI;
- SMA identifies cases for transfer to improve response rates in urban areas if necessary.

Note: Transfers to boost response rates must be completed by the end of week 1 in the third month of the collection period and will be decided by HO with SMA approval. Any other transfers past this deadline must be approved by SMA.

CATI - Importance of 5 and 20 attempts

Why are we asking for 5 attempts by the end of the first month and 20 attempts by the end of the second month?

The earlier in the collection period that CATI makes its required call attempts; the faster cases with a telephone number issue can be identified and transferred to CAPI interviewers. The quicker that CAPI interviewers receive the transferred cases, the more time they have to work on them and either resolve the cases as respondents or resolve them as being out-of-scope. Research from the 2015 CCHS indicates that making **5** attempts by the end of the first month, and **20** by the end of the second month result in obtaining responses/respondents.

Sensitive interviewing

The following steps should help you to maintain your comfort and your respondent's comfort when collecting data about sensitive topics.

Reading sensitive questions

Read sensitive questions clearly and confidently.

Ask any potentially embarrassing or sensitive questions in the same way that you ask the other questions. Most respondents will answer these questions easily, freely, and honestly, partly because they realize that we have important scientific reasons for asking, and partly because our interviewers treat these questions in the same unemotional and non-judgmental way that they ask all questions.

Even more important, you must be careful not to give anyone the impression you are overly curious about their responses.

Control the vocabulary. Use clinical words. If a respondent uses graphic words when answering the questions, do not adopt their words. By using clinical words like "menstrual cycles", you will eventually train the respondent to use your words. Do not laugh at or acknowledge jokes or banter in bad taste or of too personal a nature. Use silence as a statement that this sort of interaction is not part of your research purpose.

Your image should be of a serious professional doing serious research. At the same time, distinguish between a light comment made by a slightly nervous respondent and a joke that is simply in bad taste.

Emotional reactions and your role

Do not be surprised if a respondent reacts with emotion during the interview or even giggles or laughs at some of the questions. Remain professional, friendly and continue asking questions to help get their attention back on task. Offer to take a short break to let the respondent get a tissue or drink of water if he/she becomes upset or emotional. Give the respondent a few moments to compose him/herself, and then continue with the interview.

Remember that words aren't always necessary. Sometimes silence and pausing are excellent tools and are quite respectful for the respondent. If the respondent is too upset or emotional to continue, ask to reschedule an appointment to finish the interview.

Keep in mind that a survey on health uses the same techniques as other surveys. Also, as you know, the sample for this survey is from the general population. The respondents will be the same as for other surveys you are used to working on.

Do not volunteer personal comments, questions or experiences even if this is asked by the respondent. Establish a good rapport but not a friendship. Remember if the respondent is in need to talk to someone or needs more resources please refer them to the provincial health information telephone number. It may be tempting on a human level to continue the discussion especially after the interview is over but you are a professional Statistics Canada interviewer.

Item non-response

As in all surveys, respondents have the right to refuse to respond to a question at any time during the interview. We try to minimize this item non-response to maintain data quality but we must also be sensitive to it in the context of a health survey.

If a respondent becomes visibly upset, is hesitant or indicates an unwillingness to answer a specific question or group of questions due to the nature of the subject matter, use your best judgment in entering a non-response and continue to the next question(s). By being aware of the information provided in this section and during training, you should be able to distinguish between a 'typical' refusal and one brought on by emotional or some other type of distress.

Note: Interviewers should not enter item non-response based on their own initiative. Always offer the respondent the option to continue before skipping to the next question. If a question or set of questions is skipped due to emotional distress, enter a remark (F4) or permanent note indicating the reason for the refusal.

Respondent Resources

If, at any point, the respondent requests help with health issues, you can suggest they consult their doctor or other health practitioner.

You can also provide the toll-free provincial health information line (see below).

24 hour toll free health line numbers by province:

Province	Organization	Number
British Columbia	HealthLinkBC	8-1-1 7-1-1 (TTY)
Alberta	HEALTHLink Alberta	1-866-408-5465 Edmonton: 780-408-5465 Calgary: 403-943-5465
Saskatchewan	HealthLine	8-1-1 1-888-425-4444 (TTY)
Manitoba	Health Links/ Info Santé	1-888-315-9257 Winnipeg: 204-788-8200
Ontario	Telehealth Ontario	1-866-797-0000 1-866-797-0007 (TTY)
Quebec	Info-Santé	8-1-1 (non applicable à Terres-Cries-de-la-Baie-James ou Nunavik)
New-Brunswick	Tele-Care/Télé-Soins	8-1-1 1-866-213-7920 (TTY)
Prince Edward Island	N/A	N/A
Nova Scotia	Healthlink	8-1-1 7-1-1 (TTY)
Newfoundland	Healthline	1-888-709-2929 1-888-709-3555 (TTY)

In Quebec, you can also suggest contacting the nearest "Centre local de services communautaires (CLSC)”.

Introduction – CATI only

In the Canadian Community Health Survey (CCHS), the tracing component will be used to trace respondents who were not reached using the Head Office contact number(s).

Tracing will only be done under the following conditions:

1. Case must have a pre-selected child respondent from the CCTB sample frame.
2. All given phone numbers for a case must be exhausted before tracing can take place. Cases with pre-selected child respondents can have up to four phone numbers.
3. A maximum of 10 minutes should be allocated to tracing per case.

Who to trace

The following people are traced:

- Selected respondent's parent/guardian who cannot be contacted,
- Household members may be traced to find the respondent,
- The respondent who lives on his/her own.

Action when traced

The following actions will be taken with trace cases:

If…	and/but …	then…
the parent/guardian has moved within Canada	…and the selected child has moved with parent/guardian	… trace parent/guardian and interview (this includes moves to Yukon, NWT or Nunavut). Update address in Entry component.
the parent/guardian has moved outside of Canada	… the selected child has moved with parent /guardian	...do not trace. The case is out of scope for the CCHS.
the parent/guardian has not moved	…the child has moved, example college, foster case with family, adoption, living alone	...trace the child and interview. Update address in Entry component.
the parent/guardian has changed telephone number		…trace parent/guardian and interview.

Tracing should **not** be done if:

- The child has moved outside of Canada.
- The child is deceased.
- The child has been institutionalized (foster care).

Action when traced (continued)	If the selected respondent is absent for the duration of the collection period and cannot be reached by telephone you **must** assign the Final Outcome Code - 20 (Absent for duration of survey) to finalize the case. Do not do the interview with another member of the household as a Proxy.
Cases sent to Tracing	The application will automatically move a case to tracing during the overnight process if it was coded 2, 3 or 5 and has no other phone numbers. Cases can also be manually sent to Tracing by the interviewer, if outcome code 30 is selected in the Exit component (OC_N02). **Note:** a case should not be sent to tracing if a new telephone number for the respondent is obtained during contact. The new telephone number must be entered in the tracing information (TRA) block and it will be updated in the application on the next day. Once the TRA block is updated, that case should not be re-entered on that same day. Otherwise new information will be lost.
Legality of the use of publicly available data	**Telephone numbers obtained from contacts during the course of tracing do not need to be confirmed.** Address and telephone information may be used in tracing, even if it has not been obtained from a publicly available data source. For example, an address or telephone number provided or obtained for a selected respondent or a family member, may be used as a point of contact to trace the selected respondent or a family member even if the source is not publicly available. The address or telephone number may be a place of residence, of business, etc.
Confidentiality	The names of household members are displayed in the “Demographic” box at the bottom of the tracing function screen. When tracing, you may use this personal information to confirm a respondent but only in a non-disclosure manner. For example, you may NOT divulge the name of household members to a respondent or to a trace source. However, when you need to confirm that you have contacted the correct person you may ask a question such as: “What is the name of your spouse?... age of your spouse?…” etc.
IMPORTANT	**The respondent’s date of birth and the name of the survey may NOT be divulged.**

3. Questionnaire

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Questionnaire overview

Introduction

This chapter is a guide to the content of the Canadian Community Health Survey (CCHS) questionnaire. It is meant to be used as a reference tool while interviewing. It introduces new concepts, gives definitions of terms, and explains how to collect the information required. Questions that have special instructions are also included for review.

The questionnaire is automated and includes all the tools for contacting, listing, selecting respondents; completing interviews and, providing final and in-progress status codes to cases.

The features and screen design are similar to other Statistics Canada computer assisted surveys created using Blaise programming, such as the Labour Force Survey (LFS) and Survey of Labour and Income Dynamics (SLID).

Please refer to the Glossary (Appendix A) for definitions of terms and concepts used in the CCHS.

Layout

There are three main sections to the CCHS questionnaire:

1. Entry (Contact/Demographics),
2. Health and Socio-demographic modules, and
3. Exit.

Each of these sections is explained on the next pages.

Entry flow CATI

A single collection application will be installed for CATI. It will include sample from both the Area and the CCTB frames, from all provinces and territories. A different Entry is used for each sampling unit (dwelling and person). The Sample ID can be used to identify the sampling unit.

Sampling unit	Sample ID
Dwelling (area frame)	00131050070020_8001N
Person (CCTB frame)	C2016010000001_8001N

Note: the Sample ID of units from the CCTB will start with a C.

Questionnaire Entry Flow by Frame

CCTB: 12-17 (CATI only)	AREA 18+ (CATI & CAPI)
Make contact with designated parent/guardian or adult member of household;	Make contact with an adult member of the household;
- confirm that the pre-selected respondent has been reached/contacted;	- confirm that you have reached the right address;
- complete household demographics with adult member of the household;	- determine if the dwelling is in scope using the AIS, INST, COLL & DW modules;
- determine if pre-selected respondent is eligible for survey;	- complete household demographics with adult member of the household;
- ask parental permission to interview children between ages of 12-15 yrs;	- ask to speak to randomly selected respondent;
- complete relationship module(s) with respondent;	- complete relationship module(s) with respondent;
- complete interview with pre-selected respondent;	- complete interview with randomly selected respondent;
- or make an appointment to call back later.	- or make an appointment to call back later.

Contact information

In order to maximize the chance of contacting a selected person or dwelling, interviewers are provided with several forms of contact information.

- A listing address for each selected dwelling
- Names and address of parent/guardian of the pre-selected respondent in the CCTB frame
- Multiple telephone numbers including cellular phones
- Tracing for respondents selected from the CCTB frame but only after all given telephone numbers have been tried.

The contact information provided for each case is based on the sampling unit or frame type.

Dwelling (Area frame)	(Person) CCTB frame
A maximum of 2 telephone numbers including cellular phones (CATI 18+)	A maximum of 4 telephone numbers including cellular phones (CATI 12-17)
Listing address of dwelling (CATI 18+, CAPI)	Name and address of pre-selected respondent and parent/guardian
No tracing	Trace parent/guardian of pre-selected respondent

Note: Cellular phones are now in scope for CCHS. Consult your SSO manuals Chapter 2-15, Section I and Section II, for Statistics Canada's Cell Phone Policies and Guidelines – they are the same for both CAPI and CATI.

Multiple telephone numbers (CATI only)

The CATI sample from the Area frame (that covers the population aged 18+) will have a maximum of 2 telephone numbers.

The CATI sample from the CCTB frame (that covers the population aged 12-17, will have a maximum of 4 telephone numbers.

It is estimated that 65% of the sample will have only 1 telephone number.

The phone numbers will be programmed in the application and the interviewer will use the one that appears on the screen. The telephone number that appears on the screen depends on the number of telephone numbers available for a unit and the maximum number of attempts allowed by telephone number.

This table shows the maximum number of cumulative attempts based on the number of phone numbers available and the expected number of units.

Number of telephone numbers	Tel 1	Tel 1 & 2	Tel 1, 2 & 3	Tel 1, 2, 3 & 4	Expected number of units 18+	Expected number of units 12-17	Total # Expected units
1	40				49,600	7,700	57,300
2	15	40			30,400	900	31,300
3*	20	30	40			<100	<100
4*	20	30	35	40		<100	<100

*Only applicable to CCTB frame cases where available.

CAPI Telephone

Note: CAPI interviews can be conducted by telephone only after initial contact is made in person.

Introduction

This section covers the Contact / Demographics or Entry for CAPI and CATI cases from the Area frame (18 years and over). Some modules/questions will be asked for both CAPI and CATI environment. Others will be asked for either CAPI or CATI. Double asterisks are used to indicate when a module applies to **CAPI only** or **CATI only**.

An overview and the first question for each Entry module are provided below except for new/important modules where all the questions are provided.

The interviewer will be exposed to the full set of modules/questions by completing the full mock interviews during training.

Number of Multiples (MU)

MU_N01

In this module the CAPI application will ask interviewers if they would like to generate multiples.

INTERVIEWER: Do you want to generate multiples?

1. Yes
2. No

If yes, you will be prompted to enter the number of multiples to be generated.

CAPI only.

Multiples dwellings

A multiple dwelling is any unlisted dwelling discovered within a structure that has not already been listed on your assignment list.

Any time an unlisted multiple is discovered in a dwelling selected for the survey, **it must be interviewed as a separate dwelling**. In order to do this, a new case must be created, or generated, by the application.

There are two ways that multiple dwellings can be identified in the application. A multiple dwelling could be:

- Identified visually by the interviewer upon entering the dwelling or from outside (e.g. an extra electrical meter outside the house, address is A or B, obvious new apartment that is not listed, house is divided into apartments). This is accounted for by a question to the interviewer in Entry (MU-N01).
- Identified by the respondent in the main dwelling during an interview. This is covered by a question (LQ_Q01) for the respondent in the Exit component.

Note: **Multiples are for CAPI only.**

Contact (CN)

The Contact block contains the questions used each time a case is selected for contact. A header consisting of contact information for the household will be displayed at the top of the screen. For example:

Selected Respondent:	
Telephone Number	**(613) 321-1234**
Address:	**12 MINSK AVENUE, APT. 310**
	KINGSTON, ON
	K1A1A1

CN_N01

Interviewers will record the method of interview and indicate whether they have made contact.

INTERVIEWER: Record method of interview

1. Telephone
2. Personal

CN_N01 will appear for CAPI only.

CN_N02

INTERVIEWER: Have you made contact?

1. Yes
2. No

Interviewer Introduction (II)

Interviewers will introduce themselves using both given and last name.

II_R01

Hello, I'm from Statistics Canada. My name is ...

Adult respondent (AR)

This module is used to determine whether an adult member of the household is currently available.

AR_Q01

May I speak with an adult member of the household?

An adult member is any responsible person living in the selected dwelling who is at least 15 years of age. It is important that this person be sufficiently knowledgeable about the characteristics and activities of the other household members and that he/she is capable of providing the information requested.

Adult Respondent Appointment (ARA)

The Adult Respondent Appointment block contains questions used when contact cannot be established with an adult respondent.

ARA_Q01	**When would an adult member of the household be available?** **Note:** the Appointment block (APP) is called at ARA_N02.
Target Respondent (TR)	The Target Respondent block contains a question used when contact is to be established with a targeted respondent. This block is called only for telephone interviews when the name of a respondent to be contacted is available.
TR_Q01	**May I speak with ^RESPONDENT NAME?** 1. Yes, speaking to respondent 2. Yes, respondent available 3. No, respondent not available 4. No, respondent no longer a household member 5. Wrong number DK, RF **CATI only.**
Specific Respondent Appointment (SRA)	The Specific Respondent Appointment block contains questions used when contact cannot be established with a specific respondent. This block is called only when the name of a respondent to be contacted is available. If the respondent will be available at some time during the survey collection period, an appointment will be made. **Note:** the Appointment shared block is called at SRA_N02.
SRA_Q01	**I'd like to contact ^RESPONDENT. When would ^RESPONDENT be available?**
Language Preference (LP)	The Language of Preference module contains questions that elicit the respondent's preferred language. This includes collection of preferred non-official languages as well as the two official languages.
LP_Q01	**Would you prefer that I speak in English or in French?**
Appointment Re-contact (ARC)	The Appointment Re-Contact block contains the questions used when contact is re-established with a respondent who was unable to complete the survey.
ARC_R01	**One of our interviewers previously contacted your household regarding Canadian Community Health Survey. I would like to continue the interview where we left off. [As you know, all information we collect in this survey will be kept strictly confidential.]**

Refusal Re-contact (REF)	The Refusal Re-Contact block contains the questions used when contact is re-established with a household that has refused to participate in the survey.
REF_R01	**One of our interviewers previously contacted your household regarding the Canadian Community Health Survey.**
REF_R02	**Your participation is essential if the results are to be accurate. All information collected in this survey will be kept strictly confidential**
Initial Contact (IC)	This module provides a survey introduction for respondents and explains the objectives of the survey as well as confidentiality provisions.
IC_R01	**I'm contacting you about the Canadian Community Health Survey. This survey collects information related to the health and well-being of Canadians. Results will help communities as well as provincial and territorial ministries of health better understand and address issues, such as** • **access to local health care and home care services;** • **lifestyle choices affecting health; and** • **chronic health conditions.**
Note:	CATI cases with incomplete mailing addresses will receive a different introductory statement. The application will automatically provide the corresponding introduction.
IC_R01	**I'm calling regarding the Canadian Community Health Survey. This survey asks Canadians from all provinces and territories about their health, the factors that affect their health and their use of health care services. One of the main goals of the survey is to gather information to help improve health programs and services provided in your region. All information collected in this survey will be kept strictly confidential. Your information may also be used by Statistics Canada for other statistical and research purposes.** INTERVIEWER: Press <1 > to continue.
CATI Monitoring (MON)	The MON module contains a single question and is called only for CATI interviews. It is used to inform respondents that another person may listen in at some time during the interview.

MON_R01	**My supervisor may listen to this call for the purpose of quality control.** This person may be a senior interviewer, a program manager, a Head Office representative, or other authorized individual. This ensures that the survey questions are being asked correctly, so that the quality of the information collected is consistent for all interviews. **Note:** the MON_Q01 should be repeated (along with the introduction to the survey) to each new respondent and on subsequent interviews. **CATI only.**
Confirm Listing Address (CLA)	This module is used to confirm the listing address.
CLA_Q01:	**I would like to confirm your address. Is it:** **Note:** CAPI interviewers will confirm address through observation.
Check Mailing Address (CHM)	The CHM module is used in subsequent interviews when the existing mailing address from the sample file is the same as the confirmed/corrected listing address.
CHM_Q01:	**Is this also your mailing address?** **CATI only.**
Listing Address (LA)	This module is used to confirm the address for personal interviews. **CAPI only.**
Mailing Address (MA)	This module is used to confirm the mailing address for personal interviews. It allows for corrections if mailing address is not the same as the listing address. **CAPI only.**
Telephone Check (TC)	The Telephone Check block is used to confirm that the correct household was contacted.
TC_Q01	**I would like to make sure I've dialled the right number.** **Is this (000) 000 0000?** **CATI only.**

New Address (NAD)

The Address block is called from within other modules when an address is to be collected.

Address in Scope (AIS)

(18+ only)

These questions are used to determine whether or not the sampled address is out of scope for the survey and is only used for adult respondent cases from the Area frame.

The second question asks if anyone uses this address as a private home. If the respondent replies “Yes”, a Business or Seasonal/Secondary dwelling will be considered in scope.

Otherwise, the case will be considered out of scope and autocoded appropriately.

AIS_Q02

Is this address for a private home, a business, an institution or seasonal (or secondary) dwelling?

1. Private home – skips to dwelling type (DW)
2. Business – continues to AIS_Q04
3. Both home and business – skips to dwelling type (DW)
4. Institutional collectives – skips to the INST block
5. Non-institutional collectives – skips to the COLL block
6. Seasonal/Secondary dwelling – continues to AIS_Q04

DK, RF

AIS_Q04

Does anyone use this address as a private home?

1. Yes
2. No

DK, RF

Institutional Collectives (INST)

Under certain circumstances, data on institutions may have to be collected. This block contains questions for gathering the name and address of an institution. Hospitals, penal institutions/correctional facilities, jails, and any specified other institutions will always be out of scope for CCHS.

Facilities for the disabled, nursing care homes, establishments for care of children and minors, and young offenders facilities may be in scope depending on if there are owners, employees or spouses of patients/residents living in the institution and have no other usual place of residence.

Senior citizen’s residences and religious establishments may be in scope if the institution provides little to no care to residents.

Interviewer instructions will appear on screen to confirm this with the respondent in these scenarios.

INST_Q01 **What is the name of the institution?**

INST_B02 **What is the address of the institution?**

Note: This calls the Address Block (NAD) to capture address information for the institution.

INST_Q03 **What kind of institution is this?**

01.	Hospital	(Auto Code 45) OOS
02.	Facility for the disabled	(Go to INST_Q04)
03.	Nursing care home	(Auto Code 45) OOS
04.	Senior citizen's residence	(Go to INST_Q04)
05.	Religious establishment (e.g., monastery, convent)	(Go to INST_Q04)
06.	Establishment for care of children or minors	(Auto Code 45) OOS
07.	Young offenders facility	(Auto Code 45) OOS
08.	Penal institution / correctional facility	(Auto Code 45) OOS
09.	Jail	(Auto Code 45) OOS
10.	Other – specify	(Go to INST_S03)
	DK, RF	(Go to INST_END)

INST_S03 **(What kind of institution is this?)**

INTERVIEWER: Specify.

Non-institutional Collectives (COLL)

Interviewers will be asked to specify the type of non-institutional collective dwelling and the total number of people living there.

Note: all non-institutional collectives will be in scope and should not include any institutional dwelling types associated with INST.

To be counted, a person must either regard the dwelling as his or her usual residence or be staying in the dwelling and have no usual place of residence.

BLEA_N100	INTERVIEWER: Enter the name of the non-institutional collective dwelling. If necessary, ask: **(What is the name of the collective dwelling?)**
COLL_N01	Specify the type of non-institutional collective dwelling (e.g., lodging/rooming house, hotel, motel, hostel, tourist home, staff residence, working camp, etc.) If necessary, ask: **(Please specify the type of dwelling.)**
COLL_Q02	**What is the total number of people living here?** (Min: 1) (Max: 995) **Note**: If more than 25 is entered, a soft edit will be triggered for confirmation.
Dwelling Type (DW)	In the CAPI environment, interviewers will select the dwelling type. In the CATI environment, interviewers will ask the respondent what type of dwelling they live in. If an interviewer or respondent selects the options for Institution or Hotel, rooming/house or camp, the application will ask them to go back to the AIS module and select either institutional or non-institutional collectives.
DW_Q01	**What type of dwelling do you live in? Is it a:** 1. Single detached 2. Double 3. Row or terrace 4. Duplex 5. Low-rise apartment of fewer than 5 stories or a flat 6. High-rise apartment of 5 stories or more 7. Institution 8. Hotel; rooming house/hotel; camp 9. Mobile home 10. Other – specify DK, RF **CATI only.**

DW_N02

INTERVIEWER: Select the dwelling type.

1. Single detached
2. Double
3. Row or terrace
4. Duplex
5. Low-rise apartment of fewer than 5 stories or a flat
6. High-rise apartment of 5 stories or more
7. Institution
8. Hotel; rooming house/hotel; camp
9. Mobile home
10. Other – specify

DK, RF

CATI only.

Roster information

Demographic information for each household member will now be collected.

Usual Roster (USU)

USU_Q01

What are the names of all persons who usually live here?

Enter the first name, then the last name, of each usual household member. Begin with adults who have responsibility for the care or support of the family.

If rostering a collective dwelling, include members that consider the dwelling their permanent place of residence, or that are present and do not currently have a permanent place of residence.

Only twenty names can be rostered. If there are more than 20 permanent residents, randomly list the first twenty.

Note – use of fictitious names

CCHS will no longer accept fictitious names from respondents.

Obtaining respondents real first and last names should be a priority and a **maximum level of effort** should be put into obtaining them. Interviewers should **never** offer respondents an opportunity to not provide their first and/or last name. Interviewers should be asking for the respondents first and last names by reading the questionnaire **as it appears in the application.**

If the respondent is hesitant or says “no”, Interviewers should attempt to discover what the respondents’ specific concerns are and address them.

Note – use of fictitious names (continued)

If the respondent is adamant about refusing to provide their name (first and/or last), and all effort has been made to obtain it, interviewers should **not** create/enter a fictitious name, nor should interviewers ever provide it as an option to a respondent. Instead, interviewers should enter "x" in the first/last name categories, continue with the interview and make a Permanent note with three asterisks (***) to say a fictitious name has been used. **Do not enter in creative fictitious names as this can create larger problems with the data.**

For example, if there is ever a situation where the respondent wants to confirm their participation or make changes to their data, we must have the correct name to locate their data. If they cannot provide the same name that was provided during the interview, we cannot even confirm that they ever participated let alone revoke sharing consent or delete data. In addition, we also need the correct name for accurate record linkage.

Our success rate for record linkage in CCHS is going down because of fictitious names even though the technology is evolving.

RS_Q04

Are there any other persons who usually live here but are now away at school, in hospital, or somewhere else?

Age without Date of Birth (ANDB)

This block is used to collect the roster member or respondent's age without asking for date of birth.

Age is a mandatory element as it determines which questions are asked of the respondent. 'Don't Know' and 'Refuse' are not valid responses.

If the respondent cannot or will not reveal their age, estimate it yourself. If the age of another household member is not known, ask for an estimate. In the 'Notes' pane, indicate when an age has been estimated and enter the approximate age.

IMPORTANT NOTE TO INTERVIEWERS: when adding 17 and 18 year-olds to the roster be sure to **confirm the exact age**.

Care is required because it's possible that an 18 year-old child could be mistakenly reported as only 17 years old and therefore would **not** be considered eligible for the survey if a case is from the Area Frame. It is also possible that a child who is 'almost' 18 is reported as an 18 year-old when in fact they are only 17.

When adding an 18 year-old, an edit message will appear asking for confirmation that the child's birth date indicates they are 18 years-old or older.

Sex (SEX)

The Sex block is used to collect the gender of the respondent.

Marital Status without Confirmation (De-facto) (MSNC)

This block is used to collect the respondent's social marital status when any previously collected marital status is not to be confirmed.

Note: This question is only asked of household members aged 16 and over. It is important for the respondent to hear the entire list before giving a response. For same-sex partners, verify if they are married or living common-law.

MSNC_Q01

What is your marital status? Are you...?

INTERVIEWER: Read categories to respondent

1 **Married**
2 **Living common-law**
3 **Widowed**
4 **Separated**
5 **Divorced**
6 **Single, never married**
DK, RF

Educational Attainment (EHG2)

This block asks for details pertaining to education attainment for household members that are 14 years and older.

EHG2_Q01

What is the highest grade of elementary education or high school he/she has ever completed?

Canadian Armed Forces (CAF)

The Canadian Armed Forces question is used to determine whether or not the respondent is a full time member of the regular Canadian Armed Forces. This question will be asked of persons 16 – 65 years old.

CAF_Q01

Are you a full time member of the regular Canadian Forces?

1 Yes
2 No

Note: Full time members of the regular armed forces are not included in this survey. If all members of the household are members of the armed forces, the interview will end.

If some household members are CAF, everyone in the household must be listed, but only those who are not in the forces will be eligible for selection.

Sign off Block (SO)	This module signals the end of household component once you press the < 1 >, you will not be able to change the roster data. **IMPORTANT NOTE TO INTERVIEWERS**: You can make corrections by arrowing-up before pressing <1>. Once you press <1>, you can no longer make changes to the roster and characteristics of the household membership (for example, age, sex, relationships, etc.). If an error was made you must enter a **critical note** in the permanent notes. **Note:** See page 3-4 for protocol on critical notes.
Respondent selection	For this part of the interview, the system will select a respondent from the list of eligible household members.
Selected respondent introduction (SRI) **(18+ only)**	**The survey application will now randomly select a member of your household who is 18 years of age or older to answer the rest of the survey questions.** **Interviewers have no control over the selection of household member. It is made automatically in order to ensure that the survey is as accurate as possible (ensures sample is representative of the population).** **Note:** The SRI module is skipped in one-person households.
Survey Introduction (INT)	This block contains the introduction of the survey to the respondent.
INT_R02	**Your participation is essential and your responses will be kept strictly confidential.**

Audio Recording (AREC)

This question will be asked as part of the CAPI quality control program

This question is included just before the Main Activity (MAC) module for respondents who are 16 years or older. The purpose of this question is to ensure that the survey questions are being asked correctly, so that the quality of the information collected is consistent for all interviews.

AREC_Q01

Some of this interview may be recorded for the purpose of quality control. Do you agree to being recorded?

Note: The question should be asked each time the case is entered, and if there is a change of respondent (e.g. after rostering if another person is selected for the survey other than the person currently answering roster questions)

CAPI only.

End of 18+ Entry.

Questionnaire Overview - Entry 12-17

Introduction

The Contact / Demographics section is the first part of the questionnaire, also referred to as “Entry”. The Entry for 12-17 uses many of the modules /questions for the 18+, however there are several modules/block that are used specifically for 12-17. Questions in the 18+ Entry are not repeated.

Note: this section applies to CATI only.

Contact (CN)

The Contact block contains the questions used each time a case is selected for contact.

A header consisting of the name, gender, address and telephone number of the contact person, and the name age, and gender, of the pre-selected respondent number will be displayed at the top of the screen.

For example:

CN_R02

Parental Guardian: Jane Smith
Selected Respondent: Jeremy Smith, M, 13
Telephone number: (613) 321-1234

Address: 12 MINSK AVENUE, APT. 310
KINGSTON, ON
K1A1A1

INTERVIEWER: Have you made contact?

II_R01

Hello, I'm from Statistics Canada. My name is ...

CCHS Target Respondent (CTR)

To determine whether the parent or guardian of the selected respondent is available.

CTR_Q01

May I speak with …….?

1. Yes, speaking to ^ PARENT _NAME
2. Yes, ^ PARENT _NAME available
3. No, ^ PARENT _NAME not available
4. No, ^ PARENT _NAME no longer a household member
5. Wrong number

Note: For cases with child respondents chosen from the CCTB frame, there will **always** be a target respondent for first contact since they are pre-selected. However, before speaking with the child, the interviewer will first have to talk to the parent or guardian (also provided by the CCTB frame) first. This block asks to speak with this parent or guardian.

CATI 12-17 only.

Target Respondent (TR)	The Target Respondent block contains a question used when contact is to be established with a targeted respondent. The TR block is called only for telephone interviews when the name of a respondent to be contacted is available.
Specific Respondent Appointment (SRA)	The Specific Respondent Appointment (SRA) block contains questions used when contact cannot be established with a specific respondent. This block is called only when the name of a respondent to be contacted is available. If the respondent will be available at some time during the survey collection period, an appointment will be made. **Note:** The Appointment shared block is called at SRA_N02.
Telephone Check (TC)	The Telephone Check block is used to confirm that the correct household is contacted. **Note:** If the phone number is wrong, the survey will end and the application will ask the interviewer to choose an appropriate outcome code. However, cases for 12-17 year-olds may have up to 4 phone numbers for interviewers to try and make contact with the selected child. If another phone number is available, the application will let the interviewer know and ask if they want to re-start the interview with new contact information.
Language Preference (LP)	The Language of Preference module contains questions that elicit the respondent's preferred language. This includes collection of preferred non-official languages as well as the two official languages.
Adult Respondent (AR)	This module is used to determine whether an adult member of the household is currently available. Adult member: any responsible person living in the selected dwelling who is at least 15 years of age - it is important that these persons be sufficiently knowledgeable about the characteristics and activities of the other household members that they are capable of providing the information requested.
Adult Respondent Appointment (ARA)	The Adult Respondent Appointment block contains questions used when contact cannot be established with an adult respondent. **Note**: the Appointment shared block is called at ARA_N02.

Refusal Re-contact (REF)	The Refusal Re-Contact block contains the questions used when contact is re-established with a household that has refused to participate in the survey.
Appointment Re-contact (ARC)	The Appointment Re-Contact block contains the questions used when contact is re-established with a respondent who was unable to complete the survey.
Survey Introduction (IC)	This module provides a survey introduction for respondents and explains the objectives of the survey as well as confidentiality provisions. This block will be generated for respondents of all ages.
CATI Monitoring (MON)	The MON module contains a single question and is called only for CATI interviews. It provides a reason for monitoring how the interview is conducted.
Specific Respondent Confirmation (SRC)	Before the survey can proceed with the household rostering, this block is used to determine if we have made contact with the right child. Questions in this block confirm: 1. Whether the selected child lives or stays in the dwelling 2. Whether that dwelling is on an Indian reserve 3. The child's age 4. Opportunity to update the child's name or gender if necessary **CATI 12-17 only.**
SRC_R01	**^CHILD_NAME has been selected to participate in our survey and in order to confirm we have found the correct household, I need to ask you a few questions.**
SRC_Q01	**Does ^CHILD_NAME live or stay in this dwelling?** 1. Yes 2. No DK, RF **Note:** If the parent/guardian answers Yes, the interview will skip to IR_Q01, otherwise it will go to SRC_Q02.

IR_Q01

Is this dwelling on a First Nation reserve?

1 Yes
2 No
DK, RF

Note: If the parent/guardian answers "Yes", the interview will end since First Nation reserves are out of scope for CCHS and it will autocode the case appropriately. Otherwise the application will go to SRC_Q03.

SRC_Q02

Was ^CHILD_NAME's address ever:
^ADDRESS

1 Yes
2 No
DK, RF

Note: If the parent/guardian answers "Yes" the application will skip to the Reason Left (RL) module. Otherwise, the interview will end and the application will use the next phone number to gain contact. If no other phone numbers are available the case should proceed to tracing.

SRC_Q03

Our records show that ^CHILD_NAME is XX years old, can you confirm?

1 Yes
2 No
DK, RF

CATI Interviewers: Verification of 12-17 year olds

When verifying the age of 12 and 17 year-olds, be sure to confirm the exact age.

Care is required because it's possible that a 17 year-old child could be mistakenly reported as 18 years old and therefore would **not be eligible** for the survey (only children aged 12-17 are eligible). It is also possible that a child who is 'almost' 12 is reported as a 12 year-old when in fact they are only 11 and would **not be eligible** for the survey since these cases are out-of-scope.

Note: If the parent/guardian says "Yes", the application will go to the update name (UN) and gender (SEX) modules. If the parent/guardian is unable to confirm the child's age or provide the child's birth date the application will proceed to SRC_Q06A to ask whether another adult in the household can confirm the child's age.

CATI Interviewers: Children aged 11

Attention should also be paid to sample where the pre-selected child's age indicates they are 11. If this is the case, interviewers should view the child's date of birth and wait until after they are 12 to contact the household.

The sample will be divided by age so likely the child's birth date will be during that collection period if it has been sent out to the field.

SRC_Q06A

Is there another adult in your household that can provide ^CHILD_NAME's age?

If another adult can confirm the child's age and is available the interviewer should ask to speak with them and the interview will loop back to the Interviewer Introduction (II) module at the beginning of the entry. If another adult is not available at that moment, an appointment should be made to call back at a later date/time. For all other responses, the case should be coded as a refusal (code 80).

UN_N01 & UN_N02

This block gives the interviewers the opportunity to update the spelling of the respondent's first and last names, if necessary.

The original first and last name of the selected child respondent will be shown on the screen.

SEX_Q01

INTERVIEWER: Enter ^CHILD_NAME's sex.

If necessary, ask: **(Is ^CHILD_NAME male or female?)**

1. Male
2. Female

Note: If the gender entered in this question is different to that on the sample file an edit will appear to let the interviewer know that the gender has been changed.

CATI 12-17 only.

Reason Left (RL)

This block is used in determining the reason why the individual left the household. If the selected child respondent does not currently live at the dwelling but did at one point the parental guardian will be asked to give a reason as to why the child moved.

RL_Q01

Why is ^CHILD_NAME no longer in this household?

1. Change in marital status (got married/separated)
2. To follow a spouse or parent
3. Job-related reasons
4. Institutionalized for less than 6 months
5. Institutionalized for 6 months or more
6. Deceased
7. Moved to a new residence
8. Joint custody to live with other parent
9. Personal study-related reason
10. In foster care
11. Other

DK, RF

Conditions where the case will be autocoded out of scope:

- The child is institutionalized.
- The child is deceased.
- The child is in foster care.

For all other responses, the case will proceed to tracing provided that all given phone numbers for a case have already been used.

CATI 12-17 only.

Tracing (TRA)

For some surveys, information for use in tracing individuals that have moved is needed. This block contains questions that ask for an address, a telephone number and any other contact information that will help trace the selected child respondent.

CATI 12-17 only.

Tracing Options (TO)

This block is displayed in CATI if an in-progress code of 30 (tracing required) is assigned to a case. The block contains a question which asks interviewers if they would like to continue working on the case in the tracing component. If “Yes” is selected, the case will open in the CATI Tracing component. If ‘No’ is selected, the interviewer will exit the case and the case will be coded In-progress 30.

CATI 12-17 only.

Tracing Language Preference (TRLP)	The Tracing Language Preference block contains a question that elicits the trace source's preferred language. This includes collection of one of the two official languages or a non-official language. **Note:** if the language is set to “other” the case will be coded In-progress as 22. **CATI 12-17 only.**
Tracing Specific Respondent (TSR)	The Tracing Specific Respondent block is used to know if the trace source has knowledge of the specific respondent. **CATI 12-17 only.**
Roster information	Demographic information for each household member will now be collected.
Usual Roster (USU)	USU_Q01: **What are the names of all persons who usually live here?** Enter or update the **first** name, then the **last** name, of each usual household member. Begin with adults who have responsibility for the care or support of the family. **Note:** The first position on the roster will automatically be pre-filled with the first and last name of the selected child respondent for 12-17 cases. These fields cannot be changed by the interviewer. When rostering, include members that consider the dwelling their permanent place of residence, or that are present and do not currently have a permanent place of residence. Only twenty names can be rostered. If there are more than 20 permanent residents, randomly list the first twenty.
Fictitious names	**CCHS will no longer accept fictitious names from respondents.** Please refer to page 3-14 of this Interviewers Manual for a more detailed explanation on fictitious names.
Marital Status without Confirmation (MSNC)	This block is used to collect the respondent's social marital status when any previously collected marital status is not to be confirmed. **Note:** This question is only asked of household members aged 16 and over. It is important for the respondent to hear the entire list before giving a response. For same-sex partners, verify if they are married or living common-law.

Educational Attainment (EHG2)

This asks for details pertaining to education attainment for household members 14 years and older.

Canadian Armed Forces (CAF)

The Canadian Armed Forces question is used to determine whether or not the respondent is a full time member of the regular Canadian Armed Forces. This question will be asked of persons 16 – 65 years old.

Target Respondent (TR)

The Target Respondent block contains a question used when contact is to be established with a targeted respondent. This block is called only for telephone interviews when the name of a respondent to be contacted is available.

TR_Q01

May I speak with ^CHILD_RESPONDENT?

1. Yes, speaking to respondent
2. Yes, respondent available
3. No, respondent not available
4. No respondent no longer a household member
5. Wrong number

RF, DK

Parental consent (PGC)

(Respondents 12 to 15 years old)

Selected Respondent: NAME OF RESPONDENT Age: XX

INTERVIEWER: Since the selected respondent is under 16 years of age, verbal consent from the parent or guardian must be obtained. If a parent or guardian resides in the household and is currently unavailable, please press <F10> to exit the application and assign an appropriate in-progress outcome code.

INTERVIEWER: Have you obtained verbal consent from parent or guardian of selected respondent to complete the interview?

1. Yes
2. No
3. Not Applicable (No parent or guardian living in the household)

There are two coding options available for a “No” answer:

1. The parent refuses to give permission for selected 12-15 year old to complete the survey. This should be coded as a refusal.
2. The parent or guardian is not available to provide permission. This should be coded as a 23, 24, or 25.

End of Entry for 12-17 year olds.

Questionnaire Overview – Health Modules

Introduction

This section discusses how the modules and their questions are presented. The information will guide you through each module.

Health and socio-demographic modules (C2)

The Health modules are the main body of the questionnaire and contain all the health related questions to be asked. These are collected for the selected respondent only, although some modules collect information for the entire household.

Prior to exiting the application, we collect information on the respondent's socio-demographic characteristics, i.e. ethnic background, employment activities, level of income, level of education of members aged 15 years and over, as well as housing questions. We also ask for their permission to share the information with researchers and we inform them that their data can be linked to existing provincial and territorial administrative data.

Note: At no time are we able to access the respondent's personal file with their physician. Data can only be linked to existing information on the respondent's use of the health care system that is recorded every time they use their health care card.

Module Guide

The content is presented in a module-by-module format in the order that the modules will appear during a interview.

Many of the modules are asked only of a particular age group or sex. Many of the modules are not asked in proxy interviews.

Some modules are considered optional content (mentioned in brackets beside the module's name). These will only be asked in provinces or territories that selected the modules.

Please note the alphabetical list of module order can be found on page 3-34 of this manual.

Question presentation

Many of the questions are self-explanatory and need no clarification. These questions have not been included in this Interviewer's Manual and can be reviewed using training cases in the CATI or CAPI applications. Questions that do have special instructions or require additional clarification have been included in this manual for review.

There are many different types of questions that can require different types of responses. These include single response, multiple response, Yes / No, entering numbers or frequencies, or entering a specific activity or place as a word. In each case, the type of response that is required has been specified.

In order to move from one question to the next in the application, it will always be necessary to press the <Enter> key.

Question numbering

The margin to the left of each question displays the **question number**. These are descriptors containing a 3- or 4-letter module identifier and a question identifier (the specific number of each question). They relate exactly to the questions in the application. Interviewers should be familiar with this numbering system.

Refuse Don't know

Please note that DK, RF is listed in the manuals only when the option is available in the application.

Types of modules

There are 4 types of modules:

- **Core**: The content of these modules should remain unchanged for several years (planned for 8 years). They are asked of all respondents.
- **Theme**: These modules are grouped together under specific themes that will be asked every year or every two years. They are asked of all respondents.
- **Optional**: These modules can vary from province to province but all health regions within a province have the same optional content selected. Optional content modules may change every year.
- **Rapid Response:** The rapid response option is a CCHS component that allows clients to study an emerging health issue by introducing a short module to the survey. Two minutes of content is reserved for Rapid Response issues, allowing the survey the flexibility to respond to new priorities within cycles without modifying main survey content. This option functions on a cost recovery basis, and facilitates the rapid introduction of survey content and delivery of survey data for analysis by clients. Rapid response modules will be announced as they become available and will be provided as inserts to the Interviewers Manual.

Theme modules for 2016

Acronym	Module name
ALW	Alcohol - Past Week
DRM	Drug methods
FGU	Food Guide Use
HMC	Home Care
INS	Insurance Coverage
PEX	Patient Experiences
SPS	Social provisions
STS	Sources of stress
SXB	Sexual Behaviors
SUI	Suicidal Thoughts and Attempts

Core modules for 2016

Acronym	Module name
ADM	Administration
ANC1	Age of Respondent
ALC	Alcohol Consumption
CCC	Chronic Conditions
CHP	Consultation with Health Professionals
SDC	Demographics
ETS	Exposure to second-hand smoke
FGU	Food Guide use
FLU	Flu Shots
FVC	Fruit and Vegetable Consumption
GEN	General Health
HWT	Height and Weight - Self Reported
INC	Income
LBF	Labour Force
MAC	Main Activity
MEX	Maternal Experiences
PMK	Person most knowledgeable about the household situation
PAA	Physical Activities - Adults
PAY	Physical Activities- Children
PHC	Primary Health Care
GR	Proxy interview
REL	Relationship
SMK	Smoking
SPS	Social Provisions
STS	Sources of Stress

Optional modules for 2016

Acronym	Module name
ACC	Access to health care services
ADL	Activities of daily living
BPC	Blood pressure check
CCS	Colorectal cancer screening
CIH	Changes made to improve health
CP2	Contacts with health professionals – extended block
CPG	Problem gambling
DEN	Dental care
DEP	Depression
DIA	Diabetes care
DIS	Distress
DRG	Drug use
DRV	Driving and safety
EYX	Eye examinations
FDC	Food choices
FSC	Food security
HUI	Health Utility Index
INJ	Injuries
INW	Workplace injury
LOP	Loss of productivity
MAM	Mammography
MDA	Medical Doctor Attachment
MED	Medication use
MXA	Alcohol use during maternal experience
MXS	Smoking during maternal experience
NDE	Nicotine dependence
OHT	Oral health
OST	Osteoporosis
PAP	Pap smear test

Optional modules for 2016 (continued)

Acronym	Module name
PCU	Physical check-up
PMH	Positive Mental Health
PSA	Prostate cancer screening
PSC	Patient satisfaction – community-based care
SAC	Sedentary activities
SCA	Smoking cessation methods
SCH	Smoking – stages of change
SFE	Self-esteem
SLP	Sleep
SPC	Consultations with health professionals on smoking
SPI	Spirometry
SSB	Sun safety behaviours
STG	Stigma towards depression
SWL	Satisfaction with life
TAL	Tobacco products alternatives
UCN	Unmet health care needs
UPE	Use of protective equipment
WST	Work stress
WTM	Waiting times
YAC	Youth access to cigarettes

*Note: not all modules have been selected for the 2016 collection period. Please see alphabetical list on next page for list of modules not selected for 2016.

In the following tables, all modules are listed in alphabetical order with the application position numbers for reference.

List of all CCHS 2016 Modules in Alphabetical Order

Order	Acronym	Module Name	Type
71	ACC	Access to health care services	Optional
9	ADL	Activities of daily living	Optional
82	ADM	Administration information	Core
27	ALC	Alcohol use	Core
28	ALW	Alcohol use during the past week	Theme
2	ANC1	Age of respondent	Core
45	BPC	Blood pressure check	Optional
7	CCC	Chronic conditions	Core
49	CCS	Colorectal cancer screening	Optional
66	CHP	Contacts with health professionals – part 1	Core
15	CIH	Changes made to improve health	Optional
55	CMH	Consultations about mental health	Theme
67	CP2	*Contacts with health professionals – extended block	Optional
32	CPG	Problem gambling	Optional
14	DEN	Dental care	Optional
59	DEP	*Depression	Optional
51	DIA	Diabetes care	Optional
58	DIS	*Distress	Optional
31	DRG	Drug use	Optional
30	DRM	Drug use methods	Theme
42	DRV	Driving and safety	Optional
26	ETS	Exposure to second hand smoke	Core
52	EYX	Eye examinations	Optional
18	FDC	Food choices	Optional
19	FGU	Canada's Food Guide use	Theme
43	FLU	Flu shots	Core
80	FSC	Food security	Optional
17	FVC	Fruit and vegetable consumption	Core

List of all CCHS 2016 Modules in Alphabetical Order (continued)

Order	Acronym	Module Name	Type
5	GEN	General health	Core
1	GR	Proxy interview	Core
73	HMC	Home care services	Theme
8	HUI	Health Utility Index	Optional
6	HWT	Height and weight – self reported	Core
81	INC	Income	Core
10	INJ	Injuries	Optional
79	INS	Health insurance coverage	Theme
11	INW	*Workplace injury	Optional
75	LBF	Labour force	Core
76	LOP	Loss of productivity	Optional
4	MAC	Main activity	Core
48	MAM	Mammography	Optional
65	MDA	Medical Doctor Attachment	Optional –BC only
29	MED	Medication use	Optional
39	MEX	Maternal experiences	Core
41	MXA	Alcohol use during maternal experience	Optional
40	MXS	Smoking during maternal experience	Optional
24	NDE	Nicotine dependence	Optional
13	OHT	Oral health	Optional
12	OST	*Osteoporosis	Optional
33	PAA	Physical activities – adults 18 years and older	Core
46	PAP	Pap smear test	Optional
34	PAY	Physical activities for youth	Core
44	PCU	Physical check-up	Optional
70	PEX	Patient experiences	Theme
64	PHC	Primary health care	Core
54	PMH	*Positive mental health	Optional

*Modules not chosen for 2016

List of all CCHS 2016 Modules in Alphabetical Order (continued)

Order	Acronym	Module Name	Type
78	PMK	Person most knowledgeable about household situation	Core
48	PSA	Prostate cancer screening	Optional
68	PSC	Patient satisfaction – community-based care	Optional
3	REL	Relationship matrix	Core
37	SAC	Sedentary activities	Optional
22	SCA	Smoking cessation methods	Optional
21	SCH	Smoking – stages of change	Optional
77	SDC	Socio-demographic characteristics	Core
57	SFE	Self-esteem	Optional
16	SLP	Sleep	Optional
20	SMK	Smoking	Core
69	SPC	*Consultations with health professionals on smoking	Optional
50	SPI	Spirometry	Optional
63	SPS	Social provisions	Theme
36	SSB	Sun safety behaviours	Optional
53	STG	Stigma towards depression	Optional
62	STS	Sources of stress	Theme
60	SUI	Suicidal thoughts and attempts	Theme
56	SWL	Satisfaction with life	Optional
38	SXB	Sexual behaviours	Theme
23	TAL	Tobacco products alternatives	Optional
74	UCN	Unmet health care needs	Optional
35	UPE	Use of protective equipment	Optional
61	WST	Work stress	Optional
72	WTM	Waiting times	Optional
25	YAC	*Youth access to cigarettes	Optional

*Modules not chosen for 2016

Overview

The Questionnaire Overview section provides an overview for each CCHS module along with interviewer instructions and a sample of the questions that you will find in each module. For new modules or for modules considered to be complex, all questions are provided. Complex modules include modules covering sensitive subjects; modules with special instructions and modules with sub-modules like, Income (INC), Labour force (LBF) and Socio-demographic characteristic (SDC).

The health modules are presented in the order they appear in the application. **Note** that all modules are included even the ones that have not been chosen by any province or territory.

The module name and type (core, theme or optional) are indicated in the left column. To the right is an overview of each module along with interviewer instructions and the first question in that module. In some cases, the full module (all questions) has been included where modules are new, where they require special instructions or additional clarification is required

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A list of all CCHS 201601 modules is presented by type and in alphabetical order, starting on pg 3-30 of this manual.

Note: not all CCHS modules are chosen for 201601.

New modules for 2016

There is one new module for 2016, the Medical Doctor Attachment (MDA) module. This module is an **optional** module available to British Colombia only.

Rapid Response

All rapid response modules will be provided separately as an insert to your Interviewer's Manual. Please ensure when you receive a rapid response module insert, you read it thoroughly and include it in your manual for reference. If you have any questions regarding the rapid response modules, please speak with your SI.

Modules with special instructions

Primary Health Care (PHC)

Regular Health Care - This is a relatively new term. It is the person that the respondent sees the most for their regular health care. In certain remote areas, it could be a registered nurse (or regular nurse). Or it could be a specialist for respondents with certain medical conditions.

Chronic conditions (CCC)

If the respondent doesn't know about a certain condition, you can use the Glossary to explain, but it probably means they don't have it. Remind them in these cases that it has to be diagnosed.

Modules with special instructions

Income (INC)

In the Income module, the time frame for providing the respondent's revenue is based on the last calendar year. It should be easier for the respondent to remember their personal and household revenue from last calendar year because it is in sync with their most recent income tax filing. If you are interviewing closer to the end of the calendar year (eg, September to December of 2016), it might be good to remind the respondent that we are asking about their previous year's revenue (so from Jan to Dec 2015).

The INC module has two sub-modules: Total Personal Income (TPI) which is asked only when there is more than one person living in the household and where the respondent is 16 years and over. The other sub-module is Total Household Income (THI).

Drug Use (DRG) / Drug Methods (DRM)

In these modules, we are considering the use of illicit drugs. This could include prescribed drugs that are wrongfully used, either by modifying the method of taking them (e.g. crushing a pill to inject it instead of taking it orally), or by taking the drugs that were not prescribed for that person.

Administrative information (ADM)

In this module, respondents are now told that their information may be used for data linkage (we no longer ask for permission to link). However, respondents are still asked to give their permission to share their data with health partners.

Respondents are also asked to provide their provincial or territorial health card number. If the respondent cannot provide his or her health number at the time of the interview, indicate "no", and continue with the questionnaire. Do not code the case as in-progress; the case should be finalized whether a health number was obtained or not. If a respondent offers to call back and provide his or her health card number, take down this information in the permanent notes section. Any change to the health number information is not considered a critical note.

Sensitive subjects/modules (SUI, SXB, CPG, DRG)

The CCHS survey contains questions on a number of sensitive topics like suicide, sexual orientation, gambling and drug use. Be sure to familiarize yourself with the sections on Sensitive Interviewing and Privacy and Parental Concerns in Chapter 2 of this manual.

Glossary

The glossary of terms is now part of the application. The glossary can be accessed by clicking on the Glossary under the title bar, or by using the F7 key. Topics are grouped and sorted alphabetical order. Enter the first letter of the help topic in HLP_Q01 to bring up the group. The full glossary of terms is available in this manual in Appendix A.

Proxy interview (GR)
Core

This module identifies the individual who will be providing information for the selected respondent and the reason why a proxy is conducted.

A list of all household members will be displayed. It is important to select the correct person; this will determine if the interview is to be conducted by proxy.

If the person providing the information is not the selected respondent, the following questions are asked:

INTERVIEWER: Do you want to complete this component by proxy?

New since 2013, the interview by proxy could be conducted with a knowledgeable household member, or a non-household member in very rare circumstances.

If the proxy interview is conducted with a non-household member, interviewers will be required to add a Remark (F4) to explain how the non-household member relates to the respondent and the reason why the survey is conducted with a non-household member. An example of a non-household member could be a child or a family member who is taking care of the selected respondent but does not live in the same household.

INTERVIEWER: Record the reason why this component is being completed by proxy. Proxy interviews are to occur only if the mental or physical health of the selected member makes it impossible to complete the interview during the collection period. If the reason for the proxy interview is neither of these choices, press <F10> to exit the application and assign an appropriate outcome code.

Age of respondent (ANC1)
Core

The actual age of the respondent is very important. It influences which questions will be asked and is key to the analysis of the data.

It is particularly important to confirm the age of 11- or 12-year-olds, as 12 is the lower age limit to determine if the respondent is an out-of-scope to our survey. Similarly, because of the use of a different sampling frame for youth and adults, it is also important to confirm the age of 17- or 18-year-olds, as they are the age limits to determine if the respondent is an out-of-scope to the frame (and therefore, of the survey). If you notice a child aged 11 is listed on your sample, review the date of birth and ensure you make contact with the household **after** the child has turned 12 as they will not be eligible until they are in fact 12 years of age.

Relationship matrix (RNC)
Core

The Relationships without Confirmation (RNC) block within the Relationship Matrix (REL) block is used to collect the relationship of an individual to someone else in the household. For the majority of cases, only the relationship of the respondent to the other members of the household will be asked. However, in certain cases, the relationship of everyone to everyone will be required.

The block will not be called if there is only one person in the household.

List of relationships

1. Husband / Wife
2. Common-law partner
3. Father / Mother
4. Son / Daughter
5. Brother / Sister
6. Foster father / mother
7. Foster son / daughter
8. Grandfather / mother
9. Grandson / daughter
10. In-law
11. Other related
12. Unrelated

Note: This module collects more detailed information about the relationships of the household members (e.g., a child living in the household can be a child by birth, a stepchild, an adopted child or a foster child) to classify people in families. For same sex partners, confirm if they are legally married or common-law.

Main activity (MAC)
Core

This module collects information about the respondent's main activity in the last week, for respondents that are 15 years old and more.

If their main activity is not 'working or vacation from work' and they are 15 to 75 years of age, they are asked if they have worked any time in the past 12 months.

If their main activity is not 'going to school', they are asked if they are currently attending school, college, CEGEP or university.

In this module, female respondents aged 15 to 55 are asked if they are pregnant.

MAC_Q005

Last week, was your main activity working at a paid job or business, looking for paid work, going to school, caring for children, household work, retired or something else?

INTERVIEWER: Main activity means the activity on which the respondent spends most of his/her time: work, job seeking, studies, childcare, etc.

If sickness or short-term illness is reported, ask for usual main activity.

1. Working at a paid job or business
2. Vacation (from paid work)
3. Looking for paid work
4. Going to school
5. Caring for children
6. Household work
7. Retired
8. Maternity/paternity leave
9. Long term illness
10. Volunteering
11. Care-giving other than for children
12. Other

DK, RF

MAC_Q010

Have you worked at a job or business at any time in the past 12 months?

1. Yes
2. No

DK, RF

MAC_Q020

Are you enrolled as ...?

INTERVIEWER: Read categories to respondent.

1. **A full-time student**
2. **A part-time student**
3. **Both full-time and part-time student**

DK, RF

MAC_Q025

To better understand the information you will provide on your health it is important to know if you are pregnant. Are you pregnant?

1. Yes
2. No

DK, RF

ESC1_Q01

Are you currently attending school, college, CEGEP or university?

INTERVIEWER: Ask respondent to include attendance only for courses that can be used as credit towards a certificate, diploma or degree.

1. Yes
2. No

DK, RF

General health (GEN)

Core

The general health module is used to collect data on self-perceived health, satisfaction with life, self-perceived mental health, self-perceived stress at work and with life in general, and sense of belonging to local community.

Researchers are interested in these topics because they are good basic measures of health status. They can also be used to predict other aspects of the respondent's health. For example, respondents who describe their health as fair or poor are more likely to have long-term health problems, to suffer from depression and to be heavy users of the health care system.

GEN_Q015

In general, would you say your mental health is...?

INTERVIEWER: Read categories to respondent.

1. **Excellent**
2. **Very good**
3. **Good**
4. **Fair**
5. **Poor**

DK, RF

Height & weight – self reported (HWT)

Core

In this module, respondents are asked to report their height and weight, and indicate whether they consider themselves overweight, underweight or just about right.

Answers to these questions are used in many ways, including calculating the number of people who are overweight, or obese.

Obesity, especially amongst young people, is a major health problem. It can be associated with a number of major medical problems, including high blood pressure, heart disease and diabetes.

HWT_R001

The next questions are about height and weight.

HWT_Q005

How tall are you without shoes on?

Chronic conditions (CCC)
Core

The first set of questions in this module deal with pain in the joints during the past 30 days. The remaining questions deal with long-term health conditions that have lasted or are expected to last at least 6 months. Chronic conditions reported by respondents must have been diagnosed by a health professional.

Answers to these questions can be used to estimate the number of people in Canada suffering from conditions such as diabetes, heart disease, and asthma.

By combining answers from this module with information from other modules, researchers can look at the relationship between chronic conditions and other characteristics, such as use of health care services or use of medications.

CCC_R001

The next question refers to your joints. Please do not include the back or neck.

CCC_Q005

During the past 30 days, you had any symptoms of pain, aching, or stiffness in or around a joint?

1. Yes
2. No

DK, RF

Health utility index (HUI)
Theme

This module contains nine sets of questions, each dealing with one of the following aspects of health: vision, hearing, speech, mobility (ability to get around), use of hands and fingers, feelings, memory, thinking, and pain.

The respondent's answers to the nine sets of questions will be combined to come up with an overall score. The higher the score, the better the respondent's overall level of health.

Researchers will use the results of these questions to compare the overall level of health of different groups, such as men and women, young and old, etc.

HUI_R001

The next set of questions asks about your day-to-day health.

You may feel that some of these questions do not apply to you, but it is important that we ask the same questions of everyone.

HUI_Q005

Are you <u>usually</u> able to see well enough to read ordinary newsprint <u>without glasses</u> or contact lenses?

1. Yes
2. No

DK, RF

Activities of daily living

(ADL)

Optional

This module looks at the impact of a physical or mental condition or health problem on common activities of daily living such as preparing meals, running errands, doing housework, handling personal care, moving about in the house and taking care of personal finances.

It is very important to ask all questions of everyone in order to compare those with activity limitations to those without.

ADL_R001

The next few questions are about common daily activities. These questions may not apply to you, but we need to ask the same questions of everyone.

ADL_Q005

Because of any physical condition, mental condition or health problem, do you have any difficulty:

with preparing meals?

1. **No, you have no difficulty**
2. **Yes, you have difficulty, but you do not require help of others**
3. **Yes, you have difficulty, but can do it with the help of others**
4. **You cannot do it at all**

DK, RF

ADL_Q005

Because of any physical condition, mental condition or health problem, do you have any difficulty:

with preparing meals?

1. **No, you have no difficulty**
2. **Yes, you have difficulty, but you do not require help of others**
3. **Yes, you have difficulty, but can do it with the help of others**
4. **You cannot do it at all**

DK, RF

Injuries

(INJ)

Optional

This module begins with a series of questions on repetitive strain injuries. These are caused by overuse or by repeating the same movement over a long period of time.

The rest of the module deals with the causes, the nature, frequency, and treatment of all other types of injuries, such as broken bones, cuts, burns or poisonings. This part of the module deals with injuries that are serious enough to limit normal activities, and all questions refer to the most serious injury suffered in the past 12 months.

To put effective injury prevention in place, health planners need to know which types and causes of injuries are most common and which groups of people are most likely to be injured.

INJ_Q005	**In the past 12 months, did you have any injuries due to repetitive strain?** 1. Yes 2. No DK, RF **Note**: module INJ must be selected for INW to be asked.
Workplace injury (INW) Optional	This module deals with injuries that occurred at a job other than the current main job. It determines the industry, occupation and the most important duties of the job when the injury occurred. This information will further the study of workplace injuries as a whole, including those that take place outside of the main job, as we already get the main job information from the module LBF. **Note**: this module is not selected for 2016.
Osteoporosis (OST) Optional	The questions in this module deal with the occurrence, cause and age of fractures or broken bones to the hip, arm, wrist, shoulder and spine. As well, there are a number of questions about the diagnosis and treatment of osteoporosis such as bone density tests, supplements and medication for osteoporosis. These questions are intended to measure the incidence of osteoporosis in the population aged 40 years and older. **Note:** this module is not selected for 2016.
Oral health (OHT) Optional	This module asks the respondent to describe the health of their mouth and teeth/ dentures and their satisfaction with their appearance, to identify any discomfort with eating, how often they experience pain in their mouth or if they avoid eating particular foods because of this, how often they've experienced any other persistent or on-going pain in their mouth in the past 12 months and what is the current state of their teeth and mouth, including bleeding gums, dry mouth and bad breath. The health of a person's teeth and mouth is related to many other aspects of their health. For example, poor teeth and poor nutrition or poor oral health and social problems are related to each other. Data from this module could be used to identify groups with the greatest need for improved dental health services.

OHT_R001

Now, some questions about the health of your mouth, including your teeth or dentures, tongue, gums, lips and jaw joints.

OHT_R005

In general, would you say the health of your mouth is...?

INTERVIEWER: Read categories to respondent.

1. **Excellent**
2. **Very good**
3. **Good**
4. **Fair**
5. **Poor**

DK, RF

Dental care (DEN)

Optional

This module asks respondents about the personal care of their teeth/dentures and the frequency of this care, including brushing or cleaning and flossing, how often they see a dental professional and when was their last visit, if the cost is a deterrent to seeing a dental professional, and if they have a plan that covers any of the costs.

Healthy teeth and gums are an important part of good overall health.

Answers to these questions can identify which groups are less likely to see a dentist.

DEN_R001

The next questions are about dental care.

DEN_R005

Do you have at least one of your own teeth?

1. Yes
2. No

DK, RF

Changes made to improve health (CIH)

Optional

Respondents are asked a number of questions related to changes in behaviour that they have made in the past year (and intentions for the future) to improve their health, including losing weight, stopping smoking and increasing exercise.

The module also asks about things that have stopped the respondent from making a change, including lack of time and lack of will power.

The results from this module are very valuable in designing programs to promote healthy living. For example, local health region planners might find that a large number of people want to do more exercise but find that lack of time is stopping them from doing so. Programs might be put in place to remind people of ways to include exercise in their everyday activities.

CIH_R001	**Now, some questions about changes made to improve health.**
CIH_Q005	**In the past 12 months, did you do anything to improve your health? (For example, lost weight, quit smoking, increased exercise).** 1. Yes 2. No DK, RF
Sleep (SLP) Optional	In this module, respondents are asked a number of questions related to sleep, including number of hours usually spent sleeping, difficulty in going to sleep or staying asleep, how often sleep is refreshing, and difficulty in staying awake. These questions will be used to examine the links between sleep and other factors, such as poor health and stress.
SLP_R001	**Now a few questions about sleep.**
SLP_Q005	**How long do you usually spend sleeping each night?**
Fruit and vegetable consumption (FVC) Core	The health benefits of vegetable and fruit consumption are popular topics in nutrition promotion programs and research. Among the research findings, there is thought to be a link between increased consumption of vegetables and fruit and a reduction in the risk of many cancers. **In these questions, you may have to probe respondents to remember how often each of the named foods are eaten. Have patience, and encourage the respondent to always give their best estimate.**
FVC_R001	**These next questions are about the fruits and vegetables you ate or drank during the past month. Please report your consumption either per day, per week or per month. Think about all meals and snacks, at home and away from home.**
FVC_Q005	**In the last month, how many times per day, per week or per month did you drink 100% PURE fruit juices, such as pure orange juice, apple juice or pure juice blends? Do not include fruit-flavored drinks with added sugar or fruit punch.** INTERVIEWER: Enter amount only. DO NOT include fruit drinks like reconstituted powdered drink, lemonade, cranberry cocktail or yogurt drinks.

FVC_N005	INTERVIEWER: Select the reporting period that corresponds to FVC_Q005. 1. Per day 2. Per week 3. Per month
Food choices (FDC) Optional	This module examines a range of reasons that people might have for choosing or avoiding certain foods. Reasons include concern about body weight or heart disease and concern about the fat or salt content of food. Information from this module can be used to develop programs to encourage healthy eating. This can improve overall health and may, in the long run, help reduce the cost of the health care system.
FDC_R001	**Now, some questions about the foods you eat.**
FDC_Q005	**Do you choose certain foods or avoid others:** **because you are concerned about your body weight?** 1. Yes 2. No DK, RF
Canada's Food Guide use (FGU) Optional	This module collects information on the population's knowledge and use of the Canada Food Guide. Specifically, this module asks if the respondent is aware of the Canada Food Guide, and whether the information in the guide was ever used. This information can be used to better understand the link between the use of Canada's Food Guide and eating habits and health behaviours.
FGU_Q005	**Have you ever seen or heard of Canada's Food Guide?** 1. Yes 2. No DK, RF
FGU_Q010	**Have you ever used information from Canada's Food Guide?**

FGU_Q015

What did you use the information for?

1. To choose foods for household members
2. To determine how much household members need to eat every day
3. To plan meals or to help with grocery shopping
4. To assess how well household members are eating
5. Other
6. None of the above

DK, RF

You cannot select "None of the above" and another category. Please return and correct.

Smoking (SMK)
Core

Cigarette smoking is a major cause of heart disease and lung disease. The economic cost of illnesses and deaths caused by smoking is a major concern to health care planners and administrators.

The number of people who smoke varies significantly between health regions, between men and women and among age groups.

This module includes a series of questions about current and past smoking habits.

The results of this module can be used to develop smoking prevention and cessation programs for the groups that need them most.

SMK_R001

The next questions are about cigarette smoking.

SMK_Q005

At the present time, you smoke cigarettes every day, occasionally or not at all?

1. Daily
2. Occasionally
3. Not at all

DK, RF

Smoking – stages of change (SCH)
Optional

In this module, current smokers are asked whether they intend to quit smoking, either in the next 30 days or in the next 6 months. The respondent is also asked if, and how often, they stopped smoking in the past 12 months for at least 24 hours because they were trying to reduce or quit smoking.

When health region planners develop programs to encourage people to stop smoking, it is very helpful to know which groups are more ready to quit and may need only some encouragement, and which groups need different types of programs or education.

SCH_Q005

Are you seriously considering quitting smoking within the next 6 months?

1. Yes
2. No

DK, RF

Smoking cessation methods

(SCA)

Optional

In this module, people who smoke and people who have stopped smoking within the past year are asked about their use of various aids to reduce or quit smoking, and their usefulness. These methods include nicotine patches, nicotine gum or candy and medications such as Zyban.

Information about the use of smoking cessation aids is useful in understanding the effectiveness of these different methods in reducing or stopping smoking. This can be used to design effective programs that encourage people to stop smoking.

SCA_Q005

In the past 12 months, did you try a nicotine patch to reduce or quit smoking?

1. Yes
2. No

DK, RF

SCA_Q010

How useful was that in helping you reduce or quit smoking?

INTERVIEWER: The answer should reflect the respondent's real objective (reduce or quit).

1. Very useful
2. Somewhat useful
3. Not very useful
4. Not useful at all

DK, RF

Tobacco products alternatives

(TAL)

Optional

This module deals with the use of cigars, cigarillos, electronic cigarettes, pipes, snuff, chewing tobacco, pinch, and water pipes, all of which are not included in the Smoking module.

These alternatives to cigarettes have different implications for health. The module will be used to understand which groups are most likely to use these products.

TAL_R001

Now, I would like to ask you a few questions about tobacco products, other than cigarettes.

TAL_Q005	**In the past 30 days, did you smoke any little cigars or cigarillos?** 1. Yes 2. No DK, RF
Nicotine dependence **(NDE)** **Optional**	In this module, current smokers are asked a series of questions related to how dependent they are on cigarettes. Knowing how dependent people are on cigarettes can help health region planners to develop appropriate programs to discourage smoking.
NDE_Q005	**How soon after you wake up do you smoke your first cigarette?** 1. Within 5 minutes 2. 6 – 30 minutes after waking 3. 31 – 60 minutes after waking 4. More than 60 minutes after waking DK, RF
NDE_Q010	**Do you find it difficult to refrain from smoking in places where it is forbidden?** 1. Yes 2. No DK, RF
Youth access to cigarettes **(YAC)** Optional	Smoking by teenagers is a major public health issue. Initially, we ask all current smokers where they usually get their cigarettes. Then the remaining questions are asked only of respondents aged 12-17 and deal with where and how youth obtain cigarettes. The results of this module may be used to develop regulations concerning the purchase of cigarettes by young people. **Note:** this module is not selected for 2016.
Exposure to second hand smoke **(ETS)** Core	Exposure to second-hand smoke is known to have a variety of negative effects on health, including lung disease and childhood asthma. This module includes questions about the number of people who smoke regularly inside the home (including household members and regular visitors), and questions about smoking restrictions in the home. These questions are asked of all respondents. Respondents who do not smoke are asked how often they have been exposed to second-hand smoke in vehicles, the workplace, school and in public places in the past month.

ETS_R001

The next questions are about exposure to second-hand smoke.

ETS_Q005

Including both household members and regular visitors, does anyone smoke inside your home, every day or almost every day?

INTERVIEWER: Include cigarettes, cigars and pipes. Smoking inside the home excludes smoking inside the garage, whether attached or detached.

1. Yes
2. No

DK, RF

Alcohol use

(ALC)

Core

Consumption of alcohol has a number of implications for health. Excessive drinking is related to a number of diseases and social and mental health problems. Drinking is also a leading cause of accidents and injuries.

This module includes questions about how often and how much alcohol the respondent has drunk in the past 12 months, and if they have ever had a drink in their life.

This module will help researchers and health planners to understand patterns of alcohol consumption. For example, which groups are more likely to binge drink, i.e., drink 4 (or 5 in the case of males) or more drinks on one occasion.

ALC_R001

Now, some questions about your alcohol consumption.

A 'drink' refers to:

- **a bottle or small can of beer, cider or cooler with 5% alcohol content, or a small draft;**
- **a glass of wine with 12% alcohol content;**
- **a glass or cocktail containing 1½ oz. of a spirit with 40% alcohol content.**

ALC_Q005

Have you ever had a drink in your lifetime?

1. Yes
2. No

DK, RF

Alcohol use during the past week

(ALW)

Theme

This module asks those who had a drink in the past 12 months about their drinking consumption on each day of the past week.

This module will be used to see how the population abides by the Low Risk Drinking Guidelines.

ALW_Q005

Thinking back over the past week, that is, from last ^DAYLASTWEEK to yesterday, did you have a drink of beer, wine, liquor or any other alcoholic beverage?

1. Yes
2. No

DK, RF

Medication use (MED)
Optional

This module deals with the use of prescription and non-prescription medications in the past 12 months, including different pain relievers, stimulants and sedatives.
The results of this module will be used to create a picture of which medications are taken by various groups. Researchers will use the results to examine the relationship between health problems, use of medications, and use of other health care services.

MED_R001

The next series of questions are about various medications.

The first series of questions are about your use of various <u>pain relievers</u>. By pain relievers, we mean products that contain opioids such as codeine or morphine, or related drugs. Most of these products require a prescription, although some codeine products are available without a prescription, for example, Tylenol #1 or 222s.

We are <u>not</u> interested in pain relievers such as Aspirin, Advil, regular Tylenol, Celebrex, etc.

<u>INTERVIEWER</u>: Tylenol Regular and Extra-strength do not contain codeine and are not counted. Tylenol 1, 2, 3, and 4 contain codeine and are counted.

MED_Q005

During the past 12 months, have you used any <u>codeine</u> products like Tylenol #3, Tylenol #1, 292s or 222s?

1. Yes
2. No

DK, RF

Drug use methods

(DRM)

Theme

For those respondents who have used illicit drugs, this module deals with the various methods that are used when taking illicit drugs. Methods for illicit drug use can include injection, smoking, inhalation, or oral use.

This module will be used to study risky drug method behaviours.

DRM_R001

The next questions are about various methods that are used when people use illicit drugs.

DRM_Q005

Have you ever smoked illicit drugs?

1. Yes
2. No

DK, RF

Drug use

(DRG)

Optional

This module asks about the use of a number of drugs used and whether they were used in the past 12 months, including marijuana (for both medical and illicit purposes), cocaine, amphetamines, ecstasy, hallucinogens, solvents and non-prescription needle injections.

The answers to these questions will be used to identify which groups could benefit from treatment programs.

DRG_R001

I am now going to ask questions on your use of various drugs.

DRG_Q005

Have you ever used or tried marijuana or hashish?

1. **Yes, just once**
2. **Yes, more than once**
3. **No**

DK, RF

Problem gambling

(CPG)

Optional

This information will be used to get a better understanding of the extent of problem gambling, and of the frequency of different types of gambling.

Respondents are asked how often they bet or spent money on 14 different types of gambling, and how much money they spent in total on gambling in Canada over the past 12 months.

A series of questions related to attitudes and experiences of gambling are then asked. These answers are used to calculate the likelihood that the respondent has a gambling problem.

Respondents who might be problem gamblers are asked a final series of questions which are used to measure how much gambling interferes with the respondent's life.

CPG_R001

The following questions are about the various kinds of gambling activities and experiences that you have had while in Canada. Please <u>note</u> that people have different definitions of gambling.

CPG_Q005

In the past 12 months, have you bet or spent money on the following gambling activities:

On weekly lottery tickets such as Lotto 6/49 or Lotto Max?

1. Yes
2. No

DK, RF

Physical activities – adults 18 years and older

(PAA)

Core

Answers to this module are used to determine how active respondents, 18 years of age and older, have been in the last seven days.

Respondents are asked if they have spent a minimum of 10 minutes of physical activity, and on which days of the week, doing various activities, such as walking or cycling to get to places, sports or recreational activities that caused sweating or heavy breathing. They are also asked how much time in total was spent doing vigorous activities.

Higher levels of activity are widely thought to be associated with better health. This module will be used to see how the population abides by Canadian Physical Activity Guidelines for adults.

PAA_R001

The following questions are about various types of physical activities done in the last 7 days. I want you to only think of activities you did for <u>a minimum of 10 continuous minutes</u>.

PAA_Q005

In the last 7 days, that is from last (day last week) to yesterday, did you use active ways like walking or cycling <u>to get to places</u> such as work, school, the bus stop, the shopping centre or to visit friends?

1. Yes
2. No

DK, RF

Physical activities for youth

(PAY)

Core

Answers to this module are used to determine how active respondents, aged 12 to 17, have been in the last seven days.

Respondents are asked on which days of the week they have been active, and for how long, doing various activities such as walking or cycling (to get to school, work, the shopping centre or to visit friends), or participating in sports or recreational activities or other activities that caused sweating or heavy breathing. They are also asked how much time in total was spent doing vigorous activities.

Higher levels of activity are widely thought to be associated with better health. This module will be used to see how the population abides by Canadian Physical Activity Guidelines for youth.

PAY_R001

The following questions are about various types of physical activities that you have done each day in the past week.

PAY_Q005

During the last 7 days, that is from last ^DAYLASTWEEK to yesterday, did you...?

1. **Attend school**
2. **Attend a day camp**
3. **Attend paid or unpaid work**
4. None of the above

DK, RF

Use of protective equipment

(UPE)

Optional

Injuries suffered while participating in leisure activities are a major burden to both the people injured and to the health care system.

This module asks about the use of helmets, wrist protectors, elbow pads and mouth guards to protect against injuries while bicycling, in-line skating, skiing, snowboarding, skateboarding and playing hockey.

Results from this module are very useful to health region planners attempting to design programs to promote the use of protective equipment.

UPE_R001

Now a few questions about precautions you take while participating in some physical activities.

UPE_Q005

In the past 12 months have you done any bicycling?

Sun safety behaviours

(SSB)

Optional

This module asks about sunburns in the past 12 months, typical exposure to sunshine during the summer months, and the steps that people can take to reduce the risk of exposure to the sun, such as wearing a hat, seeking shade, and applying sunscreen. It also asks about the use of tanning beds and the reason for doing so.

Results of this module can be used to measure the effectiveness of public health programs which warn about the dangers of sun exposure.

SSB_R001

The next questions are about exposure to the sun and various sun behaviours.

SSB_Q005

In the past 12 months, has any part of your body been sunburnt?

1. Yes
2. No

DK, RF

Sedentary activities

(SAC)

Optional

Sedentary activity refers to time spent seated with little or no physical activity. In this module, respondents are asked how many hours in the last seven days they spent playing video or computer games (involving physical or sedentary activity), watching television, reading, using the computer, and using a phone, during leisure time only (this excludes activities that occur during class time, during transportation, or while exercising).

The answers to these questions are important in understanding overall health because too much time spent doing sedentary activities might be associated with lack of physical exercise, poor nutrition and other health problems.

SAC_R001

Now, a few additional questions about activities you do in your leisure time, that is, activities not at work nor during class time nor during transportation.

Some of these activities may appear similar, but please only report your activity once.

SAC_Q005

In the last 7 days, how much of your free time did you spend:

reading books, magazines or newspapers, including in electronic formats? Include time spent reading as part of your homework, but do not include time spent reading at work, during class time, during transportation or while exercising.

INTERVIEWER: Include books, eBook readers, magazines, newspapers and homework. Do not include reading on a computer, a tablet or Internet.

Enter time in hours to the nearest half hour.

Sexual behaviours

(SXB)

Theme

Understanding sexual behaviours is a very important part of understanding overall health. For example, unprotected sex can lead to the spread of sexually transmitted diseases which have serious long-term consequences. Some women who become pregnant unexpectedly or outside a stable relationship may not get adequate prenatal care and may not have the ability to give their baby a healthy start to their life.

This module includes an introduction which explains the reasons for asking the questions and reminds the respondent that all answers will be kept confidential. Even so, some respondents may find the questions very personal and may not be comfortable answering some or all of the questions. Don't Know and Refusal are acceptable responses for all questions in the module. **Please refer to the Sensitive Interviewing material provided in Chapter 2 of this manual.**

Sexual behaviours

(SXB) Theme – continued

This module is asked of respondents aged 15 to 64. It deals with (for vaginal or anal intercourse only): the age of the respondent and partner at the time of first sexual intercourse and at the last sexual encounter; the sexual orientation of the partner; the number of sexual partners in the past 12 months; the use of condoms and the reason for using or not using a condom.

Then there are some questions concerning other methods of contraception (besides condoms), including the emergency contraceptive pill and how often is has been used in the past 12 months (for females aged 15 to 48).

The last part of the module deals with questions related to testing for sexually transmitted infections (HIV and others).

SXB_R001 Sexual behaviours

The next questions are about sexual behaviours. Some questions may sound sensitive to you, but they are included in this survey because they will help monitor important public health issues such as risk of sexually transmitted infections and unintended pregnancies.

SXB_Q005

Have you ever had sex? Please include vaginal and anal sex.

1. Yes
2. No

DK, RF

A response inconsistent with a response to a previous question has been entered. Please confirm.

Maternal experiences

(MEX)

Core

Increasingly, health promotion programs are targeted at very young children. Researchers believe that children who have a good start in life become healthier teenagers and adults.

The focus of the next 3 modules (MEX, MXS & MSA) is the behaviour of new mothers (aged 15 to 55) of babies under 6 years old.

The questions in the first module cover topics such as folic acid and iron supplements, weight gain, breastfeeding, sleeping arrangements for the child, and the child's diet.

The results of this module can be used to identify which groups of women could benefit from educational programs that promote healthier babies.

MEX_R001

The next questions are specific to women's health.

MEX_Q005

Are you taking a vitamin supplement containing folic acid?

INTERVIEWER: If respondent says she takes a prenatal vitamin but is unsure what the vitamin contains, please select 'Yes'.

1. Yes
2. No

DK, RF

Smoking during maternal experience

(MXS)

Optional

This module explores whether women smoked cigarettes before or during their pregnancy, and whether they were exposed on a regular basis to second-hand smoke during the pregnancy.

The results of this module can be used to identify which groups of women could benefit from educational programs that promote healthier babies.

MXS_Q005

In the three months before your pregnancy with ^BABYSNAME, or before you realized you were pregnant, did you smoke cigarettes?

INTERVIEWER: Read categories to respondent.

1. **Everyday**
2. **Almost everyday**
3. **About 2 or 3 times a week**
4. **About once a week**
5. **Once or twice**
6. **Never**

DK, RF

Alcohol use during maternal experience

(MXA)

Optional

This module explores whether women drank alcohol before or during their pregnancy or while they were breastfeeding.

The results of this module can be used to identify which groups of women could benefit from educational programs that promote healthier babies.

MXA_Q005

In the three months before you were pregnant with ^BABYSNAME or before you realised you were pregnant, did you drink any alcohol?

1. Yes
2. No

DK, RF

MXA_Q005

In the three months before you were pregnant with ^BABYSNAME or before you realised you were pregnant, did you drink any alcohol?

1. Yes
2. No

DK, RF

Driving and safety (DRV)

Optional

Understanding the safety behaviours of drivers and passengers is an essential part of reducing deaths and injuries caused by road accidents and collisions.

In this module, respondents who have driven in the past 12 months are asked about a number of topics, including their use of cell phones (with and without a hands-free device) while driving in a motor vehicle other than a motorcycle.

There are also other questions, such as tiredness while driving, the frequency of driving while under the influence of an illicit substance and the frequency of driving, or being a passenger in a vehicle, after the driver has been drinking. These questions relate to any vehicle.

The module also includes questions about helmet use for passengers or drivers of an ATV or snowmobile. There are also questions about the frequency of driving ATVs or snowmobiles while under the influence of an illicit substance, and the frequency of driving, or being a passenger, after the driver has been drinking.

DRV_R001

The next questions are about the use of a motor vehicle.

DRV_Q005

In the past 12 months, excluding motorcycles and off-road vehicles, have you driven a motor vehicle such as a car, truck or a van?

1. Yes
2. No

DK, RF

Flu shots (FLU)

Core

Respondents are asked when they last had a flu shot, excluding the H1N1 shot. Respondents who have not had a shot in the past year are asked the reasons why they did not receive one.

Influenza is a serious illness for certain groups, including the elderly. For all groups, the flu is a major cause of days lost from work, school and other activities.

Results of this module will be used to identify groups of people who are less likely to have a flu shot. This information may be useful in planning and designing new programs to promote immunization.

FLU_R001	**Now a few questions about your use of various health care services.**
FLU_Q005	**Have you ever had a seasonal flu shot, excluding the H1N1 flu shot?** 1. Yes 2. No DK, RF
Physical check-up **(PCU)** Optional	In this module, the respondent is asked when they last had a physical check-up that was not related to pregnancy or a specific problem. Respondents who have not had a check-up in the past 3 years are asked to report the reasons why. They are also asked whether they had their height, weight, blood pressure and cholesterol checked and whether the physician discussed the risks related to weight. The results of this module are valuable in understanding which groups are less likely to have a physical check-up and why.
PCU_R001	**Next, I would like to ask you some questions related to general physical check-ups. Please, do not include check-ups you may have had during a visit for a specific health (problem / problem or during a pregnancy).**
PCU_Q005	**Have you ever had a general physical check-up?** 1. Yes 2. No DK, RF
Blood pressure check **(BPC)** Optional	Heart disease is one of the leading causes of death in Canada. Regular blood pressure checks are an effective way of diagnosing hypertension, which is very strongly related to heart disease. In this module, respondents are asked if they have had their blood pressure checked in the past 2 years and, if so, when, and if not, why not. As well, females aged 15 to 55 are asked if they were pregnant the last time their blood pressure was measured. By looking at reasons why certain groups do not have their blood pressure checked as recommended, health region planners and others can develop promotional campaigns to remind people of the benefits.
BPC_R001	**Now, a few questions about blood pressure measurement.**

BPC_Q005

Have you ever had your blood pressure measured by a health professional?

1. Yes
2. No

DK, RF

A response inconsistent with a response to a previous question has been entered. Please confirm.

Pap smear test (PAP)
Optional

Cervical cancer is a leading type of cancer among Canadian women. Regular PAP smear tests are an effective way of detecting this cancer early enough for treatment to be effective.

In this module, female respondents aged 18 to 70 are asked if they have had a PAP smear test in the past 3 years and, if not, why not.

PAP_Q005

Have you ever had a PAP smear test?

1. Yes
2. No

DK, RF

Mammography (MAM)
Optional

Mammography, or an x-ray of the breast, is an important tool for detecting breast cancer.

In this module, female respondents aged 40 to 74 are asked if they have ever had a mammography exam and, if so, when, and if not in the past 3 years, why not.

MAM_Q005

Have you ever had a mammogram, that is, a breast x-ray?

1. Yes
2. No

DK, RF

Prostate cancer screening (PSA)
Optional

This module includes questions about two common methods of screening for prostate cancer:

1 A PSA blood test involves taking a small sample of blood and testing the level of a specific chemical, prostate specific antigen. A high level indicates that the patient may have a higher than normal risk of prostate cancer and further tests are usually ordered.

2 A digital rectal exam involves a doctor inserting a gloved finger into the rectum to feel whether the prostate gland is enlarged.

Results from this module can be used to plan prostate cancer screening programs and to identify which groups of men might benefit the most from campaigns to raise awareness of the importance of early prostate cancer detection.

The questions are asked to male respondents aged 35 and older.

PSA_R001	**Now, a few questions related to prostate exams.**
PSA_Q005	**Have you ever had a prostate specific antigen test for prostate cancer, that is, a PSA blood test?** 1. Yes 2. No DK, RF
Colorectal cancer screening (CCS) **Optional**	Colorectal cancer is one of the most common and serious types of cancer in Canada. In this module, respondents are asked about two common methods of screening for colorectal cancer: 1 FOBT checks for blood in the stool. 2 A colonoscopy or sigmoidoscopy involves inserting a tube into the rectum to look for various health problems, including cancer and polyps. Results from this module can be used to plan colorectal cancer screening programs and to identify which groups might benefit the most from campaigns to raise awareness of the importance of early detection of colorectal cancer. The questions are asked to all respondents aged 40 to 74.
CCS_R001	**Now a few questions about various colorectal exams.**
CCS_Q005	**An FOBT is a test to check for blood in your stool, where you have a bowel movement and use a stick to smear a small sample on a special card.** **Have you ever had this test?** 1. Yes 2. No DK, RF
Spirometry (SPI) Optional	In this module, respondents are asked if they have ever had this common lung function test. If they have, they are asked when the last time was. These questions will be used to examine the proportion of the population who had the Spirometry test and the characteristics of this population. Early diagnosis of common lung diseases helps in managing their progression and consequences.

SPI_Q005

Spirometry is a common lung function test that consists of blowing into a small tube attached to a machine.

Have you ever had this test?

1. Yes
2. No

DK, RF

Diabetes care (DIA)

Optional

In this module, respondents who have diabetes (non gestational) are asked about various aspects of the care of their diabetes.

They are asked about:

- tests or examinations done by health professionals, such as a haemoglobin “A-one-C” test, a check of the feet, a urine test, an eye examination and nutrition therapy
- tests or examinations done by themselves or other non-professionals, such as a glucose check and a check of the feet
- medications they may be taking.

Diabetes is a growing health problem in Canada. Answers to these questions can be used to study the care that people in Canada are getting to help manage their diabetes.

DIA_R001

You reported earlier that you have diabetes. The following questions are about diabetes care.

DIA_Q005

In the past 12 months, has a health care professional tested you for haemoglobin "A- one-C"? (An "A-one-C" haemoglobin test measures the average level of blood sugar over a 3-month period.)

1. Yes
2. No

DK, RF

Eye examinations

(EYX)

Optional

In this module, respondents are asked about the health and care of their eyes, including questions about insurance that covers the cost of glasses/contacts. They are asked if they have seen an optometrist/ophthalmologist in the past 12 months and, if not, when they last had an eye examination. Respondents who are younger than 19 or older than 65 or have not had an examination in the past 2 years, are asked why not. Those respondents 40 and over are also asked if they have cataracts or glaucoma.

The results of this module will be valuable in understanding which groups are less likely to have their eyes examined and why.

EYX_R001

Now, some questions about the health and care of your eyes.

EYX_Q005

Do you have insurance that covers all or part of the cost of eye glasses or contact lenses?

Stigma towards depression

(STG)

Optional

This module asks about the respondent's perception of others' attitudes towards people with depression, including trustworthiness, confidence in, likelihood of befriending, dating and hiring.

Researchers are interested in any negative perceptions Canadians may feel towards people with depression. It is important to understand the stigma attached to depression as it can often serve as a barrier to treatment.

Data collected will provide baseline information about perceptions toward people with depression, and can then be used to track changes over time.

STG_R001

The next questions deal with how people feel toward those who have had depression. By depression, we mean a prolonged period of sadness or loss of interest in usual activities that interferes in daily life.

I am going to read you a series of statements. For each of the following statements, please tell me how you think most people you know would feel even if you don't share their opinion. We'll ask about your own personal views later.

STG_Q005

Most people you know would not willingly accept someone who has had depression as a close friend. Do you…?

INTERVIEWER: Read categories to respondent. Most people you know includes for example family, friends, colleagues.

1. **Strongly agree**
2. **Agree**
3. **Neither agree nor disagree**
4. **Disagree**
5. **Strongly disagree**

DK, RF

Positive mental health

(PMH)

Theme

Answers to this module are used to determine the positive mental health of the respondent. Higher levels of positive mental health are widely thought to be associated with better overall health.

The questions in this module will determine the past month frequency that respondents report having positive feelings about themselves, their lives, their experiences, their environment and society.

PMH_R001

The following questions are about how you have been feeling during the past month.

PMH_Q005

In the past month, how often did you feel:

happy?

1. **Every day**
2. **Almost every day**
3. **About 2 or 3 times a week**
4. **About once a week**
5. **Once or twice**
6. **Never**

DK, RF

Consultations about mental health

(CMH)

Theme

This module deals with the number of times in the past 12 months that the respondents have consulted with mental health professionals to discuss their emotional or mental health, and which professionals they saw or talked to.

The results will help planners to understand how existing mental health services are being used and to target new services to the groups that need them the most.

CMH_R001	**Now I would like to ask you some questions about mental and emotional well-being.**
CMH_Q005	**In the past 12 months, that is, from one year ago to yesterday, have you seen or talked to a health professional about your emotional or mental health?** 1. Yes 2. No DK, RF
Satisfaction with life **(SWL)** Optional	This module includes questions about the respondent's level of satisfaction with various aspects of their life, including their job, their experiences, their financial situation, themselves, their relationships, their housing and their neighbourhood. Researchers are interested in the connection between satisfaction with life and overall physical and mental health and well-being.
SWL_R001	**Now, I'd like to ask about your satisfaction with various aspects of your life. For each question, please tell me whether you are very satisfied, satisfied, neither satisfied nor dissatisfied, dissatisfied or very dissatisfied.**
SWL_Q005	**How satisfied are you with your job or main activity?** INTERVIEWER: Read categories to respondent. **1. Very satisfied** **2. Satisfied** **3. Neither satisfied nor dissatisfied** **4. Dissatisfied** **5. Very dissatisfied** DK, RF
Self-esteem **(SFE)** Optional	Self-esteem, or positive feelings and confidence about oneself, is considered to be very important in helping people to deal with physical and mental health problems. This module consists of 6 questions that will be used to calculate an overall self-esteem score for each respondent.

SFE_R001

Now a series of statements that people might use to describe themselves.

Please tell me if you strongly agree, agree, neither agree nor disagree, disagree, or strongly disagree with the following statements.

SFE_Q005

You feel that you have a number of good qualities.

INTERVIEWER: Read categories to respondent.

1. **Strongly agree**
2. **Agree**
3. **Neither agree nor disagree**
4. **Disagree**
5. **Strongly disagree**

DK, RF

Distress (DIS)

Optional

This module explores how often the respondent has experienced a range of feelings in the past month, including tiredness, nervousness, hopelessness, worthlessness, feeling restless or fidgety, feeling sad or depressed, and feeling that everything is an effort.

Respondents are also asked to compare their feelings of distress from their usual feelings, and to rate how much their feelings of distress interfered with their lives in the past month.

The answers to all the questions are combined to create an overall measure of the respondent's level of distress.

Distress is an important part of overall mental well-being. Feelings of distress can interfere with many aspects of life, including physical and emotional health.

Note: this module is not selected for 2016.

DIS_R001

The following questions deal with feelings you may have had during the past month.

DIS_Q005

During the past month, that is, from ^ONEMONTHAGO to yesterday, about how often did you feel:

tired out for no good reason?

INTERVIEWER: Read categories to respondent.

1. **Every day**
2. **Almost every day**
3. **About 2 or 3 times a week**
4. **About once a week**
5. **Once or twice**
6. **Never**

DK, RF

Depression (DEP)

Optional

Many people feel depressed at one time or another, to varying degrees.

However, long-lasting depression is a serious mental and physical health concern.

This module consists of a series of detailed questions about depression. The answers will be combined to determine how likely it is that the respondent is depressed.

This module gathers information on the length, timing and consequences of depressive episodes.

DEP_R001

The following questions deal with problems you may have had during the last two weeks.

DEP_Q005

Over the last two weeks, how often have you:

had little interest or pleasure in doing things?

INTERVIEWER: Read categories to respondent.

1. **Not at all**
2. **Several days**
3. **More than half the day**
4. **Nearly every day**

DK, RF

Suicidal thoughts and attempts

(SUI)

Theme

Suicide is an important and growing concern in society today.

This module collects information on respondents' suicidal thoughts and attempts, including whether or not they've made a plan to attempt suicide in the past 12 months, the age when they last contemplated and/or planned and/or attempted suicide, the number of attempts in their lifetime and whether they required medical attention for the most recent attempt.

Please refer to the Sensitive Interviewing material provided in Chapter 2 of this manual.

The questions are asked only of respondents aged 15 and older.

SUI_R001

The following questions may be sensitive to some people, but we have to ask the same questions of everyone.

SUI_Q005

Have you ever <u>seriously</u> contemplated suicide?

1. Yes
2. No

DK, RF

Work stress

(WST)

Optional

This module includes a series of questions dealing with aspects of the respondent's job or business that could be stressful. This includes whether the respondent is required to learn new things, whether the job is hectic and whether the respondent is exposed to conflict with colleagues or supervisors.

This module is asked only of respondents aged 15-75 who have worked at a job or business at some point in the past 12 months.

The results of this module will be used to study the connections between work-related stress, health problems, and use of health care services.

WST_R001

The next few questions are about your main job or business in the past 12 months. I'm going to read you a series of statements that might describe your job situation. Please tell me if you strongly agree, agree, neither agree nor disagree, disagree, or strongly disagree.

WST_Q005

Your job required that you learn new things.

INTERVIEWER: Read categories to respondent.

1. **Strongly agree**
2. **Agree**
3. **Neither agree nor disagree**
4. **Disagree**
5. **Strongly disagree**

DK, RF

Sources of stress

(STS)

Optional

The relationship between stress and health is complex. For example, stress may be the cause of certain health problems and the effect of other

Health planners will be able to use the results of this module to identify the most common sources of stress and develop programs that promote healthy ways of dealing with stress.

This module covers a range of issues related to stress, including the ability to deal with unexpected and difficult problems, the ability to deal with everyday demands, and the main source of day-to-day stress.

STS_R001

Now a few questions about the stress in your life.

STS_Q005

In general, how would you rate your ability to handle unexpected and difficult problems, for example, a family or personal crisis? Would you say your ability is...?

INTERVIEWER: Read categories to respondent.

1. **Excellent**
2. **Good**
3. **Fair**
4. **Poor**

DK, RF

Social provisions

(SPS)

Optional

The relationships an individual has with friends, family, co-workers and others can have a strong impact on their own mental and physical health.

This module consists of a series of questions about the respondent's level of attachment, social integration, reassurance of worth, reliable alliance, and guidance.

SPS_R001

The next questions are about your current relationships with friends, family members, co-workers, community members, and so on. Please indicate to what extent each statement describes your current relationships with other people.

SPS_Q005

There are people I can depend on to help me if I really need it.

INTERVIEWER: Read categories to respondent.

1. **Strongly agree**
2. **Agree**
3. **Neither agree nor disagree**
4. **Disagree**
5. **Strongly disagree**

DK, RF

Medical doctor attachment (MDA)

This module provides information on the level of attachment of Canadians to their doctor. If they have a family doctor or general practitioner (GP), it asks about the length of time they have had their doctor. If they don't have a doctor, it asks if they are looking for one.
If they have a 'usual place of care' where there is more than one doctor, it asks about the number of doctors they can see, and how often they are taken care of by the same doctor.
The data collected will enable researchers to get a sense of whether Canadians are followed-up by a regular doctor.

MDA_Q005

How long have you had your family doctor or general practitioner for?

INTERVIEWER: Read categories to respondent. If needed, you can prompt the respondent 'This is the doctor previously indicated as being consulted regularly'.

1. **Less than 6 months**
2. **6 months to less than 12 months**
3. **1 year to less than 2 years**
4. **2 years or more**

DK, RF

Primary health care (PHC)

Core

Primary health care is often the first point of entry into the Canadian health care system. It incorporates diagnosis, treatment and management of health problems.

This module covers a number of topics related to the use of primary health care, including whether the respondent has a usual place to go for immediate care for minor health problems, whether the respondent has a regular health care provider, language spoken during appointments, and the types of health care professionals that are involved in the care of the respondent.

Primary health care (PHC)
Core (continued)

This module has a number of uses for governments, health care planners and researchers. For example, results could be used to compare how different groups—such as men and women, young and old, urban and rural—use health services. It will also track the change over time as to where Canadians go to and whom they chose to see when they need care.

PHC_R001

Now I'd like to ask about your primary health care. It is often the first point of entry to the Canadian health system. It incorporates diagnosis, treatment and management of health problems.

PHC_Q005

Is there a place that you usually go to when you need immediate care for a minor health problem?

INTERVIEWER: If respondent has more than one place he/she went to in the last 12 months, please refer to the one place he/she went to the most often.

1. Yes
2. No

DK, RF

PHC_Q010

What kind of place is it?

INTERVIEWER: Read categories to respondent. Please exclude Internet as a usual “place” to go to for care.

1. **A doctor's office**
2. **A hospital outpatient clinic**
3. **A community health centre ^CLSC**
4. **A walk-in clinic**
5. **A hospital emergency room**
6. **Some other place**

DK, RF

Contacts with health professionals – part 1 (CHP) -
Core

The objective of this module is to gather information about contact with various health professionals and use of health care services. This includes the use of home care services during the past 12 months, the use of hospital emergency rooms, or the number of overnight stays for hospital visits, etc.

Contacts with health professionals – part 1 (CHP) Core (continued)	As well, it asks about contact with, and frequency of visits to, a family doctor/ general practitioner, eye specialist, any other medical doctor, a nurse or a dental professional for physical, mental or emotional reasons. This module has a number of uses for governments, health care planners and researchers. For example, results could be used to compare how different groups—such as men and women, young and old, urban and rural—use health services. This type of research can be used to determine who needs better access to care.
CHP_R001	**Now I'd like to ask about your contacts with various health professionals and use of various health care services during the past 12 months, that is, from ^ONE YEAR AGO to yesterday.**
CHP_R005	**Home care services are services provided in your own home because of a health condition or limitation in daily activities; they can include for example, nursing care, meal preparation, someone to help you bathe or do housework. Do not include the help from family, friends or neighbours.**
CHP_Q005	**In the past 12 months, have you received any home care services?** 1. Yes 2. No DK, RF
Contacts with health professionals – extended block (CP2) Optional	The objective of this module is to gather information on the place and frequency of contact with various health professionals during the past 12 months. Questions pertain to one of the following health professionals: chiropractor, physiotherapist, psychologist, social worker/counselor, audiologist, speech therapist, occupational therapist, dietician or nutritionist, and pharmacist. **Note:** this module is not selected for 2016.
CP2_Q005	**Have you seen or talked to any of the following health professionals about your physical, emotional or mental health:** INTERVIEWER: Include both face to face and telephone contacts. **a chiropractor?** 1. Yes 2. No DK, RF
CP2_Q010	**How many times (in the past 12 months)?**

Patient satisfaction – community-based care (PSC)
Optional

This module assesses whether respondents received any community-based health care services in the past 12 months (outside of a hospital or doctor's office), and includes their rating of the quality and satisfaction with the care received.
Examples of community–based health care services are: home nursing care, home-based counselling or therapy, personal care and community walk-in clinics.

PSC_R001

The next questions are about community-based health care which include other health care services received outside of a hospital or doctor's office.

Examples are: home nursing care, home-based counselling or therapy, personal care and community health clinics.

PSC_Q005

In the past 12 months, have you received any community-based care?

1. Yes
2. No

DK, RF

Consultations with health professionals on smoking (SPC)

Optional

In this module, respondents who smoke are asked whether their regular health care provider or dentist knows that they smoke, or has advised them to stop or reduce smoking. Questions about the type of help, if any, provided by the regular health care provider, dentist or pharmacist are also asked.

The results of this module will be used to understand what role regular health care providers, dentists and pharmacists can play in helping people reduce smoking.

Note: this module is not selected for 2016.

SPC_Q005

Does your ^HEALTH_PROFESSIONAL know that you smoke cigarettes?

1. Yes
2. No

DK, RF

Patient experiences

(PEX)

Theme

This module asks questions about the respondent's last consultation with a health professional, in person or over the phone. It excludes overnight hospital stays, dental care visits, accidents or emergency care, care received in their home or alternative health care. It also evaluates the quality of the time spent with the health professional and asks for the respondent's overall rating of the consultation.

The information from this module will be used to identify areas where patient's experience with the health system can be improved.

PEX_R001

The next questions are about your last consultation with a health professional

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PEX_Q005

When was the last time that you had a consultation with any health professional about your own health? Exclude care you got when you stayed overnight in a hospital, dental care visits, accident or emergency care, or care received in your home.

1. In the last 30 days
2. Between 1 and 3 months ago
3. More than 3 but less than 6 months ago
4. Between 6 and 12 months ago
5. Between 1 and 2 years ago
6. More than 2 years ago

DK, RF

Access to health care services

(ACC)

Optional

This module deals with respondents' experiences and perceptions regarding access to various types of health care services. It is asked only of those respondents aged 15 or over. These include:

- health care received from a medical specialist
- non-emergency surgical procedures
- diagnostic tests such as MRIs, CAT scans and angiographies
- obtaining health information or advice
- health care services for routine or on-going medical or dental care
- immediate care for a minor health problem

Respondents are asked if they experienced any difficulties obtaining the health service(s) that they required.

This module will be used to compare different groups of Canadians according to how they use health care services (including the time of day and day of the week) and what barriers they faced when trying to access health care services.

ACC_R001

The next questions are about the use of various health care services.

I will start by asking about your experiences getting health care from a medical specialist such as a cardiologist, endocrinologist, allergist, or psychiatrist (excluding an optometrist).

ACC_Q005

In the past 12 months, did you require a visit to a medical specialist for a diagnosis or a consultation?

Note: the ACC module must be selected for WTM to be asked.

1. Yes
2. No

DK, RF

Waiting times (WTM)
Optional

This module collects information on the respondent's perceptions and experiences regarding waiting times for specific health care services, including: visits to medical specialists for new health conditions, non-emergency surgery and specific diagnostic tests. It is asked of respondents 15 years of age and older.

The module is divided into three sections, one for each of the three types of health care service.

For the three types of service, this module collects information on:

- the type of service received or type of condition treated
- difficulties in receiving the service
- the length of time spent waiting for the service
- the respondent’s perception of whether the waiting time was acceptable

The respondent’s perception of the impact of waiting, on their health and other aspects of their life, is also asked.

WTM_R001

Now some additional questions about your experiences waiting for health care services.

WTM_Q005

You mentioned that you required a visit to a medical specialist such as a cardiologist, endocrinologist, allergist, gynaecologist or psychiatrist.

In the past 12 months, did you require a visit to a medical specialist for a diagnosis or a consultation <u>for a new illness or condition</u>?

1. Yes
2. No

DK, RF

Home care services (HMC)
Theme

This module asks respondents aged 18 and over if they received any home care services, such as nursing care, meal preparation, help with bathing or housework, either for themselves or for someone else in their household.

Respondents are also asked about the length of services, cost and payment, level of satisfaction with care, unmet care needs, reasons for not getting the services, and where they tried to obtain services.

Home care is considered to be an important part of health care reform. Both the health care system and the patient may benefit if some types of care are provided in the home instead of in a hospital or institution.

This module can be used to identify which groups are receiving home care and which are not. The health status of these groups and how they use other health services can be compared. This can help researchers understand whether home care is working as planned.

HMC_R001

Now some questions on home care services that you or someone else in the household may have received because of a health condition or a limitation in daily activities. These include services provided in your own home such as nursing care, meal preparation, someone to help with bathing or housework, etc. Do not include the help from family, friends or neighbours.

HMC_Q005

In the past 12 months, what type of home care services did you or someone else in the household receive?

INTERVIEWER: Read categories to respondent. Mark all that apply. Do not include post-partum care.

1. **Nursing care (e.g., dressing changes, preparing medications, V.O.N. visits)**
2. **Other health care services (e.g., physiotherapy, occupational or speech therapy, nutrition counselling)**
3. **Medical equipment or supplies (e.g., wheelchair, pads for incontinence, help with using a ventilator or oxygen equipment)**
4. **Personal or home support (such as bathing, housekeeping, meal preparation)**
5. **Other services (such as transportation, meals-on-wheels)**
6. **No one in the household received any home care services**

   DK, RF

Unmet health care needs (UCN)

Optional

The purpose of this module is to identify respondents who needed health care (other than home care services) in the last 12 months but did not receive it. Questions are asked about the type of care they needed, whether they actively tried to obtain the health care, where they went to get care, and why they didn't receive it.

Data will be used to assess the extent of unmet health care needs, identify geographic areas where improvements could be made and determine why these needs are not fulfilled.

UCN_Q005

During the past 12 months, was there ever a time when you felt that you needed health care, other than homecare services, but you did not receive it?

1. Yes
2. No

DK, RF

Labour force (LBF)

Core

There are many relationships between work and health. For example, unemployment can cause various stress-related illnesses while some occupations are more likely to cause repetitive strain injuries.

This module includes questions about the respondent's current labour force activities during the past week, including job status and absenteeism, job classification (employee or self-employed), industry, occupation and main activities, number of jobs, and usual hours worked per week.

The concepts and definitions are, for the most part, the same as those used in the Labour Force Survey.

The module is asked of respondents aged between 15 and 75. Not all questions will be asked of all respondents - pay attention to the skip patterns.

LBF_R010

The next questions are about your current job or business.

INTERVIEWER: If person currently holds more than one job, report on the job for which the number of hours worked per week is the greatest.

LBF_Q030

Did you have more than one job or business last week?

1. Yes
2. No

DK, RF

Labour market activities minimal (LMAM)

This is a sub-module for the Labour Market Activities (LBF). It contains the questions required to establish whether a respondent is employed or not.

LMAM_Q01

Many of the following questions concern labour market activities last week.

Last week did you work at a job or business?

(regardless of the number of hours)

1. Yes
2. No

DK, RF

Class of worker (LMA3)

This is a block within the extended content for the Labour Market (LBF) It contains the question required to identify the class of worker for a respondent who is employed.

LMA3_Q10

Were you an employee or self-employed?

1. Employee
2. Self-employed
3. Working in a family business without pay

DK, RF

Industry (LMA4)

This is a block within the extended content for the Labour Market Activities Harmonized Content. It contains the questions required to identify the industry for a respondent who is employed.

LMA4_Q11

What was the name of your business?

INTERVIEWER: Enter the full name of the business.

If there is no business name, enter the respondent's full name.

(50 spaces)

Occupation (LMA5)

This module is a sub-module of the Labour Market Activities (LBF). It contains the questions required to identify the occupation for a respondent who is employed.

LMA5_Q14

What was your work or occupation?

INTERVIEWER: Enter a detailed description.

For example:

1. legal secretary
2. plumber
3. fishing guide
4. wood furniture assembler
5. secondary school teacher
6. computer programmer

(50 spaces)

Usual hours of work (LMA6)

Usual Hours of Work (LMA6) is another sub-module for the Labour Market Activities (LBF) module. It contains the questions required to identify the number of usual hours worked for a respondent who is employed.

Loss of productivity

(LOP)

Optional

In this module, respondents are asked if they have missed work in the past three months due to health reasons (chronic physical or mental condition, injury, infectious disease, etc.), or other reasons such as caring for own children, temporary lay-off, retirement or other.

Subsequent questions identify, in more detail, the respondent's health issues that caused the loss of productivity.

This set of questions is asked of respondents 15 and over who have worked at any time over the last 12 months.

Data will be used to analyze the economic burden resulting from Canadians' health issues.

LOP_Q005

At any time during the past three months, did you work at a job or a business?

INTERVIEWER: Include only paid job or business.

1. Yes
2. No

DK, RF

Socio-demographic characteristics (SDC)

Core

Very important knowledge of understanding the health of Canadians is obtained by comparing the health of different groups. For example, do recent immigrants to Canada use health care services more or less than people born in Canada? Are people from European ethnic groups more likely to have certain chronic conditions than other groups?

In this module, important social and demographic information is collected, including immigrant status, country/province of birth, ethnic origin, aboriginal status, racial/cultural group, language, dwelling type and sexual orientation.

SDC_R001

Now, some general background questions which will help us compare the health of people in Canada.

SDC_Q010

To which ethnic or cultural groups did your ancestors belong? (For example: French, Scottish, Chinese, East Indian).
INTERVIEWER: Mark all that apply. An ancestor is usually more distant than a grandparent. If "Canadian" is the only response, probe. If the respondent hesitates, do not suggest Canadian. If the respondent answers "Eskimo", enter "20".

01 Canadian
02 French
03 English
04 German
05 Scottish
06 Irish
07 Italian
08 Ukrainian
09 Dutch (Netherlands)
10 Chinese
11 Jewish
12 Polish
13 Portuguese
14 South Asian (e.g., East Indian, Pakistani, Sri Lankan)
15 Norwegian
16 Welsh
17 Swedish
18 First Nations (North American Indian)
19 Métis
20 Inuit
21 Other - Specify
DK, RF

SDC_S010

(To which ethnic or cultural groups did your ancestors belong? (For example: French, Scottish, Chinese, East Indian))
INTERVIEWER: Specify.

(80 spaces)
DK, RF

Immigration minimum block (IMM)

This is the CATI-CAPI block for Place of birth, Landed immigrant status and Year of immigration in Canada.

IMM_Q01

In what country were you born?

INTERVIEWER: Ask the respondent to specify country of birth according to current boundaries.

Start typing the name of the country to activate function.

Enter "CAN" to select Canada.

Enter "Other - Specify" if the country is not part of the list.

1. Search
2. Other - Specify

DK, RF

Person most knowledgeable about household situation

(PMK)

Core

The purpose of this module is to identify someone who may be more knowledgeable and better prepared than the 12-17 year-old selected respondent to answer household level questions such as income, insurance and food security questions.

This module will be generated only if the selected respondent is between the ages of 12 to 17 and does NOT live alone or does not have any other person living in the household who is older than 17. If this respondent is not less than 14 years old, they will be asked if they agree to share their information. The interview will not continue unless a suitable PMK is found and is willing to continue the interview. If one is not found then the interviewer will exit the application and select an appropriate outcome code.

Health insurance coverage

(INS)

Theme

In this module, respondents are asked whether they have insurance to cover all or part of the cost of prescription medications, home care or long-term care.

Respondents should include any private, government, association or employer-paid insurance plans.

The results of this module will be used to get an overall picture of which groups have various types of insurance. Researchers will be able to study this in combination with levels of health and use of health care services.

INS_R001

Now, turning to your health insurance coverage. Please include any private, government or employer-paid plans.

INS_Q005

Do you have insurance that covers all or part of the cost of your prescription medications?

1. Yes
2. No

DK, RF

Food security (FSC)

Optional

This module asks respondents if anyone in the household has experienced any issues associated with food security in the previous 12 months, (the ability to afford the purchase of sufficient nutritious food), with specific questions about the actions and needs of the adults who are responsible for children.

Even in households that appear to have high incomes, budget constraints may make it difficult to purchase nutritious foods. Food security questions will be asked of all respondents regardless of income.

Note: This module should be answered by a knowledgeable member of the household. Persons 18 and older are considered adults. Youths aged 16 and 17 living with a relative are considered children, all others are considered adults.

FSC_R010

The following questions are about the food situation for your household in the past 12 months. I'm going to read you several statements that may be used to describe the food situation for a household. Please tell me if the statement was <u>often</u> true, <u>sometimes</u> true, or <u>never</u> true for you in the past 12 months.

FSC_Q010

The first statement is: you and others worried that food would run out before you got money to buy more. Was that <u>often</u> true, <u>sometimes</u> true, or <u>never</u> true in the past 12 months?

1. Often true
2. Sometimes true
3. Never true

DK, RF

Administration information

(ADM)

Core

In this module, respondents are <u>informed</u> that their information may be used for data linkage. Respondents are asked to provide their provincial or territorial health number. Respondents are also asked to give their permission to share their data with health partners.

The interviewer is then asked to supply additional information. For CAPI respondents, identify whether the interview was completed in person or by telephone. For all respondents, state the language of the interview, whether the respondent was alone when completing the interview and, if not, did this affect the respondent's answers.

ADM_Q005

Respondents NOT linkable to tax data

If a respondent is flagged as **not** having linkable tax data (identified by the sample file and pre-determined by HO), he/she will be directed to the INC module to answer the income questions. At that point the linkage statement will be read.

To enhance the information you have provided, Statistics Canada will combine your responses with information from your tax data. Statistics Canada may also combine your responses with information from other survey or administrative sources. This may include information on past and continuing use of health services such as visits to hospitals, clinics and doctor's offices.

INTERVIEWER: Press <1> to continue, unless respondent objects to combining responses with other data sources.

ADM_Q005

Respondents linkable to tax data

If the respondent refuses linkage and he/she is flagged as having linkable tax data, he/she will be directed to the following statement:

To minimize the number of questions asked, Statistics Canada will combine your responses with information from the tax data of all members of your household. Statistics Canada may also combine your responses with information from other survey or administrative sources. This may include information on past and continuing use of health services such as visits to hospitals, clinics and doctor's offices.

INTERVIEWER: Press <1> to continue, unless respondent objects to combining responses with other data sources.

ADM_Q010

Having a provincial or territorial health number will assist us in linking to this other information.

Do you have a health number?

1. Yes
2. No

DK, RF

ADM_Q015

For which province or territory is your health number?

IMPORTANT note on health numbers

If the respondent cannot provide his or her health number at the time of the interview, indicate “no”, and continue with the questionnaire. Do not code the case as in-progress; the case should be finalized whether a health number was obtained or not. If a respondent offers to call back and provide his or her health card number, take down this information in the permanent notes section. Any change to the health number information is not considered a critical note.

ADM_R025

To avoid duplication of surveys, Statistics Canada has signed agreements with provincial and territorial ministries of health, Health Canada and the Public Health Agency of Canada to share the information that you provided on this survey.

Provincial ministries of health may make this information available to local health authorities, but no identifiable information such as...

INTERVIEWER: The personal identifiers (names, addresses, telephone numbers and health numbers) will not be provided to Health Canada or the Public Health Agency of Canada. "Provincial ministries of health" includes the territorial ministries of health.

ADM_R030

To avoid duplication of surveys, Statistics Canada has signed agreements with provincial and territorial ministries of health, the " Institut de la Statistique du Québec ", Health Canada and the Public Health Agency of Canada to share the information that you provided on this survey.

The " Institut de la Statistique du Québec " and provincial ministries of health may make this information available to local health authorities, but no identifiable information such as ...

INTERVIEWER: The personal identifiers (names, addresses, telephone numbers and health numbers) will not be provided to Health Canada or the Public Health Agency of Canada. "Provincial ministries of health" includes the territorial ministries of health.

ADM_R035

These organizations have agreed to keep your information confidential and use it only for statistical purposes.

Do you agree to share the information provided?

1. Yes
2. No

DK, RF

New order of module: **Income (INC) now a sub-block of ADM**	Income is now a sub-block of Administration Information module (which means it will follow ADM). As mentioned the 2016 application has been modified so that respondents with records that contain linkable tax data will no longer be asked the income question (unless they refuse the linkage statement in ADM). Individuals whose records cannot be linked to tax data will be asked the INC question. This change has been made in the application and will occur automatically.
Income **(INC)** Core	In this module, respondents are asked to provide the amount of their individual and household incomes, all sources of income, and their main sources of household and personal income, for the last calendar year. Personal income is asked only when there is more than one person living in the household, and when the respondent is aged 16 and over. The respondent is also asked if either total household or personal income includes a supplement for people with disabilities. Although income is a sensitive topic, this information is very important for studying the link between income and health. Although many health care costs in Canada are covered by insurance, income still plays an important role in people's health. People with lower incomes are more likely to have poorer health. People with varying income levels also tend to use the health care system differently.
INC_R001	**Although many health expenses are covered by health insurance, there is still an important relationship between health and income. Please be assured that, like all other information you have provided, these answers will be kept strictly confidential.**

INC_Q005

Thinking about the total income for all household members, from which of the following sources did your household receive any income in the year ending December 31, 2014. ?

INTERVIEWER: Read categories to respondent.

Mark all that apply

01	**Wages and salaries**	09	**Old Age Security and Guaranteed Income Supplement**
02	**Income from self-employment**	10	**Provincial or municipal social assistance or welfare** - (Go to INC_Q010)
03	**Dividends and interest (e.g., on bonds, savings)**	11	**Child Tax Benefit or family allowances**
04	**Employment insurance**	12	**Child support**
05	**Workers' compensation**	13	**Alimony**
06	**Benefits from Canada or Quebec Pension Plan -** (Go to INC_Q010)	14	**Other (e.g., rental income, scholarships)**
07	**Job-related retirement pensions, superannuation and annuities -**	15	**None**
08	**RRSP/RRIF (Registered Retirement Savings Plan/Registered Retirement Income Fund)**		DK, RF

INC_Q010

Does this amount include a supplement for people with disabilities?

1. Yes
2. No

DK, RF

INC_Q015

What was the main source of household income?

Total household income (THI)

THI_Q01

Now a question about your total household income.

What is your best estimate of your total household income received by all household members, from all sources, before taxes and deductions, during the year ending December 31, 2014?

Income can come from various sources such as from work, investments, pensions or government. Examples include Employment Insurance, Social Assistance, Child Tax Benefit and other income such as child support, spousal support (alimony) and rental income.

INTERVIEWER: Capital gains should not be included in the household income.

THI_Q02

Can you estimate in which of the following groups your household income falls? Was the total household income during the year ending December 31, 2014...

INTERVIEWER: Read categories to respondent.

1. **Less than $50,000, including income loss**
2. **$50,000 and more**

DK, RF

Questionnaire Overview – Exit

Overview

The Exit portion of the interview is activated any time a case is exited.

A case may be exited in one of the following ways:

- by selecting “No” at the contact question in Entry
- by exiting the case at the components screen or at any time usingthe <F10> key
- by completing the interview.

Introduction

The last section is the Exit portion of the application where a Final Outcome code will be assigned and the case will be ready for transmission. The Exit component includes:

- selection of outcome code for exited interviews (F10),
- request for permission to share for partial interviews,
- collection of contact and alternate contact information,
- verification of the respondent’s name, and
- thank you to the respondent for their participation.

SO_N01

INTERVIEWER: This is the end of the component. Return to previously answered questions to make any necessary corrections, or select <Exit> to exit the component.

1. Exit

   (DK, R are not allowed)

Display auto outcome code (DAO)

The Display Auto Outcome Code block is used to display the outcome code auto assigned by the application.

INTERVIEWER: This case has been coded fully completed.

Press <1> to continue, or go back to make any necessary changes.

Exiting cases using F10 key

When a case is exited using F10 the following screens appear. When “In-Progress codes” is selected OC_N02 will appear. When “Final codes” is selected OC_N03 will appear.

Outcome Code (OC)

The Outcome Code block is used to assign an outcome code to a case. Note that surveys must specify the lists of in-progress or final outcome codes from which interviewers will be able to select.

INTERVIEWER: Is the case in-progress or final?

1. In-progress codes
2. Final codes

   (DK, R are not allowed)

When an in-progress code is assigned to a case, the case will exit.

Record any notes or set appointments as necessary.

The in-progress code selected will be shown next to the sample ID on the Select Case screen.

For a complete list of codes and descriptions, see Appendix C.

A separate list of in-progress and final outcome codes by age group and collection mode is also provided below:

OC_N04 (Refusal)

When a case is assigned a final code 80 (Refusal) OC_N04 is used to record the reason for the refusal.

INTERVIEWER: Record the reason for the refusal.

1. Dangerous / Rude attitude
2. Won't answer the door
3. Not interested / Doesn't want to participate
4. Doesn't want to be disturbed
5. Doesn't have the time
6. Against the government or Statistics Canada
7. Doesn't believe in or want to hear about statistics
8. Doesn't believe in or want to hear about surveys
9. Recently completed a survey (doesn't want to again)
10. Doesn't believe the info is secure (confidentiality)
11. Doesn't want to give out personal information
12. Says not obligated / Wants legal proof
13. Can get info somewhere else (e.g. Revenue Canada)
14. Adamant refusal (no reason, eg: shuts the door, hangs-up the phone)
15. Doesn't want to continue the survey (no more follow-ups)
16. Same household, refusal maintained (for follow-ups only)
17. Why me? Tells you to choose someone else
18. Other – Specify

OC_S04	INTERVIEWER: Specify. _____
Exiting cases using F10 key	At this point the interview ends for cases exited using the F10 key that have not reached the partial interview point (see below).
Partial interview	An interview is considered partial when it has been completed up to and including the first question of the Primary Health Care (PHC) module but has not been completed up to the end of the Administration module. See Permission to Share (PS) module for more details.
Exit Introduction (EI)	The Exit Introduction block provides an introduction to the final section of the interview. **Before we finish, I would like to ask you a few other questions.** INTERVIEWER: Press < 1 > to continue. **Note:** <F5> "Refusal" and <F6> "Don't Know" are disabled for this question.
Permission to Share (PS)	This block contains a request to share the data collected during the interview with other agencies and departments. The text of the explanation and question is specific to each survey. Multiple screens can be programmed. It is used to introduce the request to share information collected during the interview with one or more other government departments. With data sharing, there is no need for each department to collect its own information, so fewer surveys have to be conducted. These questions are only asked in the Exit Component if the case was exited (using F10 key) after the partial cut-off point (the first question in the PHC module) and prior to completion of the Administration module where these questions appear for a fully completed interview.
PS_R01	All Provinces (except Québec) and the Territories: **Statistics Canada would like your permission to share the information collected in this survey with provincial and territorial ministries of health, Health Canada and the Public Health Agency of Canada.** **Provincial ministries of health may make this information available to local health authorities, but names, addresses, telephone numbers and health numbers will not be provided.**

PS_R01
(continued)

Québec:

Statistics Canada would like your permission to share the information collected in this survey with provincial and territorial ministries of health, the "Institut de la Statistique du Québec", Health Canada and the Public Health Agency of Canada.

The "Institut de la Statistique du Québec" and provincial ministries of health may make this information available to local health authorities, but names, addresses, telephone numbers and health numbers will not be provided.

INTERVIEWER: The personal identifiers (names, addresses, telephone numbers and health numbers) will not be provided to H4ealth Canada or the Public Health Agency of Canada. "Provincial ministries of health" includes the territorial ministries of health.

Note: <F5> "Refusal" and <F6> "Don't Know" are disabled for this question.

PS_Q02

All information will be kept confidential and used only for statistical purposes. Do you agree to share the information provided?

1 Yes
2 No

Notes

At this point the interview will continue in cases that have been coded as 70 – Fully complete. For cases that have been exited using the F10 key, the interview will end with the TY_R01 screen.

Living quarters (LQ)

This block determines whether there are other, possibly unlisted, dwellings within the structure. The F02 Cluster List should be checked to ensure a multiple is not already listed. Select "Yes" only if the other dwelling(s) are not listed on the F02 Cluster List.

Note: If "Yes" is selected, one or more new dwellings ("multiples") may have to be added to the F02 Cluster List.

CAPI only

Number of multiples (MU)

The MU module is called only when multiples were not created in the Entry and YES is selected as the answer to the LQ question.

Note: (DK, RF not allowed)

CAPI only.

Contact 1 (CON1)	**In case you move or change telephone numbers, it would be helpful if you could provide the name, telephone number and address of a relative or a friend who could help us to contact you.** **Note:** Names of all rostered household members will be listed on screen.
GCN_Q01	**I want to emphasize that Statistics Canada will contact this person only if there are difficulties in reaching you/them.** INTERVIEWER: Enter the first name. _____ (25 spaces) INTERVIEWER: Enter the last name. _____ (25 spaces)
Thank You 1 (TY1)	**Thank you for your time.** **Note:** <F5> "Refusal" and <F6> "Don't Know" are disabled for this question.
CAI Shut-Off (SO)	The CAI Shut-Off question is an indicator to interviewers that a component has ended. Once a response has been entered the component is closed and cannot be re-accessed. INTERVIEWER: This is the end of the component. Return to previously answered questions to make any necessary corrections, or select <Exit> to exit the component. **Note :** DK, R are not allowed.
	End of Exit.

4. Appendices

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Appendix A – Glossary

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Overview

Introduction

The glossary of terms is a reference tool for the Canadian Community Health Survey (CCHS). The glossary provides definitions for the survey terms and concepts used in the CCHS Interviewer Manual and application.

The purpose of the survey definitions is to ensure a common understanding of the CCHS survey concepts. By understanding and applying these definitions you and every interviewer in the country will have a consistent approach to collecting the CCHS data.

Glossary of terms

Part of the application	The glossary of terms is part of the application. The help key can be accessed by clicking on the HLP tab under the title bar, or by using the F7 key. Type in the first letter of the term you want to look up in the glossary. Press < Enter >. Press < Enter > again to continue with your interview or use the up arrow to get back to the INTERVIEWER direction screen: “Help topics are grouped and sorted in alphabetical order.” You can only type in another letter from this screen.
222s	Type of pain reliever that contains codeine and is usually available without a prescription.
292s	Type of pain reliever that contains codeine and is usually available only with a prescription.
Acid	Type of hallucinogen, often used as a slang term for LSD.
Acupuncture	A method of treating various medical conditions by stimulating acupuncture points along the skin by the insertion of thin needles.
Acupuncturist	A person skilled in the practice of acupuncture (a method of treating various medical conditions by stimulating acupuncture points along the skin by the insertion of thin needles.
Acute condition	Diagnosed or undiagnosed short-term physical or mental health condition that lasts less than 6 months.
Acute infectious disease	Short-term communicable or contagious condition such as a cold, flu, stomach flu, or any other respiratory infection.
Adderall™	Type of stimulant prescribed by a doctor to help with attention or concentration problems, such as ADHD.
Allergist	A medical doctor who specializes in the diagnosis and treatment of allergic disorders.
Allergy	A harmful immune response by the body to a substance (i.e. a particular food, dust, pollen) to which it has become hypersensitive.

Alprazolam	Type of sedative that can be obtained from a doctor, often used in the treatment of anxiety disorders and panic disorders and for short-term relief of anxiety symptoms.
Alzheimer's disease	Is the most common form of dementia in the elderly and causes impairment of memory and mental abilities.
Amphetamines	A drug that has a stimulating effect on the central and peripheral nervous systems. Amphetamines look like a white powder, capsule or tablet or clear liquid. (Other names : Speed, Crystal, Crank, Meth, Go, Jet fuel, Rocket fuel, Junk.)
Angel dust (phencyclidine)	Also called PCP, a dissociative drug that can be smoked, inhaled or injected.
Angiography	(pronounced "AN-JEE-OH-GRAPH-EE".) A diagnostic test in which a catheter is used to inject a dye into blood vessels. X-Rays are then taken to produce an image of the blood vessels. It is used as a diagnostic aid in conditions like strokes and heart attacks.
Antibiotic	A medication that inhibits the growth of or destroys bacteria.
Anxiety disorders	Illnesses that may make people feel anxious most of the time, without an obvious reason. People may get occasional but intense moments of anxiety that immobilize them.
Arthritis	A condition characterized by inflammation of one or more joints of the body, usually producing pain, redness, and stiffness.
Asthma	A chronic respiratory disorder characterized by labored breathing and wheezing resulting from obstructed and constricted air passages.
Ativan™	Trademark for preparations of lorazepam, a type of sedative obtained from a doctor. Often used in the treatment of anxiety disorders, for short-term relief of anxiety symptoms, and as a sedative-hypnotic agent.
Attention Deficit Disorder (ADD)	A chronic neurobiological disorder that includes an inability to sustain attention and concentration, distractibility and impulsivity.

Attention Deficit Hyperactivity Disorder (ADHD)	(see Attention Deficit Disorder (ADD)) with hyperactivity.
Audiologist (Audiology therapist)	A specially trained therapist who treats people with hearing difficulties.
Back problems	Damage to vertebrae, disks, ligaments, muscles, marrow or nerves causing back pain.
Bi-polar disorder	A mood disorder characterized by alternating manic and depressive episodes.
Birth control pill	An oral contraceptive containing progesterone and often estrogen, used to prevent ovulation and pregnancy.
Birth defect (Congenital anomalies)	A structural or metabolic imperfection/anomaly present at birth, such as heart defect or hypothyroidism.
Blood pressure	The pressure of the blood on the walls of the arteries, dependent on the energy of the heart action, the elasticity of the walls of the arteries, and the volume and viscosity of the blood.
Bone density test	A bone density test uses a special x-ray device (called DXA) to determine the mineral matter of bone as an indicator of osteoporosis or fracture risk.
Bowel incontinence	The loss of bowel control, resulting in involuntary passages of feces.
Brain injury	An injury to the brain that can cause a wide range of changes affecting memory, sensation, strength, language, movement or emotions.
Cancer	A disease characterized by an abnormal and uncontrolled growth of cells which may invade and destroy nearby tissues and organs or spread to other parts of the body.

Cannabis	A drug consisting of the dried leaves of the hemp plant smoked or chewed for euphoric effect (see marijuana). Considered by some to be a soft drug. May be used for medicinal purposes, especially for pain control.
Cardiologist	A medical doctor specializing in the diagnosis and treatment of heart disease.
Cardiovascular disease (heart disease)	Disease relating to the heart and blood vessels, such as stroke, angina or heart failure.
Carpal tunnel syndrome	A condition characterized by nerve damage in the palm side of the wrist due to trauma from repetitive work.
CAT Scan (also CT Scan)	Computerized Axial Tomography. This is a test in which X-Rays and computers are used to produce an image of a cross-section of the body.
Cataract	A condition characterized by opacity of the lens of the eye. An opacity, partial or complete, of one or both eyes on or in the lens, can impair vision or cause blindness.
Celebrex	A nonsteroidal anti-inflammatory (NSAID), non-prescription medication used to relieve pain and inflammation.
Celiac disease	A digestive disorder of the small intestine, causing chronic failure to digest food properly unless gluten is excluded from the diet.
Cerebral palsy	Is caused by damage to the brain either before birth or early on in life resulting in difficulties with movement, muscle control or posture.
Champix™	A brand name for Varenicline which is used as a smoking cessation drug.
Chiropractor	A practitioner of chiropractics, a science based on the theory that health and disease are related to the function of the nervous system. Chiropractors specialize in diagnosis and treatment or irritants to the nervous system.

Chronic bronchitis	A clinical disorder characterized by excessive mucous secretion in the bronchial tree and manifested by chronic productive cough. It is a long-continued form of bronchitis, often with a more or less marked tendency to recur after stages of quiescence.
Chronic condition	A health condition, such as arthritis or heart disease, that persists over a long period of time.
Chronic fatigue syndrome (CFS)	A condition characterized by unexplained, persistent fatigue lasting at least six months which is not relieved by rest. It results in a substantial reduction in occupational, social and personal activities.
Chronic Obstructive Pulmonary Disease (COPD)	Any disorder marked by a persistent obstruction of bronchial airflow in the lungs.
Clonazepam	Type of sedative obtained from a doctor, often used as an anticonvulsant and muscle relaxant in the treatment of seizures and epilepsy.
CLSC (Centre local de services communautaires)	In Quebec, community-orienteer organizations offering a broad range of health and social services to residents of a specified geographical area.
Cocaine	A powerful drug that stimulates the nervous system. Cocaine is processed into a white powder which people snort or melt and inject. (Common street names: coke, blow, powder, sugar, nose candy, rock, crack, base.)
Codeine	A drug derived from morphine that is used to relieve pain.
Colitis	Inflammation of the colon.
Colonoscopy (Sigmoidoscopy)	A diagnostic procedure in which a flexible lighted tube (endoscope) is inserted through the rectum in order to view the colon for early signs of cancer and other health problems. Tissue or cells may also be removed for microscopic study.

Colorectal cancer	A malignant tumor that arises in the colon and/or rectum.
Community health centre	Community organization that provides a broad range of health and social services as well as health promotion to the population, often composed of an interdisciplinary team of health professionals (medicine, nutrition, mental health, counselling, social work, etc.)
Community-based care	Community-based care includes health care services received outside of a hospital or doctor's office, such as, home nursing care, home-based counselling or therapy, personal care, and community health clinics.
Concerta™	Type of stimulant prescribed by a doctor to help with attention or concentration problems such as ADHD.
Concussion	A violent jar or shock to the brain resulting in disturbance of cerebral function.
Condom	A rubber sheath worn on the penis during sexual intercourse as a contraceptive or to prevent infection. Female condom: a plastic sheath inserted into the vagina prior to intercourse to prevent pregnancy or infection.
Crack	A highly addictive form of cocaine, broken into small pieces and inhaled or smoked.
Crohn's disease	An illness in which there is chronic inflammation involving any part of the gastro-intestinal tract but commonly involves the ileum (part of the small intestine) with scarring and thickening of the bowel wall. It is a type of inflammatory bowel disease.
Crystal meth	Methamphetamine in a solid form that has a stimulating effect on the central and peripheral nervous systems.
Dementia	A type of brain disorder that causes impairment of memory and mental abilities

Demerol™	A brand name of the narcotic painkiller meperidine.
Dental hygienist	A trained professional who provides preventive dental services, and works under the supervision of a dentist. Functions include scaling and polishing teeth, taking x-rays and teaching oral hygiene.
Dental professional	A professional providing dental care, such as a dentist, dental hygienist, orthodontist, denturologist, periodontist and dental therapist.
Dental prosthesis	A replacement for one or more of the teeth or other oral structure, ranging from a single tooth to a complete denture (includes crowns, bridges, dentures, etc.)
Depo-Provera™	A birth control method for women. An injection of hormones that protects against pregnancy for up to 14 weeks.
Depression	A mental state of depressed mood characterized by feelings of sadness, despair and discouragement. It affects the way a person eats and sleeps, the way one feels about oneself and the way one thinks about things. A mood characterized by dejection, psychomotor hypoactivity, thoughts of hopelessness, and guilt.
Dexedrine	Type of stimulant prescribed by a doctor to help with attention or concentration problems such as ADHD.
Diabetes	A chronic disorder of impaired carbohydrate, protein and fat metabolism that may or may not involve insulin. Symptoms include elevated sugar in the urine and the blood, excessive urination, thirst, hunger, weakness, weight loss and itching.
Diagnostic test	Includes MRI, CAT scan or angiography test, x-rays, blood tests, etc., and is requested by a physician to determine or confirm a diagnosis.
Diazepam	Type of sedative obtained from a doctor, often used in the treatment of anxiety disorders and for short-term relief of anxiety symptoms, and as a muscle relaxant, anticonvulsant or antipanic agent.
Dilaudid™	A brand name for hydromorphone hydrochloride which has a narcotic pain relieving effect similar to morphine.

Direct costs	Money spent by the individuals themselves. Another term for this is 'out-of-pocket' costs/expenses. This does not include amounts reimbursed by an insurance plan.
Disability	Having reduced physical or mental abilities that are either congenital or caused by injury or disease.
Drug discount coupon cards	Tool used by brand-name drug manufacturers for helping patients initiate therapy or switch to brand-name medications, at the cost of generic drugs. These cards are distributed either by medical doctors or pharmacies.
DXA (dual-energy x-ray absorptiometry)	Is a test that measures bone mineral density as an indicator of osteoporosis or fracture risk.
Dysthymia	(Dis-thy-mia) A depressed mood experienced for most of the day, nearly every day, and lasting at least two years or longer. Symptoms are similar to depression but are milder and more chronic (reduced energy, insomnia, social withdrawal, pessimism etc.).
Ecstasy (MDMA)	A synthetic, psychoactive (mind-altering) drug with hallucinogenic and amphetamine-like properties. (Other names : Adam, Air ball, Blue Jay, Californian Sunset, Clarity, Diamonds, Dove, E, Earth ball, Empathy, Euphoria, Fantasy, Hug-Drug, Love Doves, Love Drug, M&M, MDM, MDMA, Mitsubishi's, Paradise, Playboys, Venus, Wonder Drug, X, XTC, Yuppie Drug, Zen.)
Electronic cigarette (e-cigarette)	A battery powered vaporizer that simulates tobacco smoking by producing an aerosol that resembles smoke. An e-cigarette releases vapor that may or may not contain nicotine.
Emergency contraceptive pill	(ECP) Also known as the morning after pill. A hormone drug used to prevent pregnancy after unprotected intercourse or the failure of another birth control method.
Emphysema	An abnormal enlargement or distension of the air sacs of the lungs, causing difficulty in breathing. Usually chronic and progressive, the condition is associated with heredity, smoking, and long-standing respiratory ailments such as chronic bronchitis.

Endocrinologist	A physician who specializes in the diagnosis and treatment of conditions related to the endocrine system (e.g. thyroid, adrenal, pituitary glands).
Epilepsy	A brain disorder that causes the brain to send out the wrong signals resulting in seizures.
Eye specialist	A trained professional specializing in the care of the eye (i.e. an ophthalmologist or an optometrist but does not includes an optician).
Family doctor (family physician)	A medical doctor who does not limit his/her practice to a specialty. A family doctor may have a specialty in family medicine.
Fibromyalgia	A condition that usually presents as chronic aching and pain in the muscles, usually of the back and neck, typically accompanied by stiffness, especially in the morning.
Flu shot	A preventative vaccine for influenza viral infections.
FOBT (Fecal Occult Blood Test)	A test to check for blood in stool.
Folic acid	A vitamin of the B complex, used before and during pregnancy to prevent some birth defects.
General practitioner	A medical doctor who does not limit his/her practice to a specialty.
Glaucoma	A disease of the eye characterized by vision loss due to an increase in the pressure of fluid within the eye. This rise in pressure results from a build-up of aqueous fluid and leads to progressive damage to the optic nerve that transmits visual signals to the brain. Over time, glaucoma can lead to a gradual loss in peripheral vision.
Gout	A disease with inflammation of the smaller joints, especially the big toe.
Gynaecologist	A medical doctor who specializes in treating physiological functions and diseases of the female reproductive organs.

H1N1	H1N1 influenza is a contagious respiratory disease. During initial outbreaks it was referred to as swine flu.
Haemoglobin "A-one-C"	An "A-one-C" haemoglobin test measures the average level of blood sugar over a 3-month period.
Hallucinogens	Drugs that affect sensation, thinking, self-awareness, and emotion. Hallucinogens include LSD (acid, microdot), mescaline and peyote (mexc, buttons, cactus), amphetamine variants (PMA, STP, DOB), phencyclidine (PCP, angel dust, hog), phencyclidine analogues (PCE, PCPy, TCP), and others (psilocybin, psilocin).
Hashish	Purified resinous extract of the hemp plant; used as a hallucinogen. Other names for this are cannabis, hash, pot and marijuana.
Hepatitis B/C	Hepatitis B – a severe form of viral hepatitis transmitted in infected blood and other body fluids, causing fever, debility and jaundice. Hepatitis C – a very serious form of hepatitis transmitted through untreated blood and blood products and often resulting in chronic disease.
Heroin	A derivative of morphine, an opiate. Heroin looks like a white or brown powder. It is an opiate painkiller that comes from the poppy plant. Heroin can be smoked in tobacco, heated on aluminum foil and inhaled, injected under the skin or into the muscle/vein. (Common Street Names: H, horse, junk, smack, scag, elephant, stuff, tiger.)
High blood pressure (Hypertension)	Persistently high arterial blood pressure. High blood pressure is also known as Hypertension.
HIV	Human immunodeficiency virus is a retrovirus that causes AIDS (acquired immune deficiency syndrome), a condition marked by severe loss of resistance to infection.
Home care services	Home care services are services provided in the home because of a health condition or limitation in daily activities; they can include for example, nursing care, meal preparation, someone to help you bathe or do housework. It does not include help from family, friends or neighbours.
Home-based counselling (therapy)	Takes place in a client's home rather than a therapist's office. It is provided for people who would have difficulty accessing therapy for reasons such as illness, disability, or lack transport.

Hospital outpatient clinic	A clinic provided by some hospitals that excludes admission into the hospital as a patient and emergency room use. For example, a person may use a hospital outpatient clinic to have a bandage changed every second day. Other examples include ambulatory care, x-ray, geriatric day hospitals, orthopedics and outpatient clinics.
Hydromorph contin (Hydromorphone)	Hydromorphone has a narcotic pain relieving effect similar to morphine.
Hysterectomy	Removal of the uterus, with or without the removal of the ovaries. Removal of only the ovaries is not considered to be a hysterectomy.
Illicit drug use	Can involve the use of licit drugs for purposes other than for health benefits (e.g., overuse, misuse), or the use of licit drugs that haven't been prescribed by a doctor.
Illicit drugs	Drugs that do not have a licit (legal) medical purpose such as cocaine, heroin, hallucinogens, and marijuana (cannabis). *Marijuana - For our survey, users of marijuana for medical purposes will be considered as illicit drug users, even if they have a prescription for it.
Influenza	A highly contagious viral infection causing fever, aches, weakness, and coughing.
Inhaler	A device that for administering medication that the user breathes directly into the lungs.
Insulin	A hormone produced in the pancreas that regulates the amount of glucose in the blood. A lack of insulin causes diabetes. A commercial preparation of insulin is used in the treatment of diabetes.
Intrauterine Device (IUD, coil, loop)	A contraceptive device fitted inside the uterus that physically prevents the implantation of fertilized ova (eggs).

Irritable Bowel Syndrome (IBS)	A gastrointestinal disorder involving an abnormal condition of gut contractions (motility) characterized by abdominal pain, bloating, mucous in stools, and irregular bowel habits with alternating diarrhea and constipation, symptoms that tend to be chronic and to wax and wane over the years.
Lorazepam	Type of sedative obtained from a doctor, often used in the treatment of anxiety disorders and short-term relief of anxiety symptoms and as a sedative-hypnotic agent.
LSD (lysergic acid diethylamide)	A hallucinogenic drug and one of the most potent mood-changing chemicals. LSD can come in the form of coloured tablets, small pieces of blotter paper, clear liquid, capsules, tablets, and thin squares of gelatin. LSD is ingested orally; it is generally a small piece of paper that you eat. It can also be placed in the eyes. (Common Street Names: acid, beast, blue cheer, blue heaven, dot, Lucy in the sky with diamonds, cid (or sid).
Magic mushrooms (psilocybin)	A type of mushroom consumed for its hallucinogenic properties.
Mammography (Mammogram)	Examination of the breast by x-rays in order to detect tumors before they can be felt by palpation.
Mania	A mood disorder associated with abnormally and persistently elevated (high) mood or irritability accompanied by problems associated with sleep, energy, concentration and other emotional or behavioural difficulties
Manic depression	A mood disorder characterized by alternating manic and depressive episodes.
Marijuana	A drug consisting of the dried leaves of the hemp plant smoked or chewed for euphoric effect (see marijuana). It is considered by some to be a soft drug. Marijuana may be used for medicinal purposes, especially for pain control.
Mastectomy	Surgical excision (removal) of all or part of a breast.

Medical specialist	A medical doctor certified to practice in a specified field (e.g. cardiology, oncology, radiology, psychiatry, orthopedics, gynaecology, etc). This does not include optometrists or general practitioners.
Mental Health Professional	A health care practitioner or community services provider who offers services for the purpose of improving an individual's mental health or treating mental illness. Includes professionals such as psychiatrists, psychologists, and marriage or family therapists.
Mescaline	From the mescal buttons of the peyote cactus, mescaline is a hallucinogenic drug that is eaten.
Methamphetamines	An amphetamine derivative with quicker and longer action, that has a stimulating effect on the central and peripheral nervous systems
Microalbumin	A microalbumin test checks urine for the presence of a protein called albumin. Albumin present in the urine can indicate kidney damage from diabetes.
MirenaTM	A hormone-releasing intrauterine birth control device.
Mood disorder (Affective disorder)	A psychological disorder whose essential feature is a disturbance of mood such as depression, bi-polar disorder, manic depression, dysthymia.
Morphine	A pain relieving narcotic drug obtained from opium.
MRI (Magnetic Resonance Imaging)	This is a test in which a magnetic field and radio waves are used to produce pictures of organs and structures inside the body. In many cases, it provides information that cannot be obtained from X-ray tests.

MS Contin™	A brand name for a narcotic pain reliever.
Multiple chemical sensitivity (MSC)	An acquired disorder characterized by recurrent symptoms that occur in response to exposure to many chemically unrelated compounds at doses far below that cause harmful effects in the general population.
Multiple sclerosis (also called MS)	A disease affecting the protective coating of the nerves of the brain and spinal cord.
Myalgic encephalomy-elitis (ME)	A condition causing persistent fatigue that lasts for six months or more.
Nasal allergy	(also allergic rhinitis) An allergic reaction (runny, stuffy nose, itchy, watery eyes) caused by the inhalation (or ingestion) of an allergen such as dust, pollen, mold, etc.
Naturopath	A practitioner of naturopathy (a form of health care that uses diet, herbs, and other natural methods and substances to cure illness) whose goal is to produce a healthy body state, without the use of drugs, by stimulating innate defences.
Neurological disorders	Any disorder of the body's nervous system. There are over 600 neurological disorders, such as Alzheimer's disease, epilepsy, brain tumors.
Nicotine patch (or gum)	A set of stick-on patches or gum that provide a controlled dose of nicotine through the skin, making it possible for smokers to gradually reduce their dependence cigarettes.
Non-emergency surgery	A booked or planned surgery or diagnostic procedure provided on an outpatient or inpatient basis. Does not refer to surgery provided through an admission to the hospital emergency room as a result of, for example, an accident or life-threatening situation.

Nurse practitioner	An advance practice registered nurse who has completed the course work and clinical education beyond that of a registered nurse.
Nursing care	The provision of services essential to or helpful in the promotion, maintenance and restoration of health and well-being (e.g., checking blood pressure, drug care, catheterization, irrigation).
Nutritionist / Dietician	A person who advises on matters of food and nutrition.
Nuva-ring™ (Vaginal ring)	A contraceptive device for women consisting of a plastic, hormone-releasing ring worn in the vagina.
Obsessive-compulsive disorder	A disorder characterized by recurrent obsessions and compulsions, severe enough to interfere with functioning.
Occupational therapist	A specially trained therapist who provides therapy by means of creative activities (e.g., housework, hobbies, job-related) designed to promote recovery and independence (e.g., treating an accident victim who has become disabled in order for them to return to their home and/or job). The respondent should count telephone consultation as well as face-to-face discussion.
Oncologist	A medical doctor who specializes in diagnosing and treating cancer.
Ophthalmologist	A medical doctor specializing in the diagnosis and treatment of eye disease and correction of vision problems.
Opioid	Any compound resembling cocaine or morphine in its addictive properties or physiological effects.
Opium	An addictive drug prepared from the opium poppy and used in medicine as a pain reliever and narcotic.
Optician	A person qualified to prepare, fit and sell eye glasses and contact lenses that have already been prescribed by an optometrist or ophthalmologist.

Optometrist	A health professional specializing in examination of the eyes, measuring vision, and treating certain defects by means of corrective lenses or other methods that do not require license as a physician.
Ortho-Evra™	A hormonal contraceptive skin patch worn by women that prevents pregnancy by suppressing ovulation.
Orthopedist	A medical doctor specializing in the diagnosis and treatment of the bones and joints.
Osteoarthritis	A common degenerative joint disease, often described as “wear and tear” arthritis. Symptoms include pain and stiffness, typically of the knee or hip.
Osteoporosis (brittle bones)	A disorder in which minerals leach out of the bones, making them progressively more porous and fragile. It can be generalized or localized, becomes more common as people get older and may be caused in part by disuse, trauma, or neurological or other disease.
Oxycodone (Oxycontin)	A synthetic painkiller which is similar to morphine in its effects.
Oxy-neo	Is a new formulation of Oxycontin designed to discourage misuse of the drug.
Panic disorder	Can be an acute or chronic illness characterized by recurrent panic attacks and/or a constant and vague sense of anxiety. The symptoms of an attack - among them palpitations, breathlessness, sweating, dizziness, tingling sensations, hot flashes or chills, as well as a sense of impending doom - seem so dire and life-threatening that patients frequently turn up in emergency rooms convinced they are having a heart attack or going insane.
PAP smear test (Pap test)	A screening test for cervical cancer and precancerous cells in the cervix, performed by a medical practitioner. The test involves swabbing the opening of the cervix in order to collect cells.
Parkinson’s disease	A disorder that causes damage to nerve cells in a part of the brain that controls muscle movement.

PCP (phencyclidine)	Also known as angel dust. A dissociative drug that can be smoked, inhaled or injected.
Pediatrician	A medical doctor specializing in the care of children and the treatment of their diseases.
Percocet™	A brand name for a preparation of oxycodone (an opioid pain reliever) and acetaminophen.
Phobia	Any persistent and irrational fear of a specific object, activity, or situation that results in a compelling desire to avoid the feared or phobic stimulus.
Physiotherapist	A therapist specializing in the treatment and prevention of disease, pain, injuries, etc, by physical methods (such as massage, remedial exercise, manipulation, heat treatments, etc) rather than by drugs or surgery.
Pinch (Snuff)	Smokeless tobacco that is held in the mouth between the cheek and gum.
Pneumonia	A bacterial or other infection causing inflammation in one or both lungs.
Powder	A slang term for cocaine.
Primary health care provider	The health professional, such as a family doctor, GP, or nurse practitioner that serves as a patient's first contact with the health care system, usually involving coordination of care and continuity over time. Doctors' offices, walk-in clinics and hospital emergency rooms are examples of places were primary care is obtained.
Private clinic	A health facility, primarily for outpatient care, that offers specialized services. Typically, private clinics offer services with reduced wait times compared to the public system.
Prostate Specific Antigen Test (PSA blood test)	A test used to screen for cancer of the prostate and to monitor treatment.

Psoriasis	A skin disease marked by red scaly patches.
Psychiatrist	A medical doctor specializing in the study, diagnosis, treatment and prevention of mental illness.
Psychologist	A specialist in the study of the human mind and its functions, especially those affecting behaviour.
Rheumatoid arthritis	A chronic disease characterized by stiffness and inflammation of the joints, loss of mobility, weakness and deformity.
Ritalin	Type of stimulant prescribed by a doctor to help with attention or concentration problems, such as ADHD.
Rivotril™	Trademark name for preparations of clonazepam, a type of sedative obtained from a doctor, often used as an anticonvulsant and muscle relaxant in the treatment of seizures and epilepsy.
Salvia	A perennial herb in the mint family that can be smoked, chewed, or brewed for its hallucinogenic properties.
Schizophrenia	A major mental disorder of unknown cause typically characterized by a separation between the thought processes and the emotions, a distortion of reality accompanied by delusions and hallucinations, a fragmentation of the personality, motor disturbances, bizarre behavior, etc., often with no loss of basic intellectual functions.
Scoliosis	A medical condition in which a person's spine is curved from side to side.
Sedative	A drug that tends to calm, soothe, or induce sleep.
Sexually transmitted infections (STI) /Sexually transmitted diseases (STD)	Disease acquired as a result of sexual contact with an infected individual (e.g. venereal diseases, Chlamydia, HIV, AIDS, gonorrhoea, syphilis).

Snow	A slang term for cocaine.
Solvents	Volatile substances that are inhaled as intoxicants, e.g., glue or gasoline sniffing.
Speech therapist	A specially trained therapist who treats children or adults who have difficulty learning to speak or who have lost their power of speech.
Speed	A slang term for an amphetamine drug (also, methamphetamine, crystal meth).
Spina bifida	A birth defect in which the spine does not form completely and the spinal cord is not fully protected usually resulting in nerve damage and some paralysis of the legs.
Spirometry	Is a common lung function test used to diagnose conditions such as asthma or COPD and consists of blowing into a small tube attached to a machine.
Sprain	A joint injury in which some of the fibers of a supporting ligament are ruptured but the continuity of the ligament remains intact.
Steroids	Steroids may refer to drugs used to reduce swelling, pain, and other symptoms of inflammation. Testosterone and its derivatives are steroids with anabolic effects that can be used medically (or illicitly) to build up muscle mass.
Stimulant	A substance that raises levels of physiological or nervous activity in the body.
Stomach ulcers (intestinal ulcers)	Lesions in the wall of the stomach or intestine that won't heal. Such a lesion in the stomach is called a gastric ulcer, whereas a similar lesion in the first part of the small intestine (duodenum) is called a duodenal ulcer. Both gastric and duodenal ulcers are referred to as peptic ulcers.
Stroke	Occurs when the flow of blood to a part of the brain is cut off. This can be due to a blood clot obstructing an artery that leads to the brain (ischemic stroke), or it can be caused by a burst blood vessel bleeding into the brain (hemorrhagic stroke).

Tennis elbow/Tendonitis	A type of tendonitis. It is a painful injury to a tendon in the elbow due to repetitive twisting of the wrist or forearm which causes irritation and inflammation. Any action that involves repetitive twisting of the wrist or forearm such as using a screwdriver can lead to this injury.
Tuberculosis (TB)	An infectious bacterial disease that causes tubercles (small, rounded swellings), especially in the lungs.
Tylenol #1, #2, #3, #4	A pain reliever containing varying levels of acetaminophen and codeine. Tylenol #1 contains 325 mg acetaminophen/8 mg codeine; Tylenol #2 = 300/15; Tylenol #3 = 300/30; Tylenol #4 = 300/60. In Canada, only Tylenol #1 is available without a prescription.
Urinary incontinence (leaky bladder)	The inability to control the urinary function resulting in the unintentional loss of urine.
Urologist	Specialist in urology, a branch of medicine which concerns itself with the urinary tract in both males and females, and with the genital organs in the male.
Valium™	Trademark name for preparations of diazepam, a type of sedative obtained from a doctor, often used in the treatment of anxiety disorders and for short-term relief of anxiety symptoms, and as a muscle relaxant, anticonvulsant or antipanic agent.
VON	(Victorian Order of Nurses) A non-profit, community-based health organization that provides home care for the elderly and chronically ill.
Walk-in clinic	A clinic providing treatment of individual episodes of illness. No appointment is needed. They are often used as alternatives to hospital emergency departments and usually have more extended hours of operation than physicians' offices.
Water-pipe	A device used to smoke tobacco using water vapor. Also known as a hookah, shisha, nargeelay, hubble-bubble or gouza.
Wellbutrin™	A brand name for bupropion, an antidepressant medication which is also used as a smoking cessation medication.

Xanax™	Trademark name for preparations of alprazolam, a type of sedative that can be obtained from a doctor, often used in the treatment of anxiety disorders and panic disorders and for short-term relief of anxiety symptoms.
X-ray	Short wavelength radiation derived from a high energy electrical source. X-rays produce images of body organs on film for diagnostic purposes.
Zyban™ (medication)	A prescription medication that, as part of a comprehensive program from a healthcare professional, can help a person stop smoking.

Appendix B – Quick reference guide / card

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Overview

Introduction

The quick reference card is a reference tool for the CCHS. It provides useful information, for example, the users and uses of the survey data to show the respondent and/or navigation tips to help the interviewer.

Canadian Community Health Survey

BACKGROUND

The Canadian Community Health Survey (CCHS) is a **national health survey** that asks 110,000 Canadians about their health, the factors that affect their health and their use of health care services.

OBJECTIVES

The objectives of this survey are to improve health and health care, by informing decision makers at the **community level**. The CCHS will:

- **support local health units** by providing them with the timely information they need to evaluate existing programs and services and to design new ones suited to their communities; and
- provide more **current, detailed and uniform health information** in every province and territory.

VOLUNTARY NATURE OF THE SURVEY

The CCHS is a **voluntary** survey. Participation is important in each community. If response is low in a particular area, that local health unit will not have access to health information about people in their community.

It's also important that we have representation from all sorts of people, including those who are in good health, if the results of the survey are to be representative.

DATA SHARING AND DATA LINKAGE

Statistics Canada has signed agreements with the provincial and territorial ministries of health, Health Canada and the Public Health Agency of Canada to share data collected in CCHS. It means that the provinces and territories will not have to do their own surveys to collect this information. Data linkage is used for in-depth research; for example:

- To determine whether active people cost less to the health care system, a ministry of health has linked survey information on physical activity with information on doctor billings.
- To find out which medications are most prescribed for conditions like arthritis or angina, a provincial ministry of health has linked CCHS data on chronic conditions to information from a medication insurance program.

Sharing and linking of information you provide occurs only with your permission.

QUESTIONS AND ANSWERS

How can I be sure that my information will be kept confidential?

Statistics Canada is required by law to protect survey information. The *Statistics Act* requires that any information you provide remains strictly confidential. Statistics Canada goes to extraordinary lengths to protect you and all its employees have to take an oath of secrecy.

No personal information which could identify you or your family will be given to anyone without your permission.

Who requires this information and how will it be used?

- **Local health authorities, community groups and social agencies** need this type of information to determine how to best allocate resources in order to improve the health of the people in their community.
- A key use for the information from this survey will be to **develop public education campaigns** primarily aimed at health promotion and disease prevention.
- **Provincial and territorial health ministries and federal government departments** will use this information when making funding decisions, developing priorities and identifying areas of concern for new legislation, policies and programs.
- **Researchers** will have more health information to work with than ever before, in order to monitor, analyse and measure those factors affecting health and to report on them.
- **The media** will use this information to raise awareness about health, an issue of concern to all.

SURVEY TELEPHONE NUMBERS

TTY service (teletype): 1-866-753-7083

R.O. local:

R.O. 1-800

Community Health Centre - CLSC:

Crisis hotline:

Statistics Canada Statistique Canada

Canada

Appendix C – Outcome codes

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Overview

Introduction

This appendix provides an overview of the outcome codes that are used in the full Blaise environment. All the available outcome codes for both CATI and CAPI are listed in ascending numeric order.

A more detailed spreadsheet depicting the routing rules for all outcome codes is available upon request.

Outcome codes

Code	Mode (CAPI/ CATI	Frame (Area/ CCTB)	In Progress	Final	Assignment	Description
00	Both	Both				Not Started
02	CATI	Both		√	Manual & Auto	Incorrect phone number
03	CATI	Both		√	Manual	Phone number not in service
04	CATI	Both	√	√	Manual	Fast busy signal, strange noise, dead silence
05	CATI	Both		√	Manual	Recorded message service
11	Both	Both	√	√	Manual	No one home / no answer
12	Both	Both	√	√	Manual	Regular busy signal
13	Both	Both	√		Manual	Answering machine or service - no message left
14	Both	Both	√		Manual	Answering machine or service - message left
15	Both	Both	√	√	Manual	Call screened blocked forwarded
18	CAPI	Area	√	√	Manual	Interview prevented due to weather conditions
20	Both	Both	√	√	Manual & Auto	Absent for duration of survey
21	Both	Both	√		Manual	Interview requested in other official language
22	Both	Both	√	√	Manual	Language barrier not official language
23	Both	Both	√		Manual	Suspended interrupted
24	Both	Both	√		Manual & Auto	Soft appointment; call-back required
25	Both	Both	√		Manual & Auto	Hard appointment; call-back required
26	Both	Both	√		Manual & Auto	Verification of survey requested
27	Both	Both	√		Manual	Contact with non-household member
28	Both	Both	√		Manual	Request for interview by another interviewer

Code	Mode (CAPI/ CATI	Frame (Area/ CCTB)	In Progress	Final	Assignment	Description
29	CATI	Area	√		Manual	Personal interview requested / required
30	CATI	CCTB	√		Manual	Tracing / research required
36	CATI	CCTB		√	Manual	Unable to trace / research
38	CATI	CCTB	√		Manual	Tracing / research source appointment
39	Both	Area		√	Manual & Auto	Respondent moved - Survey does not trace
40	Both	Both	√	√	Manual	Outside of sample general
42	CATI	CCTB		√	Auto	Indian Reserve
43	Both	Area		√	Manual & Auto	Business
45	Both	Area		√	Manual & Auto	Institution
51	Both	Area		√	Manual	Dwelling Demolished
52	Both	Area		√	Manual	Dwelling Under Construction
53	Both	Area		√	Manual	Dwelling Vacant
55	Both	Area		√	Manual & Auto	Seasonal or Secondary Dwelling
56	Both	Area		√	Manual & Auto	Residents Not Eligible
57	CATI	CCTB		√	Manual & Auto	Moved outside Canada
58	Both	Area		√	Manual	Visitors/representatives of foreign government
59	Both	Both		√	Manual & Auto	Full-time members of the Canadian Armed Forces
60	CATI	CCTB		√	Manual & Auto	Institutionalized
61	Both	Both		√	Manual & Auto	Not eligible due to age
63	Both	Both	√	√	Automatic	Already interviewed for this survey

Code	Mode (CAPI/CATI	Frame (Area/CCTB)	In Progress	Final	Assignment	Description
64	CATI	CCTB		√	Manual & Auto	Deceased
66	Both	Area		√	Manual	Sample Overlap
67	Both	Both		√	Manual	Withdrawn from sample
68	CATI	Area	√	√	Manual & Auto	Transferred to other collection mode
69	CATI	Both		√	Automatic	Cap Reached
70	Both	Both		√	Automatic	Fully completed
71	Both	Both	√	√	Manual	Partially completed
80	Both	Both	√	√	Manual	Refusal
81	Both	Both	√	√	Manual	1st refusal follow-up
82	Both	Both	√	√	Manual	2nd refusal follow-up
85	Both	Both	√		Manual	Hang-up
86	Both	Both	√	√	Manual	Refusal by avoidance
90	Both	Both	√	√	Manual	Unusual / Special circumstances
91	CAPI	Area	√	√	Manual	Threat to safety
92	Both	Both	√	√	Manual	Interview prevented due to respondent's mental or physical condition
93	CATI	CCTB	√		Manual	Letter sent; soft appointment follow-up required
95	Both	Both	√	√	Manual	Technical difficulty.

Code	Description
00	**Not started** - The survey case or component was not started by the interviewer. This code is assigned automatically by the computer system.
02	**Incorrect phone number** - This code is assigned when the phone number has been dialled correctly but is not correct for the household, or the number has changed as indicated by a recorded message or telephone operator.
03	**Phone number not in service** - This code is assigned when a recording or telephone operator informs the interviewer that the number dialled is no longer in service.
04	**Fast busy signal, strange noise, dead silence** - This code is assigned when the interviewer encounters a fast busy signal, a strange noise; for example, a click, buzzing or beep, or dead silence which may indicate technical problems with the line. Strange noises can also be produced from a special telephone system used by hearing impaired persons or by fax machines.
05	**Recorded message service** - This code is assigned when the interviewer encounters a recorded message service; for example, the weather service or the time of day. This does not include voice mail or answering machine messages.
11	**No one home/ no answer** - This code is assigned when dialing the telephone number produces ringing but no one answers, or for a personal visit, when no one answers the door.
12	**Regular busy signal** - This code is assigned when dialing the telephone number produces a regular busy signal.
13	**Answering machine or service no message left** - This code is assigned when the call is answered by a telephone answering service or an electronic answering machine that records messages and no message is left.
14	**Answering machine or service message left** - This code is assigned when the call is answered by a telephone answering service or an electronic answering machine that records messages and a message is left.
15	**Call screened blocked forwarded** - This code is assigned when the interviewer is unable to reach the respondent because the telephone call is screened, blocked, or forwarded to a recorded message, and the interviewer cannot leave a message. For example, the call is answered with a recorded message such as "The party you have reached has chosen not to accept any calls at this time."
18	**Interview prevented due to weather conditions** - This code is assigned when the Interviewer cannot contact the respondent because weather prevents it; for example, the dwelling is snowed in during the entire collection cycle, no phone service during the entire collection cycle due to a storm, etc.
20	**Absent for the duration of the survey** - This code is assigned when contact has been made, but the interview has not taken place because the respondent is absent for the remainder of the survey period, for example, gone on vacation.

Code	Description
22	**Language barrier not official language** - This code is assigned when the interview cannot take place because the respondent does not speak either of the official languages and the interviewer cannot communicate in the language of the respondent.
23	**Suspended interrupted** - This code is assigned when the interview or Initial Listing or Listing Maintenance was started and then interrupted for some reason. The interviewer must make a call-back to the respondent or resume Initial Listing or Listing Maintenance at another time.
24	**Soft appointment; call-back required** - This code is assigned when the Interviewer contacts the respondent but the respondent is unable at the time to do the interview and asks the Interviewer to call back. OR The interviewer makes contact with someone other than an eligible respondent and that person asks the interviewer to call back. The call-back time is an approximate or general period of time rather than a firm appointment time.
25	**Hard appointment; call-back required** - This code is assigned when the Interviewer contacts the respondent but the respondent is unable at the time to do the interview and asks the Interviewer to call back at a specific date and time.
26	**Verification of survey requested** - This code is assigned when the respondent will not participate in the survey until he or she has verified the survey's authenticity or the interviewer's identity.
27	**Contact with ineligible or non-household member** - This code is assigned when the interviewer makes contact with a non-household member; for example a baby sitter, or an ineligible member of the household; follow-up is required.
28	**Request for interview by another interviewer** - This code is assigned when the respondent requests that another interviewer conduct the interview.
29	**Personal interview requested / required** - This code is assigned when the respondent requests that the Interviewer visit his or her home because they do not want to be interviewed by phone, or when a case needs to be returned to the field for the next collection cycle with the household information intact.
30	**Tracing / research required** - This code is assigned when the Interviewer cannot contact the respondent because he or she has moved or the telephone number has changed. Tracing must be initiated to find a new address or telephone number for the respondent. It is also assigned when some tracing/research has taken place, but further tracing/research is required.
36	**Unable to trace / research** - This code is assigned when all sources have been exhausted and tracing / research has been unsuccessful.
38	**Tracing / research source appointment** - This code is assigned when an appointment is made to contact a source at a later time.

Code	Description
39	**Respondent moved** - This code is assigned when the respondent has moved from the selected dwelling (or household) and tracing is not conducted for the survey.
40	**Outside of sample** - This code is assigned for any case which falls outside of the sample criteria when a more specific code is not required or when the reason is not covered by the specific codes in this category. NOTE: Every time this code is used, the reason must be noted.
42	**Indian reserve** - This code is assigned when the sample unit is situated on an Indian reserve and Indian reserves are out of scope for the survey.
43	**Business** - This code is assigned when the sample unit is a business and businesses are out of scope for the survey; for example, it is a household survey.
45	**Institution** - This code is used when the sample unit is an institution and institutions are out of scope for the survey (for example, it is a household survey that includes only private households).
51	**Dwelling demolished** - This code assigned in the following situations: • Dwelling torn down • Dwelling converted into business premises and is no longer a place of residence • House, house trailer or mobile home removed from its lot
52	**Dwelling under construction/renovation** - This code is assigned when the dwelling is under construction or renovation and therefore is not occupied.
53	**Dwelling vacant -** This code is assigned when the dwelling has no occupants. A dwelling may be vacant even if it is furnished, since some dwellings are rented fully furnished.
55	**Seasonal or secondary dwelling** - This code is assigned for dwellings that are not the usual place of residence of the household, for example, summer cottages, ski chalets, hunting camps, etc
56	**Residents not eligible** - This code is assigned when the residents of a dwelling do not satisfy the conditions for inclusion in the survey and a more specific code is not required or the specific codes below do not apply.
57	**Moved outside Canada** - This code is assigned when one or more residents of the dwelling have moved and are no longer residents of Canada and the survey stipulates that such residents are out of scope.
58	**Visitors/representatives of foreign government** - This code is assigned when one or more residents of the dwelling have a usual place of residence in another country, for example, foreign government representatives, visa students etc. and the survey stipulates that such residents are out of scope.
59	**Full-time members of the Canadian Armed Forces** - This code is assigned when one or more residents of the dwelling are full-time members of the Canadian Armed Forces and the survey stipulates that such residents are out of scope.

Code	Description
60	**Institutionalized** - This code is assigned when one or more residents of the dwelling have been institutionalized and the survey stipulates that such residents are out of scope.
61	**Not eligible due to age** - This code is assigned when one or more residents of the dwelling are outside the age criteria of the survey and are therefore out of scope for the survey.
63	**Already interviewed for this survey** - This code is assigned for such cases as residents of dwellings with two different telephone numbers and both numbers are selected for the survey or a couple who has joint custody of their child and the child is selected at both households.
64	**Deceased** - The selected person has died.
66	**Sample overlap** - This code is assigned by supervisors only. It is assigned to remove sample units when: • due to a sample redesign, the same dwelling is selected within two years of rotating out of the LFS sample and/or • a dwelling is selected for more than one survey at the same time
67	**Withdrawn from sample** - This code is assigned by supervisors only. Generally, this code reflects administrative or authoritative decisions having been made regarding a unit that has difficult or unusual circumstances.
68	**Transferred to other collection mode** - This code is assigned in CATI when a case is transferred to another collection mode: • LFS cases that moved from CATI to CAPI are removed from the active CATI sample and are treated as 'voids' in DIPP or • All other social surveys treat EQ cases in-progress until information is completed.
69	**Cap Reached** - This code is assigned automatically by the system when the maximum number of attempts has been made.
70	**Fully complete** - This code is assigned automatically by the computer when an interview or component or Listing Maintenance has been fully completed.
71	**Partially completed** - This code is assigned when an interview or component or Listing Maintenance was not fully completed.
80	**Refusal** - This code is assigned when the respondent has refused to provide some or all information relating to the survey.
81	**1st refusal follow-up** - This code is assigned when upon the first refusal follow-up attempt; the respondent has refused to provide some or all information relating to the survey.
82	**2nd refusal follow-up** - This code is assigned when upon the second and final refusal follow-up attempt; the respondent has refused to provide some or all of the information relating to the survey.

Code	Description
83	**Refusal without confirmation** - This code is assigned when an interviewer has made contact with a household member but he/she refuses to participate in the survey and has not confirmed whether the interviewer has reached the correct dwelling or respondent.
85	**Won't open door / Hang Up -** In a CAPI situation, this code is used when an interviewer can tell someone is home but won't answer the door. The case is treated as a no-contact. / CATI This code is assigned when a household member has hung-up the phone before the interviewer could complete the introduction and without stating that they are refusing to complete the survey.
86	**Refusal by avoidance -** This code is assigned when a case has been coded 85 on previous attempts, and should now be treated as a refusal and routed to the refusal group
90	**Unusual Special circumstances -** This code is assigned when, in Age of Selected Respondent (ANC) module, the selected respondent is specified being less than 12 years old. The case Auto Codes 90 and calls the exit block. This is an error in demographics and should be added to the permanent notes with 3 asterisks preceding it. This code can also be assigned when unforeseen circumstances such as illness or death of a household member/employee, recovery from a fire or flood, quarantined household, etc. prevent the interviewer from conducting an interview or Listing Maintenance during the collection period.
91	**Threat to safety -** This code is assigned when a dangerous situation is encountered where there is a threat to safety; for example, a vicious dog or other animal, or a violent neighborhood.
92	**Interview prevented due to respondent's mental or physical condition -** The respondent is unable to take part in the interview because he/she is disabled e.g., Alzheimer's disease, Down's Syndrome, Attention Deficit Disorder, hearing impairment, etc.
93	**Letter sent; soft appointment follow-up required -** This code is assigned when a letter is sent to a respondent and a call back is required, i.e. follow-up for a refusal letter or no contact letter.
95	**Technical Difficulty -** The interviewer is unable to proceed due to technical difficulties with the laptop.

Appendix D – Modules by province / territory

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Modules by province / territory	D-2

Overview

Introduction This appendix shows all the modules (with the type: core, optional, theme) for each province / territory. They are listed in the order they will appear in the application (without Entry and Exit components). Please refer to Chapter 3, Section C for alphabetical listing.

Modules by province / territory

Newfoundland and Labrador

Order	Type	Acronym	Module name
1	Core	GR	Proxy interview
2	Core	ANC1	Age of Respondent
3	Core	REL	Relationship
4	Core	MAC	Main Activity
5	Core	GEN	General Health
6	Core	HWT	Height and Weight - Self Reported
7	Core	CCC	Chronic Conditions
9	Optional	ADL	Activities of Daily Living
14	Optional	DEN	Dental Care
15	Optional	CIH	Changes Made to Improve Health
17	Core	FVC	Fruit and Vegetable Consumption
20	Core	SMK	Smoking
22	Optional	SCA	Smoking Cessation Methods
26	Core	ETS	Exposure to second-hand smoke
27	Core	ALC	Alcohol Consumption
28	Theme	ALW	Alcohol - Past Week
30	Theme	DRM	Drug methods
34	Core	PAA	Physical Activities - Adults
35	Core	PAY	Physical Activities- Children
39	Theme	SXB	Sexual Behaviors
40	Core	MEX	Maternal Experiences
44	Core	FLU	Flu Shots
46	Optional	BPC	Blood Pressure Check
47	Optional	PAP	PAP Test
48	Optional	MAM	Mammography
49	Optional	PSA	Prostate Cancer Screening
50	Optional	CCS	Colorectal screening
53	Optional	EYX	Eye examinations

Newfoundland and Labrador (continued)

Order	Type	Acronym	Module name
56	Theme	CMH	Consultations with mental HP
60	Optional	DEP	Depression
61	Theme	SUI	Suicidal Thoughts and Attempts
62	Optional	WST	Work Stress
65	Core	PHC	Primary Health Care
66	Core	CHP	Consultation with Health Professionals
70	Theme	PEX	Patient Experiences
73	Theme	HMC	Home Care
74	Optional	UCN	Unmet care needs
75	Core	LBF	Labour Force
77	Core	SDC	Demographics
78	Core	PMK	Person most knowledgeable about the household situation
79	Theme	INS	Insurance Coverage
81	Core	INC	Income
82	Core	ADM	Administration

Prince Edward Island

Order	Type	Acronym	Module name
1	Core	GR	Proxy interview
2	Core	ANC1	Age of Respondent
3	Core	REL	Relationship
4	Core	MAC	Main Activity
5	Core	GEN	General Health
6	Core	HWT	Height and Weight - Self Reported
7	Core	CCC	Chronic Conditions
9	Optional	ADL	Activities of Daily Living
10	Optional	INJ	Injuries
15	Optional	CIH	Changes Made to Improve Health
17	Core	FVC	Fruit and Vegetable Consumption
18	Optional	FDC	Food Choices
19	Theme	FGU	Canada's Food Guide Use
20	Core	SMK	Smoking
26	Core	ETS	Exposure to second-hand smoke
27	Core	ALC	Alcohol Consumption
28	Theme	ALW	Alcohol - Past Week
29	Optional	MED	Medication Use
30	Theme	DRM	Drug methods
34	Core	PAA	Physical Activities - Adults
35	Core	PAY	Physical Activities- Children
38	Optional	SAC	Sedentary Activities
39	Theme	SXB	Sexual Behaviors
40	Core	MEX	Maternal Experiences
44	Core	FLU	Flu Shots
46	Optional	BPC	Blood Pressure Check
47	Optional	PAP	PAP Test
50	Optional	CCS	Colorectal screening
56	Theme	CMH	Consultations with mental HP

Prince Edward Island (continued)

Order	Type	Acronym	Module name
60	Optional	DEP	Depression
61	Theme	SUI	Suicidal Thoughts and Attempts
64	Optional	SPS	Social Provisions (social support)
65	Core	PHC	Primary Health Care
66	Core	CHP	Consultation with Health Professionals
70	Theme	PEX	Patient Experiences
73	Theme	HMC	Home Care
75	Core	LBF	Labour Force
77	Core	SDC	Demographics
78	Core	PMK	Person most knowledgeable about the household situation
79	Theme	INS	Insurance Coverage
80	Optional	FSC	Food Security
81	Core	INC	Income
82	Core	ADM	Administration

Nova Scotia

Order	Type	Acronym	Module name
1	Core	GR	Proxy interview
2	Core	ANC1	Age of Respondent
3	Core	REL	Relationship
4	Core	MAC	Main Activity
5	Core	GEN	General Health
6	Core	HWT	Height and Weight - Self Reported
7	Core	CCC	Chronic Conditions
9	Optional	ADL	Activities of Daily Living
17	Core	FVC	Fruit and Vegetable Consumption
20	Core	SMK	Smoking
23	Optional	TAL	Tobacco Products - Alternatives
26	Core	ETS	Exposure to second-hand smoke
27	Core	ALC	Alcohol Consumption
28	Theme	ALW	Alcohol - Past Week
30	Theme	DRM	Drug methods
34	Core	PAA	Physical Activities - Adults
35	Core	PAY	Physical Activities- Children
39	Theme	SXB	Sexual Behaviors
40	Core	MEX	Maternal Experiences
42	Optional	MXA	Alcohol during pregnancy
44	Core	FLU	Flu Shots
45	Optional	PCU	Physical Checkups
46	Optional	BPC	Blood Pressure Check
49	Optional	PSA	Prostate Cancer Screening
56	Theme	CMH	Consultations with mental HP

Nova Scotia (continued)

Order	Type	Acronym	Module name
60	Optional	DEP	Depression
61	Theme	SUI	Suicidal Thoughts and Attempts
65	Core	PHC	Primary Health Care
66	Core	CHP	Consultation with Health Professionals
70	Theme	PEX	Patient Experiences
71	Optional	ACC	Access to health care system
73	Theme	HMC	Home Care
74	Optional	UCN	Unmet care needs
75	Core	LBF	Labour Force
77	Core	SDC	Demographics
78	Core	PMK	Person most knowledgeable about the household situation
79	Theme	INS	Insurance Coverage
80	Optional	FSC	Food Security
81	Core	INC	Income
82	Core	ADM	Administration

New Brunswick

Order	Type	Acronym	Module name
1	Core	GR	Proxy interview
2	Core	ANC1	Age of Respondent
3	Core	REL	Relationship
4	Core	MAC	Main Activity
5	Core	GEN	General Health
6	Core	HWT	Height and Weight - Self Reported
7	Core	CCC	Chronic Conditions
9	Optional	ADL	Activities of Daily Living
17	Core	FVC	Fruit and Vegetable Consumption
20	Core	SMK	Smoking
26	Core	ETS	Exposure to second-hand smoke
27	Core	ALC	Alcohol Consumption
28	Theme	ALW	Alcohol - Past Week
30	Theme	DRM	Drug methods
34	Core	PAA	Physical Activities - Adults
35	Core	PAY	Physical Activities- Children
36	Optional	UPE	Use of Protective Equipment
37	Optional	SSB	Sun Safety Behaviours
39	Theme	SXB	Sexual Behaviors
40	Core	MEX	Maternal Experiences
44	Core	FLU	Flu Shots
47	Optional	PAP	PAP Test
48	Optional	MAM	Mammography
52	Optional	DIA	Diabetes Care
56	Theme	CMH	Consultations with mental HP

New Brunswick (continued)

Order	Type	Acronym	Module name
60	Optional	DEP	Depression
61	Theme	SUI	Suicidal Thoughts and Attempts
65	Core	PHC	Primary Health Care
66	Core	CHP	Consultation with Health Professionals
70	Theme	PEX	Patient Experiences
71	Optional	ACC	Access to health care system
72	Optional	WTM	Waiting Times
73	Theme	HMC	Home Care
75	Core	LBF	Labour Force
77	Core	SDC	Demographics
78	Core	PMK	Person most knowledgeable about the household situation
79	Theme	INS	Insurance Coverage
80	Optional	FSC	Food Security
81	Core	INC	Income
82	Core	ADM	Administration

Quebec

Order	Type	Acronym	Module name
1	Core	GR	Proxy interview
2	Core	ANC1	Age of Respondent
3	Core	REL	Relationship
4	Core	MAC	Main Activity
5	Core	GEN	General Health
6	Core	HWT	Height and Weight – Self Reported
7	Core	CCC	Chronic Conditions
17	Core	FVC	Fruit and Vegetable Consumption
18	Optional	FDC	Food Choices
20	Core	SMK	Smoking
24	Optional	NDE	Nicotine Dependence
26	Core	ETS	Exposure to second-hand smoke
27	Core	ALC	Alcohol Consumption
28	Theme	ALW	Alcohol – Past Week
30	Theme	DRM	Drug methods
34	Core	PAA	Physical Activities – Adults
35	Core	PAY	Physical Activities- Children
37	Optional	SSB	Sun Safety Behaviors
39	Theme	SXB	Sexual Behaviors
40	Core	MEX	Maternal Experiences
44	Core	FLU	Flu Shots
45	Optional	PCU	Physical Checkups
48	Optional	MAM	Mammography
56	Theme	CMH	Consultations with mental HP

Quebec (continued)

Order	Type	Acronym	Module name
61	Theme	SUI	Suicidal Thoughts and Attempts
65	Core	PHC	Primary Health Care
66	Core	CHP	Consultation with Health Professionals
70	Theme	PEX	Patient Experiences
71	Optional	ACC	Access to health care system
73	Theme	HMC	Home Care
75	Core	LBF	Labour Force
77	Core	SDC	Demographics
78	Core	PMK	Person most knowledgeable about the household situation
79	Theme	INS	Insurance Coverage
80	Optional	FSC	Food Security
81	Core	INC	Income
82	Core	ADM	Administration

Ontario

Order	Type	Acronym	Module name
1	Core	GR	Proxy interview
2	Core	ANC1	Age of Respondent
3	Core	REL	Relationship
4	Core	MAC	Main Activity
5	Core	GEN	General Health
6	Core	HWT	Height and Weight – Self Reported
7	Core	CCC	Chronic Conditions
16	Optional	SLP	Sleep
17	Core	FVC	Fruit and Vegetable Consumption
19	Optional	FGU	Canada's Food Guide Use
20	Core	SMK	Smoking
23	Optional	TAL	Tobacco Products – Alternatives
26	Core	ETS	Exposure to second-hand smoke
27	Core	ALC	Alcohol Consumption
28	Theme	ALW	Alcohol – Past Week
30	Theme	DRM	Drug methods
31	Optional	DRG	Illicit Drugs
34	Core	PAA	Physical Activities – Adults
35	Core	PAY	Physical Activities- Children
37	Optional	SSB	Sun Safety Behaviors
38	Optional	SAC	Sedentary Activities
39	Theme	SXB	Sexual Behaviors
40	Core	MEX	Maternal Experiences
44	Core	FLU	Flu Shots
49	Optional	PSA	Prostate Cancer Screening
52	Optional	DIA	Diabetes Care
56	Theme	CMH	Consultations with mental HP

Ontario (continued)

Order	Type	Acronym	Module name
60	Optional	DEP	Depression
61	Theme	SUI	Suicidal Thoughts and Attempts
65	Core	PHC	Primary Health Care
66	Core	CHP	Consultation with Health Professionals
70	Theme	PEX	Patient Experiences
73	Theme	HMC	Home Care
75	Core	LBF	Labour Force
77	Core	SDC	Demographics
78	Core	PMK	Person most knowledgeable about the household situation
79	Theme	INS	Insurance Coverage
81	Core	INC	Income
82	Core	ADM	Administration

Manitoba

Order	Type	Acronym	Module name
1	Core	GR	Proxy interview
2	Core	ANC1	Age of Respondent
3	Core	REL	Relationship
4	Core	MAC	Main Activity
5	Core	GEN	General Health
6	Core	HWT	Height and Weight - Self Reported
7	Core	CCC	Chronic Conditions
15	Optional	CIH	Changes Made to Improve Health
16	Optional	SLP	Sleep
17	Core	FVC	Fruit and Vegetable Consumption
20	Core	SMK	Smoking
26	Core	ETS	Exposure to second-hand smoke
27	Core	ALC	Alcohol Consumption
28	Theme	ALW	Alcohol - Past Week
30	Theme	DRM	Drug methods
34	Core	PAA	Physical Activities - Adults
35	Core	PAY	Physical Activities- Children
37	Optional	SSB	Sun Safety Behaviors
39	Theme	SXB	Sexual Behaviors
40	Core	MEX	Maternal Experiences
44	Core	FLU	Flu Shots
56	Theme	CMH	Consultations with mental HP

Manitoba (continued)

Order	Type	Acronym	Module name
60	Optional	DEP	Depression
61	Theme	SUI	Suicidal Thoughts and Attempts
62	Optional	WST	Work Stress
65	Core	PHC	Primary Health Care
66	Core	CHP	Consultation with Health Professionals
68	Optional	PSC	Patient Satisfaction - Community-based Care
70	Theme	PEX	Patient Experiences
73	Theme	HMC	Home Care
75	Core	LBF	Labour Force
77	Core	SDC	Demographics
78	Core	PMK	Person most knowledgeable about the household situation
79	Theme	INS	Insurance Coverage
80	Optional	FSC	Food Security
81	Core	INC	Income
82	Core	ADM	Administration

Saskatchewan

Order	Type	Acronym	Module name
1	Core	GR	Proxy interview
2	Core	ANC1	Age of Respondent
3	Core	REL	Relationship
4	Core	MAC	Main Activity
5	Core	GEN	General Health
6	Core	HWT	Height and Weight - Self Reported
7	Core	CCC	Chronic Conditions
15	Optional	CIH	Changes Made to Improve Health
16	Optional	SLP	Sleep
17	Core	FVC	Fruit and Vegetable Consumption
20	Core	SMK	Smoking
26	Core	ETS	Exposure to second-hand smoke
27	Core	ALC	Alcohol Consumption
28	Theme	ALW	Alcohol - Past Week
30	Theme	DRM	Drug methods
34	Core	PAA	Physical Activities - Adults
35	Core	PAY	Physical Activities- Children
37	Optional	SSB	Sun Safety Behaviors
39	Theme	SXB	Sexual Behaviors
40	Core	MEX	Maternal Experiences
44	Core	FLU	Flu Shots
56	Theme	CMH	Consultations with mental HP

Saskatchewan (continued)

Order	Type	Acronym	Module name
61	Theme	SUI	Suicidal Thoughts and Attempts
62	Optional	WST	Work Stress
65	Core	PHC	Primary Health Care
66	Core	CHP	Consultation with Health Professionals
68	Optional	PSC	Patient Satisfaction - Community-based Care
70	Theme	PEX	Patient Experiences
73	Theme	HMC	Home Care
75	Core	LBF	Labour Force
77	Core	SDC	Demographics
78	Core	PMK	Person most knowledgeable about the household situation
79	Theme	INS	Insurance Coverage
80	Optional	FSC	Food Security
81	Core	INC	Income
82	Core	ADM	Administration

Alberta

Order	Type	Acronym	Module name
1	Core	GR	Proxy interview
2	Core	ANC1	Age of Respondent
3	Core	REL	Relationship
4	Core	MAC	Main Activity
5	Core	GEN	General Health
6	Core	HWT	Height and Weight - Self Reported
7	Core	CCC	Chronic Conditions
14	Optional	DEN	Dental Care
17	Core	FVC	Fruit and Vegetable Consumption
20	Core	SMK	Smoking
26	Core	ETS	Exposure to second-hand smoke
27	Core	ALC	Alcohol Consumption
28	Theme	ALW	Alcohol - Past Week
29	Optional	MED	Medication Use
30	Theme	DRM	Drug methods
31	Optional	DRG	Illicit Drugs
34	Core	PAA	Physical Activities - Adults
35	Core	PAY	Physical Activities- Children
36	Optional	UPE	Use of Protective Equipment
39	Theme	SXB	Sexual Behaviors
40	Core	MEX	Maternal Experiences
43	Optional	DRV	Driving safety
44	Core	FLU	Flu Shots
46	Optional	BPC	Blood Pressure Check
47	Optional	PAP	PAP Test
48	Optional	MAM	Mammography
56	Theme	CMH	Consultations with mental HP
57	Optional	SWL	Satisfaction with life

Alberta (continued)

Order	Type	Acronym	Module name
61	Theme	SUI	Suicidal Thoughts and Attempts
63	Theme	STS	Sources of Stress
64	Theme	SPS	Social Provisions (social support)
65	Core	PHC	Primary Health Care
66	Core	CHP	Consultation with Health Professionals
70	Theme	PEX	Patient Experiences
73	Theme	HMC	Home Care
74	Optional	UCN	Unmet care needs
75	Core	LBF	Labour Force
77	Core	SDC	Demographics
78	Core	PMK	Person most knowledgeable about the household situation
79	Theme	INS	Insurance Coverage
80	Optional	FSC	Food Security
81	Core	INC	Income
82	Core	ADM	Administration

British Columbia

Order	Type	Acronym	Module name
1	Core	GR	Proxy interview
2	Core	ANC1	Age of Respondent
3	Core	REL	Relationship
4	Core	MAC	Main Activity
5	Core	GEN	General Health
6	Core	HWT	Height and Weight - Self Reported
7	Core	CCC	Chronic Conditions
17	Core	FVC	Fruit and Vegetable Consumption
20	Core	SMK	Smoking
23	Optional	TAL	Tobacco Products - Alternatives
26	Core	ETS	Exposure to second-hand smoke
27	Core	ALC	Alcohol Consumption
28	Theme	ALW	Alcohol - Past Week
30	Theme	DRM	Drug methods
34	Core	PAA	Physical Activities - Adults
35	Core	PAY	Physical Activities- Children
38	Optional	SAC	Sedentary Activities
39	Theme	SXB	Sexual Behaviors
40	Core	MEX	Maternal Experiences
41	Optional	MXS	Smoking during pregnancy
44	Core	FLU	Flu Shots
54	Optional	STG	Stigma towards depression
56	Theme	CMH	Consultations with mental HP
58	Optional	SFE	Self-esteem

British Columbia (continued)

Order	Type	Acronym	Module name
60	Optional	DEP	Depression
61	Theme	SUI	Suicidal Thoughts and Attempts
65	Core	PHC	Primary Health Care
66	Optional	MDA	Medical Doctor Attachment
67	Core	CHP	Consultation with Health Professionals
70	Theme	PEX	Patient Experiences
73	Theme	HMC	Home Care
75	Core	LBF	Labour Force
76	Optional	LOP	Loss of Productivity
77	Core	SDC	Demographics
78	Core	PMK	Person most knowledgeable about the household situation
79	Theme	INS	Insurance Coverage
80	Optional	FSC	Food Security
81	Core	INC	Income
82	Core	ADM	Administration

Yukon

Order	Type	Acronym	Module name
1	Core	GR	Proxy interview
2	Core	ANC1	Age of Respondent
3	Core	REL	Relationship
4	Core	MAC	Main Activity
5	Core	GEN	General Health
6	Core	HWT	Height and Weight - Self Reported
7	Core	CCC	Chronic Conditions
17	Core	FVC	Fruit and Vegetable Consumption
20	Core	SMK	Smoking
21	Optional	SCH	Smoking - Stages of Change
26	Core	ETS	Exposure to second-hand smoke
27	Core	ALC	Alcohol Consumption
28	Theme	ALW	Alcohol - Past Week
29	Optional	MED	Medication Use
30	Theme	DRM	Drug methods
31	Optional	DRG	Illicit Drugs
34	Core	PAA	Physical Activities - Adults
35	Core	PAY	Physical Activities- Children
39	Theme	SXB	Sexual Behaviors
40	Core	MEX	Maternal Experiences
44	Core	FLU	Flu Shots
51	Optional	SPI	Spirometry
56	Theme	CMH	Consultations with mental HP

Yukon (continued)

Order	Type	Acronym	Module name
61	Theme	SUI	Suicidal Thoughts and Attempts
64	Theme	SPS	Social Provisions (social support)
65	Core	PHC	Primary Health Care
66	Core	CHP	Consultation with Health Professionals
70	Theme	PEX	Patient Experiences
71	Optional	ACC	Access to health care system
73	Theme	HMC	Home Care
75	Core	LBF	Labour Force
77	Core	SDC	Demographics
78	Core	PMK	Person most knowledgeable about the household situation
79	Theme	INS	Insurance Coverage
81	Core	INC	Income
82	Core	ADM	Administration

Northwest Territories

Order	Type	Acronym	Module name
1	Core	GR	Proxy interview
2	Core	ANC1	Age of Respondent
3	Core	REL	Relationship
4	Core	MAC	Main Activity
5	Core	GEN	General Health
6	Core	HWT	Height and Weight - Self Reported
7	Core	CCC	Chronic Conditions
15	Optional	CIH	Changes Made to Improve Health
17	Core	FVC	Fruit and Vegetable Consumption
20	Core	SMK	Smoking
23	Optional	TAL	Tobacco Products - Alternatives
26	Core	ETS	Exposure to second-hand smoke
27	Core	ALC	Alcohol Consumption
28	Theme	ALW	Alcohol - Past Week
29	Optional	MED	Medication Use
30	Theme	DRM	Drug methods
34	Core	PAA	Physical Activities - Adults
35	Core	PAY	Physical Activities- Children
39	Theme	SXB	Sexual Behaviors
40	Core	MEX	Maternal Experiences
44	Core	FLU	Flu Shots
52	Optional	DIA	Diabetes Care
54	Optional	STG	Stigma towards depression
56	Theme	CMH	Consultations with mental HP
58	Optional	SFE	Self-esteem

Northwest Territories (continued)

Order	Type	Acronym	Module name
60	Optional	DEP	Depression
61	Theme	SUI	Suicidal Thoughts and Attempts
65	Core	PHC	Primary Health Care
66	Core	CHP	Consultation with Health Professionals
70	Theme	PEX	Patient Experiences
73	Theme	HMC	Home Care
74	Optional	UCN	Unmet care needs
75	Core	LBF	Labour Force
77	Core	SDC	Demographics
78	Core	PMK	Person most knowledgeable about the household situation
79	Theme	INS	Insurance Coverage
81	Core	INC	Income
82	Core	ADM	Administration

Nunavut

Order	Type	Acronym	Module name
1	Core	GR	Proxy interview
2	Core	ANC1	Age of Respondent
3	Core	REL	Relationship
4	Core	MAC	Main Activity
5	Core	GEN	General Health
6	Core	HWT	Height and Weight - Self Reported
7	Core	CCC	Chronic Conditions
13	Optional	OHT	Oral Health
14	Optional	DEN	Dental Care
17	Core	FVC	Fruit and Vegetable Consumption
18	Optional	FDC	Food Choices
20	Core	SMK	Smoking
22	Optional	SCA	Smoking Cessation Methods
26	Core	ETS	Exposure to second-hand smoke
27	Core	ALC	Alcohol Consumption
28	Theme	ALW	Alcohol - Past Week
30	Theme	DRM	Drug methods
31	Optional	DRG	Illicit Drugs
32	Optional	CPG	Problem Gambling
34	Core	PAA	Physical Activities - Adults
35	Core	PAY	Physical Activities- Children
39	Theme	SXB	Sexual Behaviors
40	Core	MEX	Maternal Experiences
41	Optional	MXS	Smoking during pregnancy
44	Core	FLU	Flu Shots
47	Optional	PAP	PAP Test
56	Theme	CMH	Consultations with mental HP

Nunavut (continued)

Order	Type	Acronym	Module name
61	Theme	SUI	Suicidal Thoughts and Attempts
65	Core	PHC	Primary Health Care
66	Core	CHP	Consultation with Health Professionals
68	Optional	PSC	Patient Satisfaction - Community-based Care
70	Theme	PEX	Patient Experiences
73	Theme	HMC	Home Care
75	Core	LBF	Labour Force
77	Core	SDC	Demographics
78	Core	PMK	Person most knowledgeable about the household situation
79	Theme	INS	Insurance Coverage
80	Optional	FSC	Food Security
81	Core	INC	Income
82	Core	ADM	Administration

Appendix E – Address in scope chart (18+)

In this appendix This appendix contains:

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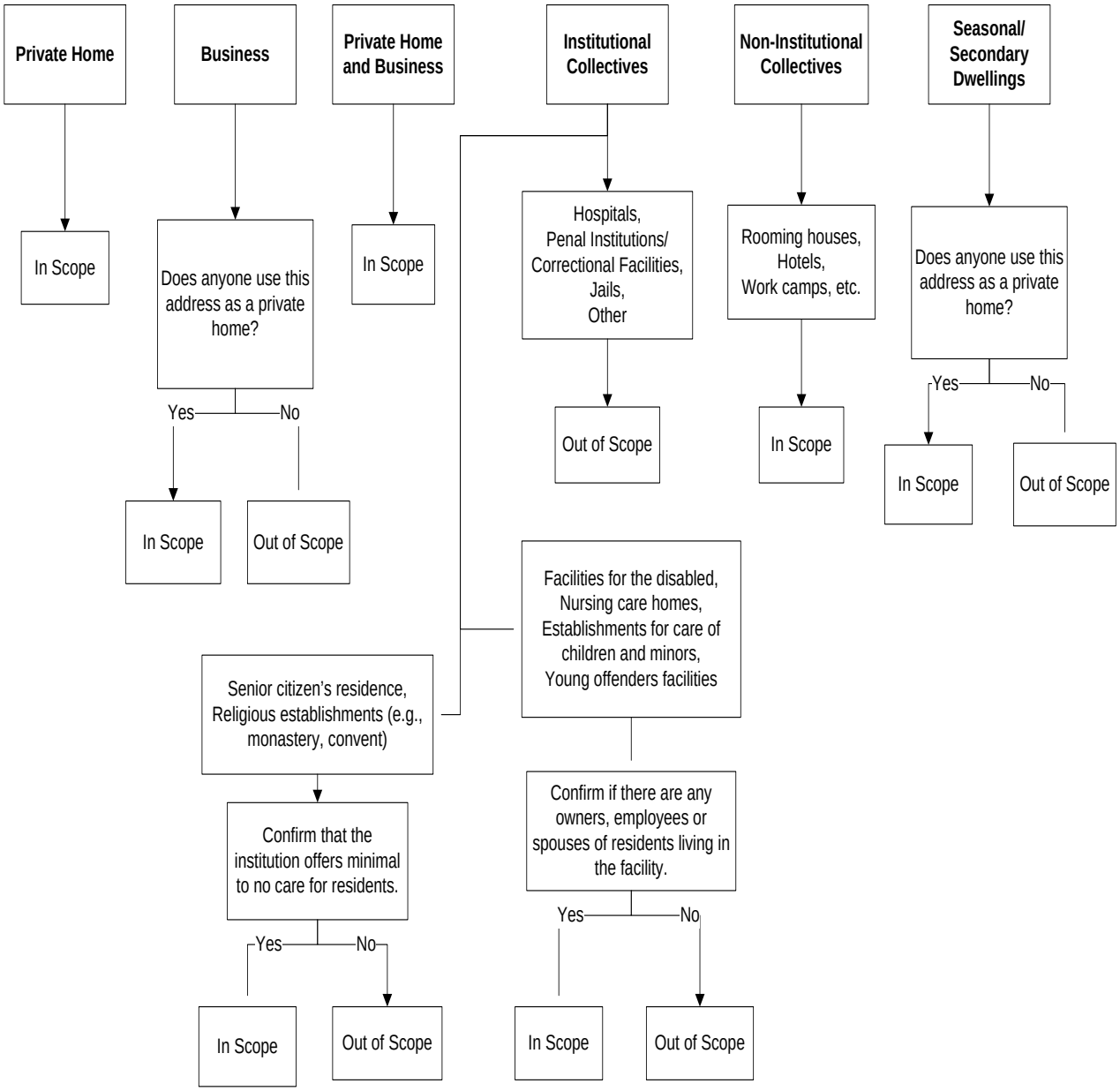
Address in scope chart (18+)..... E-2

Overview

Introduction This appendix presents the Canadian Community Health Survey address in scope chart for 18+.

Address in scope chart (18+)



Appendix F – Respondent materials

In this appendix This appendix contains:

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Overview

Introduction This appendix presents the Canadian Community Health Survey (CCHS) respondent materials.

Introductory letter

Date

Dear Sir or Madam:

The **Canadian Community Health Survey,** conducted by Statistics Canada, collects information from Canadians about their health and well-being, the factors that affect their health, and their use of health care services.

Your participation is important

The results from this survey will help communities as well as provincial and territorial ministries of health to better understand and address issues such as

- access to local health care and home care services
- lifestyle choices affecting health
- chronic health conditions.

Participating is easy and secure

- Sometime in the next few weeks, a Statistics Canada interviewer will contact you, and someone from your household aged 12 years or older will be randomly selected to participate in an interview.[1]
- To accommodate participants' busy schedules, our interviewers work days, evenings and weekends.
- This voluntary survey is conducted under the authority of the *Statistics Act*, which ensures that the information you provide will be kept strictly confidential. Your information may be used by Statistics Canada for other statistical purposes.

For more information

If you have any questions, please contact XXX toll-free at 1-800-XXX-XXXX (TTY: 1-866-753-7083). You can also consult the enclosed brochure or visit www.statcan.gc.ca/cchs.

Thank you in advance for your participation.

Sincerely,

Regional Director

1. If the selected individual is less than 16 years of age, we will seek verbal consent from a parent or legal guardian for this individual to participate in the survey.

Missed appointment letter

Date

Respondent's Name
Address

Dear Sir or Madam:

A Statistics Canada interviewer recently contacted a member of your household regarding the **Canadian Community Health Survey** (CCHS). At that time, an appointment for an interview was scheduled; however, we have been unable to reach anyone in your household since then.

Please contact us

Please contact XXX at xxx-xxx-xxxx, or toll-free at 1-8XX-XXX-XXXX (TTY: 1-866-753-7083) to reschedule your appointment. If we do not hear from you within the next few days, a Statistics Canada interviewer will follow up with you, and can answer any questions you may have.

Your participation is important

The CCHS is a national survey that asks Canadians across the country about their health and well-being, the factors that affect their health, and their use of health care services. Information is collected at the community level and the data will be used to:

- support local health units by providing information to evaluate existing programs and to design new ones
- provide key information on access and wait-time issues

For more information

Please visit www.statcan.gc.ca/cchs.

Thank you in advance for participating in the CCHS.

Sincerely,

Name
Data Collection Manager
xxx Region

No contact letter

Date

(Address)

Dear Sir or Madam:

Your household has been selected to participate in the **Canadian Community Health Survey** (CCHS), conducted by Statistics Canada. We have made several attempts to contact you, but so far have been unsuccessful in reaching you.

This survey collects information from Canadians about their health and well-being, the factors that affect their health, and their use of health care services.

Your participation is important

Regional health planners use the data to:

- ensure existing health services meet the needs of your community
- evaluate ongoing health programs
- plan new programs and services

Participating is easy and secure

Please contact XXX directly at XXX-XXX-XXXX, or toll-free at 1-8XX-XXX-XXXX (TTY: 1-866-753-7083) to schedule a convenient time for an interview.

The CCHS is conducted under the authority of the *Statistics Act*, which ensures that the information you provide will be kept confidential and used only for statistical purposes. We do not release any information that could identify you or any member of your household.

For more information

Please review the enclosed brochure, or visit www.statcan.gc.ca/cchs.

Thank you in advance for your participation.

Sincerely,

Data Collection Manager
XX Region

Refusal letter

Date

(Address)

Dear Sir or Madam:

Statistics Canada has contacted you regarding the **Canadian Community Health Survey** (CCHS), and you have told us that you do not wish to take part in this survey. Please allow us to explain why your participation is important.

Your participation is important
The results from the CCHS are used to plan health services to meet your community's future health-care needs and to evaluate existing health programs. To ensure that the data from this survey provide an accurate reflection of the health of the Canadian population as a whole, it is important to have the results of all selected participants.

The information you provide will represent many others and can make a positive difference in your community.

Participating is easy and secure
In the next few days, a Statistics Canada interviewer will contact you again. We hope that you will take the time to participate in the interview.

This survey is conducted under the authority of the *Statistics Act,* which ensures that the information you provide will be kept confidential and used only for statistical purposes. We do not release any information that could identify you or any member of your household.

For more information
Please visit www.statcan.gc.ca/cchs. You can also contact XXX at XXX-XXX-XXXX or, toll free at 1-800-XXX-XXXX (TTY: 1-866-753-7083).

We understand that your time is valuable, and thank you in advance for your participation.

Sincerely,

Data Collection Manager
XX Region

Partial completion letter

Date

Respondent's Name
Street
City (province)
Postal Code

Dear XXXXX:

As you know, Statistics Canada is currently conducting the **Canadian Community Health Survey** (CCHS), and we would like to thank you for the time you have invested in this survey so far.

Your participation is very important, and it is essential that all questionnaires be fully completed to ensure the survey results are as accurate and reliable as possible. We have attempted to reach you to complete the interview, but have been unsuccessful.

Please contact us

To arrange a convenient time to complete your interview, please contact xxx at XXX-XXX-XXXX or, toll-free, at 1-8XX-XXX-XXXX (TTY: 1-866-753-7083).

The results from this survey provide information necessary to planning new programs and services, evaluating ongoing programs, and customizing public health programs to meet specific needs in your community.

CCHS is conducted under the authority of the *Statistics Act*, which ensures that the information you provide will be kept confidential and used only for statistical purposes. We do not release any information that could identify you or any member of your household.

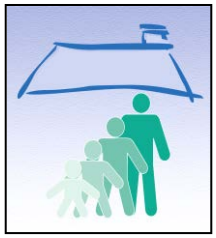
For more information

Please visit www.statcan.gc.ca/cchs.

We understand that your time is valuable, and thank you again for participating in this survey.

Sincerely,

Name
Data Collection Manager
XX Region



A note to parents and guardians
Interviewing youths for the
Canadian Community Health Survey

Information for a healthy future

The community has a vital role to play in giving today's young people the best possible chance at a lifetime of good health.

Delivering effective health care to all young people is one way for the community to carry out that responsibility. This includes delivering programs aimed at preventing disease and promoting good health.

To better determine what those programs should be in the future, regional health planners need up-to-date health information from young people in your area. Your child/teenager was randomly selected to participate in this survey.

Some of the subjects covered in the survey

- General Health
- Physical Activities
- Alcohol Use
- Chronic Conditions
- Use of Health Care Services
- Height and Weight
- Nutrition
- Smoking
- Activity Restrictions

Although the interview is being conducted with your child, for children under the age of 18, we will ask you to answer the last part of the survey which includes general questions about your household that your child may not be able to answer accurately.

Statistics Canada and your province or territory will link the information collected in the survey to other health records. If your child is less than 14, we will be asking your permission to share the information collected in the survey with Health Canada and provincial and territorial ministries of health.

The more information the better

Respondents of all ages can choose not to answer any question at any time during the interview. The more information we collect, the better the results are. However, even partially-completed interviews are useful.

We respect privacy and confidentiality

We need to get the most accurate information possible from respondents because it can lead to better health care services, programs and policies. This can prove challenging when interviewing respondents, both young and old, if they think other members of the household can hear their responses. We therefore prefer to conduct the interview in confidence.

There are ways of maintaining the confidentiality of your child's responses while ensuring that both you and your child are comfortable with the interview process. Your interviewer would be happy to discuss these with you.

We hope we can count on your support!

Statistics Canada Statistique Canada

Canada

Appendix G – Questions and Answers

In this appendix This appendix contains:

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Overview G-1
Introduction G-1
Questions and Answers G-2

Overview

Introduction This section presents the common questions and answers to the Canadian Community Health Survey

Questions and Answers

Introduction

The following information is designed to help you answer some of the questions commonly asked by respondents. Please become familiar with all of these answers so you can reply comfortably to similar questions asked while conducting an interview.

General 1.

Q. What is the Canadian Community Health Survey?

A. The Canadian Community Health Survey is a **national health survey** that asks a representative sample of people living in Canada about their health and wellbeing, factors that affect their health and their use of health care services.

This survey will provide health information to local community health planners to evaluate existing health promotion programs, and to plan future ones.

2.

Q. What are the objectives of this survey?

A. To support local health units by providing them with timely information they need to evaluate existing policies and programs and to design new ones suited to their communities.

To provide more current, detailed and uniform health information in every province.

3.

Q. What kinds of questions will you ask?

A. The survey asks a variety of questions about:

- the current state of your health
- behaviours thought to be related to your health
- your use of health care services
- socio-demographic information concerning your household

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4. **Q. Who uses this information? How will it be used?**

A.

- **Provincial health ministries and federal government departments** will use this information when making funding decisions, developing priorities and identifying areas of concern for legislation, new policies and programs.
- **Researchers** will have more health information to work with than ever before, in order to monitor, analyse and measure those factors affecting health and to report on them.
- **Local health authorities, community groups and social agencies** need this type of information to determine how to best allocate resources in order to improve the health of the people living in their community. A key use for the information from this survey will be the development of public education campaigns primarily aimed at health promotion and disease prevention.
- **The media** will use this information to raise awareness about health, and to report on trends in the health status of Canadians.

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5. **Q. Who is involved in conducting this survey?**

A. The survey is being conducted by Statistics Canada, with the support of Health Canada and the Public Health Agency of Canada. Provincial and Territorial Ministries of Health contribute to the content of the questionnaire.

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6. **Q. Why are you spending money on this survey when you could be putting it into health care?**

A. Federal and provincial governments spend billions of dollars on health care. It is important that decisions about health priorities be well informed so that your tax money can be spent in the most effective way.

Participation

7.

Q. How are people selected for this survey?

A. CAPI/CATI (18+): Your address was randomly selected. We will then roster the household to see who is currently living there, and then randomly select one person aged 18 or over to complete an interview.

In being selected, you represent the views of others in your community. Your participation in our survey ensures that we have an accurate and up-to-date picture of the health status of the population.

A. CATI (12-17): Respondents were randomly selected from administrative data produced by Statistics Canada. One eligible child between the ages of 12 to 17 years old is pre-selected for the CCHS survey.

Your participation in our survey ensures that we have an accurate and up-to-date picture of the health status of the youth population aged between 12 and 17 years old. In being selected, you represent the views of others in your community.

8.

Q. How have you chosen my household?

A. 18+: Your household has been randomly selected for the CCHS survey from a list of addresses. The phone numbers are provided by different phone companies across Canada. Your participation in our survey ensures that we have a representative and up-to-date picture of the health status of the population.

12-17: An eligible member of your household between the ages of 12 to 17 was randomly selected from administrative data produced by Statistics Canada. Your participation in our survey ensures that we have an accurate and up-to-date picture of the health status of the youth population aged between 12 and 17 years old. In being selected, you represent the views of other youth in your community.

9.

Q. I just did a similar survey. Why are you contacting me again?

A. Ask respondent for the name of the survey.

There are a number of surveys being conducted at this time. Every effort has been made to reduce the number of surveys that each person will be asked to answer. It is possible that you have been selected for more than one survey. All of the surveys we do are important and relate to different aspects of Canadian life.

10.

Q. My number is unlisted, how did you get it?

A. Unlisted numbers may appear on these lists because of potential delays between when the number is unlisted and when Statistics Canada receives the updated files. This happens rarely and is how we would have obtained your unlisted number.

11.

Q. This is impossible, my number has been unlisted for a long time, and how did you get it then?

A. **CAPI:** I am sorry to hear this. The reason why I was given your phone number to call is then unknown to me. However, I will report this situation to my supervisor for further investigation. Would you accept to continue the interview?

12.

Q. Do I have to take part in this survey?

A. Health is an important part of everyone's life. For the results to be representative, it is important that we have data from all spectrums of the Canadian population, including those in good and bad health.

Your participation in this survey is voluntary, however, your participation will help to ensure that survey results are complete and representative of the entire population

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13.

Q. How long will it take to complete this survey?

A. It takes, on average, about 45 minutes to complete the interview. The interview length varies depending on each respondent's answers.

14.

Q. I'm too old. Other people could better answer your questions

A. Health is an important part of everyone's life. We need to interview a variety of people, old and young, men and women, if the results of the survey are to be representative. The responses you provide to the questions on the Health Services you use or the medication you take are valuable to the overall picture of Health in Canada. Your answers will also help local health programmers to improve and develop health programs tailored to your local population.

15.

Q. My child is too young to answer these questions.

A. In order to design appropriate health programs for youth in Canada, we need information directly from them. We would prefer to interview each respondent alone, in order to get the most truthful answers possible. Please rest assured that we will not continue with certain questions if it becomes obvious that your child is uncomfortable.

16.

Q. Can I participate in this survey on behalf of my child?

A. In order to design appropriate health programs for youth in Canada, we need information directly from them. The only condition under which a parent or another adult may answer questions on behalf of a child respondent is if the mental or physical health of the child prevents them from completing the interview. Please rest assured that we will not continue with certain questions if it becomes obvious that your child is uncomfortable.

Privacy and Confidentiality

17.

Q. How can someone verify that the survey is legitimate?

A. To confirm that Statistics Canada is conducting this survey, you are welcome to:

- Call our toll-free number *(specify): 1 800 xxx-xxxx.* In addition to confirming the legitimacy of the survey, the person you reach can also confirm that I am a Statistics Canada interviewer.
- Call the Statistics Canada regional office telephone number listed in your telephone book. In addition to confirming the legitimacy of the survey, the person you reach can also confirm that I am a Statistics Canada interviewer.
- Visit the Statistics Canada website: www.statcan.ca and click on "Information for Survey Participants" on the main page and look under the Canadian Community Health Survey heading.

18.

Q. The Government has too much information already.

A. The focus on health has been shifting over time to health promotion and disease prevention. As a result, information needs have changed. Local health units in particular are in need of information about the health of the people in their communities, in order to design programs and services appropriate to those communities. This survey contains questions chosen by your health unit to address some of its specific information needs.

19.

Q. Why do you want to know my total income?

A. Total income is one factor needed to determine an individual's overall economic situation. Although many health expenses are covered by health insurance, there is still a relationship between people's health and their economic situation. The survey will help us learn more about that relationship. Please be assured that, like all other information you have provided, your income will be kept strictly confidential.

20.

Q. Why do you want to know my sexual orientation?

A. Information on the sexual orientation of the population is needed to better understand a wide range of health issues that affect the homosexual, bisexual and heterosexual population. These include experiences of discrimination, access to health care, and behaviours such as physical activity and smoking.

This knowledge will help policy makers determine the issues that need to be addressed to improve health and health care for various groups. For example, knowing that smoking rates are higher among homosexuals than heterosexuals may help policy departments determine how to best address the issue, or where to direct health prevention and research initiatives.

21.

Q. Why do you want to know my provincial health insurance number?

A. The health number will help us to link the information collected in the questionnaire, such as health status and factors affecting health, with provincial health records. These records contain information about the use of health services and the occurrences of some types of illness and disease.

Linking to this information saves you from answering additional questions on the survey, reduces the cost of collecting the information, and provides a richer and more complete set of data for analysis.

22.

Q. How will I know if all this will be kept confidential?

A.

- This survey is collected under the authority of The Statistics Act, which ensures that any information you provide to us remains strictly confidential. Statistics Canada goes to extraordinary lengths to protect you. No unauthorised persons outside of Statistics Canada will ever be permitted to see your individual responses without your permission.
- All employees of Statistics Canada are under oath, meaning they are subject to severe penalties including prosecution if they do not safeguard your confidentiality.
- Personal information obtained by Statistics Canada from surveys is never released. All publications and other releases are carefully reviewed from this point of view.
- If you choose to share your answers with the provincial health ministries and Health Canada, which will reduce time and costs, they too are required by law to safeguard confidentiality.

23.

Q. Are you going to sell my information?

A. No. We do not sell any personal information about survey respondents.

24.

Q. Will my name get on any mailing lists?

A. No. Survey lists from Statistics Canada are for internal use only and are kept strictly confidential. They are not used for any marketing purposes, and they are not sold to private companies.

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25. **Q.** **What is the data-sharing agreement?**

A. It is an agreement between Statistics Canada, Health Canada and the provincial ministries of health to share the information collected from this survey. Its purpose is to reduce the burden on respondents, and to reduce the cost of data collection. This means that, in many cases, the provinces will not have to collect this information through their own surveys. The information you provide to us can only be shared with your consent.

These ministries are also prohibited by law from disclosing any identifiable information based on data collected by Statistics Canada.

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26. **Q.** **How can people get a copy of the results?**

A. The survey results are made available to the public in many ways, including the Statistics Canada Web site (www.statcan.ca), the Statistics Canada regional reference centres, approximately 700 public libraries across Canada and the media.

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27. **Q.** **When will the results of the survey be available?**

A. Results from the survey will be available in the spring following the current collection year.

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28. **Q.** **How do I know that my child's privacy will be protected?**

A. All of the surveys done by Statistics Canada are collected under the Statistics Act. This means that by law, all of the information must be kept confidential. All employees of Statistics Canada are under oath and may be prosecuted or penalized if they do not safeguard that confidentiality. Survey results will be grouped together to ensure that no individuals can be identified.

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29. **Q.** **Child: You have asked some very personal questions about my body and my sexual activity. Why would you want to know anything about that?**

A. We want to know about the full range of experiences that you have. Many youth are faced with important decisions about sex, and we would like to know how these choices have an impact on their lives.

---

30. **Q.** **Can my parents or the police find out about my answers?**

A. All of your responses to the questionnaires and interview are kept confidential. That means that no one will be able to associate your name with the answers that you have given. We do not send the results to the police; in fact it is against the law (Statistics Act).

Once the interview is completed, the questions you answered cannot be reopened, not even by the interviewer. Your answers are stored and then transmitted to StatCan. The interviewer cannot tell your parents, teachers or anyone else anything you answer on the survey. Statistics Canada will provide all respondents (or their parents if the respondent is under 18) a copy of their answers if they write a letter to Statistics Canada and sign it. Since CCHS 2015 is a large study, we anticipate that it will take at least a year to process the data. We would not be able to provide your parents with your data until the fall of 2016.

We hope this makes you feel that you can be honest when you are answering the questions.

Rapid Response Module (January – June 2016)

Prescription Cost-Related Non-Adherence (PCN)

Overview

This rapid response module collects data on non-adherence to prescription medication due to cost. Non-adherence can be defined by behaviours such as failing to fill, collect or take prescribed medication, reducing or skipping the medication dosage, or delaying filling prescriptions.

The questions related to the financial aspect of non-adherence to prescription medication aim to capture the direct costs of afforded or not afforded medication, in the last 12 months.

In addition, respondents are also asked questions on the behaviours of health care professionals and pharmacists to help alleviate the cost-related burden of their patients.

They are also asked to report the health conditions for which prescriptions were issued and could not be afforded, and lastly, the negative effects on their health and other aspects of their life because of their inability to afford their prescriptions.

The data collected will be used to identify the segment of the Canadian population who engage in cost-related non-adherence of prescription medication, what health conditions are involved, and what trade-offs are made to their other expenses to compensate.

PCN_Q005

During the last 12 months, was there a time when you did not fill or collect a prescription for your medicine, or you skipped doses of your medicine because of the cost?

INTERVIEWER: Read categories to respondent. When answering on behalf of a child, ensure that the response is for the child's prescriptions only. Include when a parent or someone in the household made the decision on behalf of the child.

1. **Yes**
2. **No**
3. **Not applicable, he/she had no medication prescription to fill in the last 12 months**

DK, RF

PCN_Q010

In the last 12 months, was there a time when you reduced the dosage of your medication or delayed filling your prescription, because of the cost?

INTERVIEWER: Please include when a parent or someone in the household made the decision on behalf of the child.

1. Yes
2. No
3. Not applicable, he/she had no medication prescription to fill in the last 12 months

DK, RF

PCN_Q015 **In the last 12 months, how many different medications have you received a prescription for, including on-going prescriptions?**

INTERVIEWER: Read categories to respondent.

1. **0**
2. **1 or 2**
3. **3 or 4**
4. **5 or more**

DK, RF

PCN_R020 **Now some questions about the cost of your prescriptions.**

INTERVIEWER: Press <1> to continue.

PCN_Q020 **In the last 12 months, how much money was spent for your own medication prescribed by a healthcare professional? Please exclude any amounts reimbursed by an insurance plan.**

INTERVIEWER: Read categories to respondent. The respondent can refer to the last taxation year instead of the last 12 months, if necessary.

1. **0**
2. **1 to 200 dollars**
3. **201 to 500 dollars**
4. **501 to 1000 dollars**
5. **More than 1000 dollars**

DK, RF

PCN_Q025 **In the past 12 months, how often did you delay or not have a prescription filled, or not take the medication as prescribed because of the cost?**

INTERVIEWER: Read categories to respondent. Please include when a parent or someone in the household made the decision on behalf of the child.

1. **Rarely**
2. **Sometimes**
3. **Often**
4. **Always**

DK, RF

PCN_R030 **The next few questions concern all of your prescriptions for medication that you could not afford in the last 12 months.**

INTERVIEWER: Press <1> to continue.

PCN_Q030 **What health conditions were these prescriptions meant to treat?**

INTERVIEWER: Read categories to respondent. Mark all that apply. If the respondent answers 'ADD/ADHD', select 'Other condition'.

1. **Heart disease, high cholesterol, or high blood pressure**
2. **Asthma or COPD (chronic obstructive pulmonary disease)**
3. **Depression, anxiety, or other mental health condition**
4. **Diabetes**
5. **Gut problems (e.g. peptic ulcer, heartburn, bowel disease)**
6. **Arthritis or other chronic pain**
7. **An infection(e.g. bacterial, fungal, or viral)**
8. **A type of cancer**
9. **Other condition**

DK, RF

PCN_Q035 **Do you think that being unable to afford your prescriptions made your health worse?**

1. Yes
2. No

DK, RF

PCN_Q040 **Did being unable to afford your prescriptions result in you having to do any of the following:**

INTERVIEWER: Read categories to respondent. Mark all that apply.

1. **Go to the emergency department?**
2. **Be admitted to hospital?**
3. **Go to the doctor, which you would not have had to do otherwise?**
4. None of the above

DK, RF

PCN_R050 **The next few questions concern the last time you couldn't afford to pay for your prescription in the last 12 months.**

INTERVIEWER: Press <1> to continue.

PCN_Q055 **Did the healthcare professional, who gave this prescription to you, ask if ou could afford to pay for it?**

INTERVIEWER: If the healthcare professional only asked about the ability to pay after a prompt from the patient, mark this answer as 'No'.

1. Yes
2. No

DK, RF

PCN_Q060 **Did he/she help you come up with strategies to lower the cost, such as suggesting cheaper alternative medications, or providing free samples?**

INTERVIEWER: If the healthcare professional only suggested alternatives because of a prompt from the patient, mark this answer as 'No'.

1. Yes
2. No
3. Not applicable, no alternative or free sample exists for what respondent cannot afford.
   DK, RF

PCN_Q065 **Did the pharmacist, who filled this prescription, ask if you could afford to pay for it?**

PCN_Q070 **Did he/she help you come up with strategies to lower the cost, such as suggesting cheaper alternative medications or providing a drug discount card?**

INTERVIEWER: If the pharmacist only suggested alternatives because of a prompt from the patient, mark this answer as 'No'.

1. Yes
2. No
3. Not applicable, no alternative or discount card exists for what respondent cannot afford
   DK, RF

PCN_Q075 **In the last 12 months, have you ever spent less on any of the following in order to pay for your prescription medications.**

INTERVIEWER: Read categories to respondent. Mark all that apply. Please include when a parent or someone in the household made the decision on behalf of the child**?**

1. **Food**
2. **Housing**
3. **Heat for your dwelling**
4. **Car, public transit, or other transportation costs**
5. **Leisure or vacation**
6. **Other healthcare expenses for yourself or your family**
7. **Other expenses not already mentioned**
8. **No - Have not spent less on anything**
   DK, RF

RFH_ END